



CRUCIAL CONVERSATIONS FOR PEERS: **A FRESH APPROACH**



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MESSAGE FROM MDUDUZI MANANA, DEPUTY MINISTER OF HIGHER EDUCATION AND TRAINING

The Department of Higher Education and Training strives to identify and deploy effective strategies in order to lead national knowledge generation, advance graduate competency and throughput rates and enable healthy confident graduates to enter the workplace.

We invest in education and socio-economic development where investments are needed. We seek to magnify these investments in order to ensure better results. But this inevitably meets certain obstacles – and HIV/AIDS, STIs, TB and other health factors that impact on the wellbeing of students and staff in our sector are among them.

Peer education is one of the most powerful ways through which we can address these obstacles: it works to effect change not just at individual but at group and national levels.

It allows us to increase reach and access, empower our students with agency for positive behaviour change and enhance their ability to protect themselves and others, all the while learning from each other and feeling part of a bigger, nationally supported goal and structure.

Our work in the area of peer education is evidence-based and we are learning from local and international best practice which has informed this manual.

Peer education is linked to and strengthens our core mandate: education. It touches a number of common spheres – including lecturing, development of critical thinking, distributing materials, dialogue, drama, advocacy, counselling, making referrals to services and providing support.

The belief that peer education can work for us is based on a number of behavioural theories that tell us that people make changes on the basis of how their close, trusted peers respond to the same stimuli – such as risks, information, achievements and recognition.

It may seem paradoxical that peer educators come from the very system they seek to change because they have inside knowledge of the target group. This very quality is what makes this approach effective and cost-efficient. It is founded on involving, training and supporting committed volunteers who are in touch with the needs of their friends, colleagues, family and community members. It is participatory and its main aims are to remove the barriers that stand in the way of accepting and acting on messages that overcome stigma, denial, fear and poor access to services.

Peer education is the beating heart of the HEAIDS Programme – one that unites passion, effort, context and our sector into a common cause for healthy, protected and empowered youth.



FOREWORD BY DR RAMNEEK AHLUWALIA, DIRECTOR OF HEAIDS

The Higher Education and Training HIV/AIDS Programme (HEAIDS) is grateful for the assistance of its multi-member constituency including 76 institutions of higher education and training, the DHET, and numerous partners in the creation of this manual.

It has been designed to be fresh, practical and useful.

The manual brings to life the concept of peer education as a process, a strategy, a communication channel and a tool. Peer education is also, according to the UNFPA and Family Health International, “a process whereby well-trained and motivated young people undertake informal or organised educational activities with their peers” – that is, those similar to them in age, background and interest.

We hope that it will empower young people to work with others with whom they have physical and socio-cultural affinity. It is designed to advance education about difficult topics – those that leave many parents and teachers stumped, like pregnancy, STIs, HIV and rape. It aims to do this through participative, peer-led action learning, development of skills in communication and leadership among peer educators and their peers.

The manual advocates for the FRESH approach. It provides information that touches a number of nerves: Focus, Resources, Education and Empower, Sensitise, Health and Human Rights.

The approach is embedded in HEAIDS programmes and in curriculum-based activities in order to communicate clear messages about behaviour and result in high-impact sexual health programmes.

We might think we know which behaviours we’d like to change, but how many positive behaviours can we name? The HEAIDS programmes provide the road signs and advocate for abstaining or delaying sex, decreasing frequency of sex, decreasing number of sexual partners, avoiding concurrent or multiple sexual partners, increasing the time gap between sexual partners, and using a condom consistently and correctly every sex act. None of them are easy subjects to broach. Knowing and trusting the messenger is part of the success.

I hope that the users of this manual find that it enables role players within the higher education and training sector to respond to their mutual commitment to First Things First – to advocate that the first priority of every student, staff member and fellow South African is to know his or her HIV status and protect their own and others’ health and wellbeing.

This guidebook can answer questions regarding the appointment of peer educators, supervision of staff, recruiting, training and retaining, monitoring and evaluation.

The manual consists of modules, each of which addresses a broad subject area that tackles connected issues in a holistic manner.

Its aim is to harness the power of personal understanding, experiences and social networks, to provide a single method for training peer educators – yet to also link all of us through shared objectives, strategies and materials so that we can meaningfully address barriers to good health.

Our ultimate goal is to increase the uptake of the HEAIDS programmes and sector- and campus-based services so that our health indeed comes first.

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ABBREVIATIONS LIST

AIDS:	Acquired Immune Deficiency Syndrome
ANC:	Antenatal Care
ARVs:	Antiretrovirals
CD4:	Cluster of Differentiation 4
CICT:	Client-initiated Counselling and Testing
DHET:	Department of Higher Education and Training
DOH:	Department of Health
DOTS:	Directly Observed Treatment Short course
DNA:	Deoxyribonucleic Acid
FTF:	First Things First (HIV/TB/STI) Programme
GBV:	Gender-based Violence
HCT:	HIV Counselling and Testing
HEAIDS:	Higher Education and Training HIV/AIDS Programme
HEIs:	South Africa's Public Higher Education Institutions
HIV:	Human Immuno-Deficiency Virus
HST:	Health Systems Trust
HTS:	HIV Testing Services

LGBTI:	Lesbian, Gay, Bisexual, Transgender and Intersex
LTBI:	Latent Tuberculosis (TB) Infection
MDR:	Multi-drug Resistant
MMC:	Male Medical Circumcision
MTCT:	Mother to Child Transmission
NDOH:	National Department of Health
OI:	Opportunistic Infections
PEP:	Post Exposure Prophylaxis
PICT:	Provider-initiated HIV Counselling and Testing
PLWHA:	People Living With HIV and AIDS
Q & A:	Questions and Answers
STIs:	Sexual Transmitted Infections
TB:	Tuberculosis
TST:	Tuberculin Skin Test (also called Mantoux skin test)
TVETs:	Technical and Vocational Education and Training Colleges
UNAIDS:	Joint United Nations Programme on HIV/AIDS
WHO:	World Health Organization
XDR:	Extensively Drug-Resistant

CHAPTER 1: **PEER EDUCATOR MANUAL**



PROGRAMME OVERVIEW

INTRODUCTION

The Higher Education and Training HIV/AIDS Programme (HEAIDS) is a national programme, designed to develop and support general health and wellness efforts at South Africa's public Higher Education Institutions (HEIs) and Technical Vocational Education and Training (TVET) Colleges. HEAIDS special focus intervention areas include the Human Immunodeficiency Virus, Tuberculosis and Sexually Transmitted Infections (HIV/TB/STI). Over and above that the programme targets general health and wellness.

Young South African students face many health risks which include HIV, STI and TB infection, chronic conditions such as diabetes and hypertension (caused by poor lifestyle choices), cancers, unwanted pregnancies and substance use/abuse. The programme seeks to contribute to the higher education sector's goal to produce healthy and productive graduates. Based on the belief that health care is the priority of all South Africans, the HEAIDS programme aims to increase knowledge, get rid of misinformation and encourage and motivate young people to remain healthy through regular testing and/or screening for major ailments such as HIV/TB/STI, cancer and cardiovascular risk factors. HEAIDS operates in both the university and TVET college sectors.

BACKGROUND

South Africa has the largest HIV epidemic in the world. It is estimated that more than 6 million people are infected with HIV. According to statistics provided by the United Nations, 19.2% of the adult population between the ages of 15 and 49 are infected with the virus.

Becoming sexually active at an early age increases the lifetime risk of getting HIV and is associated with other HIV risk factors including alcohol and drug use, unprotected sex, unplanned pregnancy, and multiple sexual partners.

On 10 May 2016, the South African Minister of Health, Aaron Motsoaledi, announced in his Health Budget Vote Speech to the Parliament of South Africa that the country will implement a new policy of offering HIV treatment to all people living with HIV by September 2016. He also announced that there would be no need to check the CD4 cell count to determine whether people can get access to treatment. Instead, the "test and treat" strategy means that people who are diagnosed HIV-positive will be offered HIV treatment as soon as possible after diagnosis. However even though there is a lot of progress regarding the country's HIV treatment programme, with approximately 3.5 million people on HIV treatment today, the number of new HIV infections is still very high, with an estimated 340 000 new HIV infections in 2014.

Over and above HIV, South Africa has the third highest burden of disease of TB in the world, after India and China. There were approximately 450 000 cases of active TB in 2013, an increase of 400% over the last 15 years according to the World Health Organisation (WHO, 2014). Of the 450 000 incident cases, it was reported that 60-73% have HIV and TB co-infection. The incidence of multidrug-resistant (MDR) and extensively drug-resistant TB (XDR-TB) are increasing and South Africa has the second highest number of reported multi-drug-resistant TB (MDR-TB) cases globally (NDOH, 2014 and HST, 2014). TB remains the leading cause of death in South Africa, contributing to 12% of deaths in 2009 (StatsSA, 2014).

The HEAIDS programme seeks to address the four broad causes of death (quadruple burden of disease) in South Africa. These include HIV/AIDS and TB, non-communicable diseases (diseases which cannot be transmitted from one person to another such as cancer, diabetes and heart disease), and injuries. The programme is designed to educate through participative, peer-led action learning, as well as to develop skills in communication and leadership amongst peer educators.

HOW TO USE THE TOOLKIT

WHAT DOES THE TOOLKIT CONTAIN?

The toolkit comprises:

A **Peer Mentor Guide** called ***Crucial Conversations with Peers: A Guide for Peer Mentors***. It contains a basket of tools comprising background information (research evidence and facts), tools and strategies, and skills required by peer mentors to do the following:

- Recruit and select peer educators
- Prepare and co-ordinate training for peer educators
- Train and upskill peer educators
- Facilitate peer educator training workshops

- Manage and oversee the peer educator programme
- Supervise and mentor peer educators
- Monitor and evaluate the peer educator programme
- Compile reports

A **peer educator's Leader's Resource** called ***Crucial Conversations with Peers: A Leader's Resource***, contains a package of information, tools (support material –see details below) and guidelines to enable peer educators to conduct peer education. It contains the following resources for training:

- Facts sheets (can be used as handouts)
- Handouts/exercises for participants
 - Model answers (for quizzes, Q&As, and other exercises)
 - Energisers for each module

A **peer educator facilitator guide** called ***Crucial Conversations with Peers: A FRESH Approach***. The guide is designed to grab the attention of peers and peer educators and move away from the concept of “education” which is traditionally associated with work and studying. The idea of the “FRESH” approach is about “keeping it fresh”, in other words, keeping up to date with new developments, changes, policies etc. It's also about giving the programme the feel of being new and refreshing and holds the promise of something different. In this way it seeks to capture their interest and draw them towards the programme. It provides a step-by-step guideline to peer educators on how to conduct every activity, leading questions to guide discussions and guidelines on how to prepare for training.

Each module which targets a specific health focus area may be divided into the following components that form the building blocks for the

FRESH ACRONYM:

1. **FOCUS:** Every module is launched using a ‘crucial conversation’ between “peers” that targets the focus area of the module. A crucial conversation is simply a simulated conversation between “peers” about the subject matter. These “conversations” are “crucial” because they seek to highlight the way young people think about crucial issues and target ignorance and misinformation, anxieties and fears as well as peer pressure. It is recommended that peer educators act out the mini scenarios OR invite participants to volunteer to read it out aloud. They provide an interesting youth-friendly way to kick-start a discussion that invites participants to demonstrate baseline knowledge about the module's core focus. This section also sets out the objectives for the module, which appear on the introductory slides that have been prepared for every module.
2. **RESOURCES:** Each module contains **key information** that provides a background to the focus area of the module, emphasising key aspects to capture participants' attention. **Resources** include:
 - **Fact Sheet/s** which provide facts and knowledge relating to the focus area of the module (these may be found in the **Leader's Resource** and may be used as handouts)
 - **Tools** to execute activities (games, energisers, creative techniques and ideas, step-by-step instructions)
 - **Directory of Services** – information about where to go for more information counselling and testing/treatment for the condition/s in question
3. **EDUCATE AND EMPOWER:** Each module contains **lessons pertaining to specific aspects targeted within the module. Each lesson achieves its objectives using youth friendly activities** for action learning designed to educate and empower participants with knowledge and skills relevant to the focus area. Detailed instructions are provided to guide peer educators on how to conduct each activity. Each lesson has the following structure:
 - a. Purpose
 - b. Requirements
 - c. Time
 - d. Procedure
 - e. Resources – everything that would be required to run the activities is included in this section (e.g. Q&A, setting the scene, role plays, etc.) NB: Model answers for activities (quizzes/Q&As etc.) may be found in the **leader's resource**
4. **SENSITISE:** Each lesson provides opportunities for discussions, debates and multi-media input designed to sensitise peers to issues pertaining to the focus area of the module
5. **HEALTH AND HUMAN RIGHTS:** Each lesson contains a summary at the end showing the relevance of the focus area of the module to the health and human rights of peers.

HOW TO USE THIS GUIDE

The **Peer Educator Facilitator Guide** is divided into five modules. Each module contains a number of lessons which can be facilitated as stand-alone topics or focus areas.

MODULE ONE: FIRST THINGS FIRST PROGRAMME

First Things First is a HEAIDS programme that aims to increase knowledge and eliminate misinformation around communicable and non-communicable diseases. The primary goal of FTF is to encourage health-seeking behaviour and enhancement of quality of life in young people, through regular testing and/or screening for major ailments such as HIV/TB/STI, cancer and cardio-vascular risk factors, amongst others. The First Things First Programme specifically targets the following issues:

- HIV/AIDS
- TB
- Sexually Transmitted Infections (STI)
- Hypertension, diabetes and/or cardiovascular risk factors
- Cancer
- Family planning
- Condom Promotion
- Male medical circumcision.

The ***First Things First module presents 3 lessons*** focused on the following areas:

- HIV/AIDS
- TB
- Sexually Transmitted Infections (STI)

All other areas pertaining to the First Things First programme will be covered in either the Women's Health or Men's Health modules.

MODULE TWO: WOMEN'S HEALTH AND EMPOWERMENT

The Women's Health and Empowerment module aims to encourage the utilisation of sexual and reproductive health services, empower young women on how to protect themselves from infection and gender-based violence and motivate them to maintain optimal levels of health and well-being. The module presents four lessons focused on the following areas:

The module has 4 lessons which focus on 3 targeted areas viz:

- Overview of Women's Health, screening tests for women and specifically women's cancers
- Family Planning
- Gender –based violence
- Personality and power

MODULE THREE: MEN'S HEALTH

The Men's Health module aims to encourage men to utilise health services including sexual and reproductive health services, empower them on how to protect themselves from infection and maintain optimal levels of health and well-being motivate them to become involved in the prevention of physical and sexual violence.

The module has 5 lessons which focus on 3 targeted areas viz:

- Overview of Men's Health, screening tests for men and specifically men's cancers
- Condom use
- Male medical circumcision
- Challenges of being a man in South Africa
- Gender-based

MODULE FOUR: LESBIAN, GAY, BISEXUAL, TRANSGENDER AND INTERSEX HEALTH (LGBTI)

The LGBTI module aims to review accurate and relevant information about each of the key health risks pertaining to LGBTI persons and address stigma and discrimination of the LGBTI community. The module has 4 lessons which focus on the following targeted areas viz:

- Human sexuality
- Overview of LGBTI Health
- Myths and misconceptions related to LGBTI
- Stigma and discrimination

MODULE FIVE: ALCOHOL AND DRUG ABUSE PREVENTION (ADAP)

The aim of the ADAP module is to expose peers to information pertaining to alcohol and drug use/abuse that can lead to better health outcomes and ultimately influence students to reduce alcohol and drug use/abuse. The module has 5 lessons which focus on the following targeted areas viz:

- Overview of alcohol and drug abuse prevention (Rationale, goals and key definitions)
- Health issues related to ADAP
- Reviewing knowledge on the SA drug scene
- Impact of alcohol and drug use/abuse: mental, social and physical
- Principles of good practice in working with ADAP programmes
- Knowledge, Skills, Attitudes and Values required by peer educators working with ADAP programmes

MATERIALS NEEDED

Materials required for activities in each of the modules are set out separately for each activity. The list below suggests standard materials that will be required for all modules:

- Flip chart and pens to record plenary discussion
- Post-its in different colours
- Slides and projector
- Laptop
- Power cables/adaptors and flash drives (if applicable)
- Suggestion/worry box for anonymous questions or comments
- Activity: Hand-outs
- Fact Sheets: Hand-outs
- Specific materials as required per activity
- List of local resources for accessing services

RUNNING ORDER

1. Read/act out the '**crucial conversation**'
2. Ask for comments/ what do participants know about the subject being discussed
3. Present slides on module objectives
4. Ask for comments/contributions and/or any further areas that require clarity
5. Facilitate lesson and all relevant activities
6. Lead plenary discussion
7. Present slides with key information to fill in gaps in knowledge, update information, correct misconceptions and clarify issues and show how the subject matter links to HEALTH and HUMAN RIGHTS
8. Lead discussion on way forward

MODULES	Objectives and rationale	Activities	Plenary, reflection and wrap up	TOTAL DURATION
First Thing's First	20 mins	140 mins	60 mins	240 mins
Women's Health and Empowerment	40 mins	300 mins	20 mins	360 mins
Men's Health	20 mins	200 mins	20 mins	240 mins
LGBTI Health	15 mins	120 mins	15 mins	150 mins
Alcohol and Drug Abuse Prevention (ADAP)	15 mins	180 mins	15 mins	210 mins
TOTAL PROGRAMME DURATION				1200 mins (20 hrs)

MODULE ONE: FIRST THINGS FIRST



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CRUCIAL CONVERSATIONS

Boni and her friends Sashin, Thashni and David are sitting in the café chatting:



Woohooooo! 3 days of no campus!

What do you mean? It's not a holiday. Or if it is no one told the chief! Duhhh.



Where have you been Sash? Don't you know we have 3 days of the FTF activation, so they cancelled lectures

OK- so let me get this right... no campus, but FTF activation?



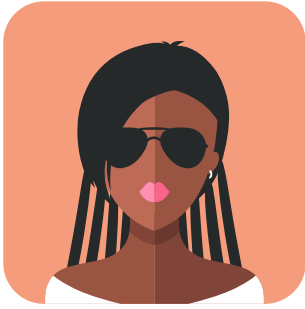
Oh yeah, oh yeah! (dances around) Time for movies and love. Mwah! (kissing sound)



So I've never been to one of those first... whatever's... so, it's pointless then, right?

Hey, I wouldn't say that, eh. I would never have known I was HIV-positive if I hadn't gone last year. And who knows, I could have passed it on to you, Sash. At least now I know, since I started on ARVs it's all under control.





Oh yeahhhhh, that's kinda true hey...hmmm...
Aaargh! I HATE you Cam. Why did you have to say that? Now I feel like I have to go get tested and stuff. So much for my free day!

Well Bons, I heard they're doing mammos as well this time. They have a mobile clinic set up. And you know you've been putting it off right. With your mum having breast cancer AND the lump you found, I'm super stressed for you. You should go.



Okaaay fine, don't nag, I will.
Are you coming with?

Of course I am. I also want to go for HCT. I feel anxious every time I think about the fact that I don't know what's happening inside my body.



Hey Thash, you don't have to stress. You and Greg have been together for 2 years now. And seriously, that oke is mal over you.

Seriously Cam?! Are you kidding me? You think I should assume I'm safe just because Greg loves me right? OMG, you definitely need to attend. I think you have a lot to learn.



AIM

First things First is a HEAIDS programme that aims to increase knowledge and eliminate misinformation around communicable and non-communicable diseases. The primary goal of FTF is to encourage health seeking behaviour and enhancement of quality of life in young people, through regular testing and/or screening for major ailments such as HIV/TB/STI, cancer and cardio-vascular risk factors, amongst others.

OVERALL OBJECTIVES OF THE MODULE

By the end of this module, participants will:

- Be clear about the goals and objectives of the FTF programme
- Understand the importance of, and need for, testing and screening on a regular basis
- Have a clear definition of each of the 3 health risk areas outlined above
- Demonstrate an understanding of the facts around causes, symptoms, treatment, and preventive measures for HIV, TB and STIs
- Be equipped with factual information on health risks to encourage informed decision making and health seeking behaviour
- Be equipped to know how and where to get help or additional information on each of the areas

MODULE CONTENT

The module has 4 lessons which focus on 3 targeted areas:

- Overview of the First Things First programme
- HIV and AIDS
- TB
- Sexually Transmitted Infections (STIs)



LESSON ONE: OVERVIEW OF THE FIRST THING'S FIRST PROGRAMME

AIM: To give participants a clear understanding of the FTF program and to ascertain and integrate their expectations.

TIME: 30 minutes

LESSON OBJECTIVES:

On completion of this lesson, participants should demonstrate enhanced understanding of:

- The purpose and goals of the First Things First (FTF) programme
- Why First Things First is necessary and important
- The roles and responsibilities of peer educators and peers in FTF

PLANNING NOTES

Prepare/review slides for:

- The purpose of First Things First, and the objectives of the training programme.

Key statistics that demonstrate the seriousness of the problems with HIV, STIs, TB, unplanned pregnancy, and non-communicable diseases.

PROCEDURE

1. Introduce yourself and your role in the project
2. Ask participants what they know about First Things First
3. Ask why FTF is necessary, and fill in gaps after all group input has been taken
4. Present slides on First Things First
5. Present slide on peer educator roles and responsibilities (R&R) and discuss peers' R&R in relation to these



LESSON TWO: HIV AND AIDS

AIM: To update participants' knowledge regarding all aspects relating to HIV and AIDS transmission, prevention, treatment including HIV counselling, and testing protocols.

TIME: 90 minutes (2 activities)

LESSON OBJECTIVES:

On completion of this lesson, participants should demonstrate enhanced understanding of:

- The difference between HIV and AIDS
- How treatment with antiretroviral therapy (ARVs) impacts on the progression of the disease
- Transmission of HIV, including Mother to Child Transmission (MTCT)
- Risks associated with HIV infection, and mitigating these risks
- Terminology and acronyms specific to HIV (e.g. window period, PICT, PEP, etc.)
- HIV Counselling and Testing policy guidelines
- The link between HIV management and human rights violations

PURPOSE OF ACTIVITIES

- To review baseline knowledge, create opportunities to clarify myths and misconceptions, and empower participants with knowledge and skills regarding HIV and AIDS.
- To illustrate how quickly HIV can be spread, and how correct, consistent condom use can stop its spread.
- To show the importance of knowing your HIV status, and empower participants to get tested.

ACTIVITY: THE SMARTEST VS. THE BRIGHTEST

OBJECTIVE: To assess baseline knowledge and create opportunities to address myths, misconceptions, and misinformation.

REQUIREMENTS: HIV and AIDS Q&A (Leaders' resource)
Flip chart, paper, markers, a bell (optional)

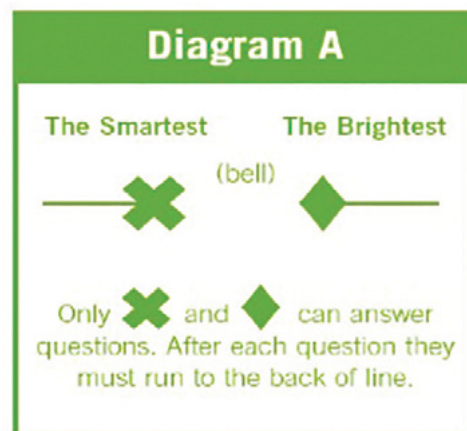
TIME: 60 minutes

PLANNING NOTES:

- Familiarise yourself with the content of the Fact Sheet: HIV/AIDS ([Leaders' resource](#)).
- Read through prepared questions and answers for the HIV and AIDS quiz ([Leaders' resource](#)).
- Review all updated policies around HIV (Treatment guidelines, HCT guidelines, etc.).
- Decide on a suitable 'reward' for winners.
- **Divide** a sheet of flip chart paper into two columns. Write 'The Smartest' as the heading for one column, and 'The Brightest' as the heading for the other.
- Familiarise yourself with the rules of the game, namely:
 - Only the team members at the front of the line can answer a question.
 - When each question is asked, the first person (of the two front lines) to raise his/her hand (or ring the bell) can answer.
 - If they answer correctly their team gets one point.
 - If they answer incorrectly their teams gets no points, but the opposing team gets a chance to answer the question **for one point, after which they still have their turn.**
 - If the opposing team also answers wrong, the facilitator reveals the correct answer, and encourages questions and discussion.
 - After each question has been answered, the team members at the front must run to the back so that two new teammates face off for the next question.

PROCEDURE

1. Introduce the lesson to participants and explain that the facilitation team will be conducting a fun quiz on HIV and AIDS.
2. Explain that they will be given clear instructions on how the game works, and that together they will refresh their knowledge regarding all aspects relating to HIV and AIDS transmission, prevention, treatment including HIV counselling, and testing protocols.
3. Explain the rules of the game (see above in planning notes).
4. Take time to clarify any questions from participants.
5. The group is randomly divided into two teams, 'The Smartest' and 'The Brightest', for a showdown on who knows the most about HIV and AIDS.
6. Line teams up as shown in diagram A.
7. Participants at the front of each line should face each other (see diagram A).
8. Conduct activity: QUIZ on HIV.
9. Start game with one facilitator asking questions (using the prepared HIV and AIDS Q&A), and the other taking note of each team's points.
10. After game calculate scores and announce a winner.
11. Present slides on current statistics, showing trends and patterns in respect to HIV and AIDS in South Africa.
12. Facilitate a plenary discussion using the discussion questions provided.
13. Present slides on HCT.
14. Present slides Health and Human Rights, and HIV and AIDS



DISCUSSION QUESTIONS

- Are there any further questions for clarity about the HIV or AIDS?
- Did new information change the way you think about the virus, people living with it, or their own responsibility in relation to HIV and AIDS?
- Did anything surprise you about how you experienced the activity?
- What stood out for you the most?
- What ideas do you have on how to keep yourselves refreshed in respect of information? (Reinforce the fact that information needs to be constantly updated, as policies, protocols and legislation are constantly changing.)
- What ideas do you have to 'keep it fresh' for peers? (Discuss resistance to attending workshop/education sessions on HIV because people are "tired of hearing about the same old thing...".)

KEY LEARNINGS:

- HIV is preventable through avoiding contact with other people's body fluids, such as blood and sexual fluids.
- If a person is infected with HIV, there is a 4-12 week period where they cannot know that they have the virus (the window period), but the virus is very active and can easily pass to other people at this time.
- It is important to know your HIV status so that you can protect your own health and the health of your partners.
- HIV and AIDS can be managed and treated so that people with HIV can live happy, healthy, and productive lives.

HIV counselling and testing (HCT) is now referred to as HIV testing services (HTS), to embrace the full range of services that should be provided together with HIV testing. These services include:

- Counselling (pre-test information and post-test counselling)
- Linkage to appropriate HIV prevention, treatment and care services, and other clinical and support services
- Coordination with laboratory services to support quality assurance and the delivery of correct results

The South African government has embarked on a deliberate effort to scale up HTS, and strengthen its quality at all health facilities and non-health sites. With increasing availability of quality HTS and its uptake in all public health facilities in South Africa, the proportion of

people who have ever had an HIV test, and are aware of their status, has increased from 50% in 2008 to 66.5% in 2014. In addition, 92.3% of South Africans are aware of HTS services, and 66.2% had actually utilised them in the past year, according to 2014 data.

There is a global initiative to accelerate universal access to HIV prevention, treatment, as well as care and support services for people living with HIV and AIDS (PLWHA). The main entry point for the HIV continuum of care is through HTS, which has become increasingly available. South Africa has more than 4 000 public health facilities offering provider-initiated counselling and testing (PICT), and client-initiated counselling and testing (CICT). In addition, HTS is also available through non-medical sites as well as the private sector.

South Africa has adopted UNAIDS' 90–90–90 strategy, which calls for 90% of all people living with HIV to be diagnosed, 90% of eligible people with diagnosed HIV to receive ARTs, and 90% of those on ARTs to have a suppressed viral load by 2020.

The 5 Cs

The 5 Cs are the foundation of effective HTS. Consent, Confidentiality, Counselling, Correct test results, and Connection are the 5 Cs and are described below.

- **Consent:** People who receive testing must consent to be tested and counselled. Clients or patients must be informed of the process for HTS, and of their right to decline testing.
- **Confidentiality:** Discussions between the HTS provider and the client should not be disclosed to anyone without the expressed consent of the person being tested. Shared confidentiality with a partner, family members, or trusted others must be encouraged.
- **Counselling:** Pre-test information can be done in a group setting, but a private setting must be provided for individuals who have questions that they do not wish to share with others. HIV testing must be followed by appropriate, high quality post-test counselling.
- **Correct:** Quality assurance (QA) mechanisms are essential to ensure that people receive a correct diagnosis.
- **Connection:** Linkage to prevention, treatment and care services, and effective and appropriate follow-up should be provided.

Higher education-based HTS will be offered continually to all young people attending higher education institutions, as well as to the staff at these institutions. As this is a high risk group, HTS providers should ensure that as many young people as possible are voluntarily tested. These are aligned to the National HTS Programme. Outreach services should also target higher education institutions, and all HIV testing conducted in these settings shall be reported to the local health office.

ACTIVITY: HOW NICE TO MEET YOU

REQUIREMENTS:

- Small pieces of paper, one for each participant
- Flip chart paper
- Markers
- Seats in a circle
- Condoms
- Dildo, or other penis substitute
- Female condoms

TIME: 60 minutes

PLANNING NOTES:

- **Prepare** small folded pieces of paper with 'N' (for no precautions) written on it for each participant, except 5
- **Write 'X'** (for HIV-positive) on two pieces, 'C' (for correct and consistent condom use) on another two, and 'FC' (for correct and consistent use of the female condom) on the final piece of paper
- **Ensure** that you are familiar with how to facilitate the activity

PROCEDURE

1. **Arrange the** seats in a circle, and place flip chart paper where it will be seen by everyone in the circle.
2. **Hand out** a folded piece of paper to each participant, and ask them not to show the contents to anyone until the end of the game.
3. **Instruct** them to walk around within the circle and shake hands with three people. They have the right to choose whom they prefer to shake hands with, but must shake hands with three people before they sit down again.
4. **Explain** that they have just had sex with each of the individuals whose hands they shook.

5. **Ask those with 'X'** on their paper to stand. Explain that these two individuals are HIV-positive, and they did not know it.
6. **Ask everyone who shook hands with them** to stand up. Explain that all those now standing may be HIV-positive, and not know it.
7. **Ask if any of those standing have a 'C' on their paper.** If they do, explain that they can sit, because they used a condom correctly, every time they had sex.
8. **Ask if any of those standing have 'FC' on their paper.** If they do, explain that they can also sit, because they used a condom correctly, every time they had sex.
9. **Ask** those still standing, if one of the two holding 'X' was the first person they shook hands with. When some indicate yes, explain that it only takes one sexual encounter to become infected with HIV.
10. By now, most of the group should be standing. **Explain** that they may all be HIV-positive. After allowing a few moments for the impact to settle in, tell everyone to sit, and lead a discussion using the **discussion questions** provided.

DISCUSSION QUESTIONS

- How did it feel to be HIV-positive, or negative?
- Did anything surprise you about the game?
- What stood out for you the most?
- How do you feel about testing? (Reinforce that it only takes one sexual encounter to become infected.)
- How can we motivate our peers to test? (Discuss resistance to getting tested.)
- Where can you get tested for HIV? (Refer to the Directory of services to ensure that they know where he or she can be tested for HIV.)



KEY LEARNINGS:

- A negative test can put your mind at rest.
- If the result is HIV-positive, knowing can help you to be proactive and take better care of your health.
- A doctor can keep an eye on your health, and Antiretroviral drugs can slow down the virus and maintain your health.
- You can practice safer sex to protect your sexual partners from contracting the virus, and prevent yourself from re-infection, which makes treatment more complicated, and sometimes ineffective.
- If you choose to have children, knowing your HIV status will help you protect your children from HIV infection.

HIV TESTING AND HUMAN RIGHTS

A human rights-based approach to HTS ensures that the essential elements of the programme are aimed towards realisation of rights, and that those rights are used as standards. HTS must be offered in a way that is consistent with the rights described in the Constitution of South Africa and in the Children's Act. It must be ethical, and be conducted within a supportive environment.

SOME IMPORTANT HUMAN RIGHTS ARE DESCRIBED BELOW:

- **Right to dignity and non-discrimination:**

Every person has inherent dignity, and the right to have their dignity respected and protected. No actions should be taken against any individual solely on the basis of their HIV status, as this will constitute stigma and discrimination.

- **Right to privacy and confidentiality:**

All personal information concerning a client; his or her health status, treatment, or stay in a health establishment must be kept confidential, unless ordered by the court of law or done so for the advancement of the client's care and treatment after following the necessary procedure.

- **Right to refuse HIV testing:**

Clients have the right to refuse HIV testing, without compromising their access to standard health care. There shall be no mandatory HIV testing and all testing shall remain voluntary with informed consent, even when the services are initiated by the service provider.

- **Informed**

consent:

Informed consent refers to a person being given relevant and appropriate information about an HIV test, and based on that information, given an opportunity to either accept or refuse to do the HIV test. Informed consent should always be in written form and signed by only the client and the health care provider, to avoid unintended disclosure of results.

- **Requirements of informed consent:**

The information that clients and patients require, in order to give their informed consent, may vary based on the service delivery approach and setting, but should generally include information about:

- Benefits and implications of knowing one's status, and reasons for recommending HTS
- Client's right to withdraw consent at any stage of the process
- Availability of follow-up treatment, care and support, and prevention services
- Importance of disclosure and partner/family testing and availability of couple HTS
- HTS process and procedure

WHAT IS HCT?

HCT refers to HIV counselling and testing. HCT services are offered at all public health facilities, as well as many community-based organisations and Non-Governmental Organisations, private health facilities, educational institutions and workplaces.

WHAT ARE IMPORTANT GUIDING PRINCIPLES FOR HCT?

HCT clients must be given enough information about HIV to help them understand what HIV is, how it is transmitted and not transmitted, risks, rights and responsibilities, different types of tests and what the results mean, explanation of the window period, etc. Once they are informed, they're in a position to give or withhold informed consent.

Informed consent must be **given voluntarily** to enable clients to receive services. Clients must not be forced to test, neither can a test be done without a client's informed consent. Informed consent must be explained in the **language appropriate to the client** and recorded in the client's file.

HCT services should be **confidential**, meaning that anything discussed between the client and the HCT provider may not be shared with other persons; except in cases where the results need to be shared for client's medical benefit or when ordered by the court or law. This is called **shared confidentiality**. Even in such cases, the clients should be informed that their HIV results will be shared.

HCT services must include **accurate counselling** that addresses the needs and risks of HCT clients. Counselling must be provided before the test (pre-test counselling) and after the test (post-test counselling).

- HCT services must be **in line with national quality assurance guidelines** for testing, to make sure that accurate and correct test results are provided.
-
- Once an individual or couple has tested, they must be **referred for follow up** treatment, care and support, and management. Referrals may include:
 - Health facility for ongoing care and treatment for HIV-positive clients and patients
 - PMTCT for pregnant women
 - Male circumcision for HIV-negative men
 - Support groups for discordant couples
 - TB and STI screening and treatment
 - Family Planning

DIFFERENT TYPES OF HCT APPROACHES

PROVIDER-INITIATED COUNSELLING AND TESTING (PICT)

- PICT refers to HIV counselling and testing which is offered by health care providers to persons attending health care facilities (clinics, hospitals etc.) every time they attend, as a routine part of the care offered by the facility.
- Provider-initiated HIV counselling and testing (PICT) should be offered to all persons attending clinical services, in both public and private sector.
- Health care providers should recommend HCT to all patients, regardless of whether they show signs or symptoms of HIV infection.
- PICT contributes to increased rates of HIV testing and early identification of HIV-infected persons who may not otherwise know their HIV status.

CLIENT-INITIATED COUNSELLING AND TESTING (CICT)

- Client-initiated counselling and testing, also referred to as VCT, are HCT services provided for clients who seek out HCT services.
- Clients may voluntarily make the decision to learn their HIV status as an individual, couple, or family.
- CICT is conducted in a wide variety of settings, including health facilities, outside health institutions, in community-based settings through mobile vans, stand alone, and even in people's homes.

WHEN IS RE-TESTING NECESSARY?

Re-testing is recommended for persons with an HIV-negative test result.

Window period repeat test:

- All individuals should test for HIV; after the first initial HIV-negative test, a repeat window period test should be conducted at 6-12 weeks. This is because the body takes 4-6 weeks, and in some cases 3-6 months, to produce antibodies for HIV.

Recommendations for Retesting (HCT guidelines):

- Clients with low risk and no exposure should retest once every year.
- Clients at high risk should retest every 6 to 12 weeks.
- Pregnant women who have tested HIV-negative in their initial antenatal clinic visit, should retest on every ANC visit thereafter.
- Clients who have an STI to re-test in 6 weeks.
- Clients presenting with opportunistic infections (OIs) to re-test in 6 weeks.
- Clients with continuing or ongoing risk of acquiring HIV; re-test in 6 weeks.
- Clients who report HIV exposure within the past three months; to re-test in 6 weeks.
- Those who received an HIV-negative test result on a baseline HIV test for an incident of possible HIV exposure (occupational or rape) in the past 72 hours. In this case, and if PEP has not been initiated, re-test at 6 weeks after exposure, and if the results are still negative, the person should be re-tested again at 12 weeks after exposure.

WHY IS IT IMPORTANT TO TEST?

- To know your status
- To be empowered with knowledge and information about HIV
- To understand more about transmission and prevention
- To be aware of lifestyle risk and risk management
- To be able to receive treatment, care, and support if needed

HIV AND AIDS: HEALTH AND HUMAN RIGHTS

There is a clear link between human rights and the spread and impact of HIV on individuals and communities around the world. The disregard for human rights contributes to the spread of the disease. This can be seen in the differences in incidence and spread of HIV amongst certain groups which include women and children; those who are poverty stricken, the LGBTI (Lesbian, Gay, Bisexual, Transgender and Intersex) community, injecting drug users, sex workers, and other stigmatised or disempowered groups.

The relationship between HIV/AIDS and human rights is highlighted in three areas

Increased vulnerability: Groups that are unable to realise their social, cultural, economic, and political rights are more vulnerable to contracting the HIV virus. For example, individuals who are denied the right to freedom of association and access to information may not be able to freely discuss issues related to HIV, disclose their status, get treatment, join support groups, or take preventive measures to protect themselves from HIV infection.

Discrimination and stigma: Stigma and discrimination may obstruct access to treatment, and may prevent individuals from seeking treatment, care, and support which in turn, contributes to the vulnerability of others to infection. The result is that those most needing information, education, and counselling will not benefit, even where such services are available.

Prevents an effective response: Strategies to address the epidemic are hampered in an environment where human rights are not respected. For example, discrimination against and stigmatisation drives the affected groups underground. This means that prevention efforts do not reach these groups, and thus increases their vulnerability to HIV. Likewise, the failure to provide access to education and information about HIV, or treatment, care, and support services further fuels the AIDS epidemic. These elements are essential components of an effective response to AIDS, which is hampered if these rights are not respected.

What is a human rights approach to HIV and AIDS?

- The right to education, free association, information and, most importantly, non-discrimination is recognised and upheld
- An open and supportive environment exists for those infected with HIV:
 - The infected and affected are protected from discrimination, treated with dignity, and provided with access to treatment, care, and support
 - HIV and AIDS is de-stigmatized

In these circumstances, individuals are more likely to seek testing in order to know their status. In turn, those people who are HIV-positive may deal with their status more effectively, by seeking and receiving treatment and psychosocial support, and by taking measures to prevent transmission to others, thus reducing the impact of HIV on themselves and on others in society.

NOTES

LESSON THREE: SEXUALLY TRANSMITTED INFECTIONS

AIM: The aim of this lesson is to improve participants' knowledge of sexually transmitted diseases, to dispel myths, and to provide a framework for effectively communicating with others on the subject.

TIME: 60 minutes

LESSON OBJECTIVES:

On completion of this lesson, participants should demonstrate enhanced understanding of:

- What STIs are
- How STIs are spread
- Symptoms, diagnosis and treatment of STIs
- How to prevent contracting and/spreading STIs
- Myths and misconceptions related to STIs
- Risks and vulnerabilities to STIs
- Rights and responsibilities regarding STIs
- Where to go for treatment

PURPOSE OF ACTIVITIES

- To provide participants with the opportunity to demonstrate what they know about STIs
- To assess the level of baseline knowledge of participants, and fill in the gaps about STIs, how they are contracted and how they affect health

To provide an interesting, informal and safe space for discussion and debate on STIs, a generally sensitive and uncomfortable topic to discuss.

ACTIVITY: STI GAME – IT'S ALL ABOUT THE BASE

REQUIREMENTS: Leader's Resource - STI Facts: True or False; container; prizes (optional); STI pamphlets

TIME: 45 minutes

PLANNING NOTES:

- Invite a practitioner from a local STI or health clinic to co-lead this session with you, and to provide an opportunity for participants to become familiar with information about the clinic. Knowing a friendly face at a local clinic may make it easier for them to go there when they need health services.
- Read Leader's Resource: STI Facts: True or False
- Cut out the statements only (they are the sentences in boldface). Then cut the statements into strips so that there is one statement per strip.
- Fold the strips and place them in a container for steps 5, 6, and 7.

Obtain pamphlets on STIs from a local health department or a family planning clinic. Display the pamphlets and information from the clinic in a prominent area of the room.

PROCEDURE

1. Introduce the lesson and explain that it is important for participants to know the risks of sexually transmitted infections, how they're transmitted, and what to do to reduce that risk.
2. Acknowledge that participants may already have knowledge and access to information about STIs.
3. Explain that it is for this very reason that they will be given the opportunity to demonstrate what they know, as well as learn some new information about STIs.
4. Divide the group into four teams and have each team move to different corners of the room.
5. Tell participants that their team will play against the others in a game, and the team with the most points wins.

6. Go over instructions for the game:
 - Each team will draw a statement about STIs. The team must consider the statement and decide whether the statement is **true or false**.
 - If their answer is correct, the team scores **two points**. If the team can also explain why the answer is correct, they get an **extra point**.
 - If the team cannot correctly explain the answer, another team can try for the extra point.
 7. Have someone from the first team draw a statement and confer with his/her team members to decide whether it is true or false. Ask the team member that drew the statement to declare whether the team has agreed that the statement is either true or false.
 8. Ask the team to explain the statement and award an extra point if the explanation is correct. If not, allow any other team to try for the extra point.
 9. As the game progresses, use the Fact Sheet (Leader's Resource) and the content section in this lesson to provide additional information about the statements
 10. Conclude the activity using the **discussion questions** provided for extra points.
 11. Add up points and announce first, second, third, and fourth places.
-

DISCUSSION QUESTIONS: RAPID FIRE

IN 30 SECS THE FIRST GROUP TO RING THE BELL CAN ANSWER

- What are the signs and symptoms of STIs?
[Answers include: redness or soreness of the genitals, pain when urinating (mostly among males); strong-smelling or cloudy urine; unusual discharge from the penis or vagina; sores or blisters on or around the genitals, mouth, or anus; a sexual partner with symptoms.]
 - What are the most effective ways to avoid STIs?
[Answers include: 1) abstaining from vaginal, anal, and oral intercourse and 2) using latex condoms every time you have any kind of sexual intercourse.]
 - What three things should you do if you are worried that you have been infected with an STI?
[Answers include: 1) seek medical testing and treatment right away, 2) inform your sexual partner(s), and 3) abstain from sexual contact until there is no evidence of infection.]
 - Men who have sex with men can use condoms to protect themselves and their partners from most STIs. What can women who have sex with women use?
[Answer: barriers, such as squares of latex called dental dams, latex condoms which have been cut open, or plastic wrap to cover the vulva and form a barrier so body fluids cannot be exchanged.]
-

KEY LEARNINGS

- STI's are infections that are spread from person to person through intimate sexual contact.
- They can be dangerous and easily spread, and it is hard to tell just by looking if somebody has an STI.
- There are **three different types** of STIs: those caused by a virus, those caused by bacteria, and those caused by a fungus.
- If an STI is left untreated, it can result in serious long-term health problems such as infertility, cancer, long-term pain, infection of a child from mother, and death.
- Viral STIs are systemic and invade the whole body and its cells. They are treatable, but not curable and include Hepatitis B, HIV, Genital Herpes (HSV-2) and Genital Warts (HPV).
- Bacterial STIs are localised to one part of the body and are curable by antibiotics, and include diseases such as Chlamydia, Gonorrhoea and Syphilis.
- Fungal infections commonly grow on or in the top layer of skin, are not always sexually transmitted, and are curable.
- Other infections of the genital areas, which can be cured with antibiotics or topical creams, include Pubic Lice, Trichomoniasis and Thrush (Candida), and other inflammations of the vagina caused by organisms such as bacteria or yeast or irritations from chemicals in creams, sprays, or clothing.

TESTING AND TREATMENT FOR STIS

- If you have symptoms, you should go to your health care provider for a check-up.
- Testing for STIs includes taking a urine sample, taking a swab of the vagina or penis for secretions, examining sores or bumps on the genitals, or doing a blood test.
- Managing an STI includes the following steps:
 - o Counselling for the individual, so they understand the STI, its spread and treatment

- o Condom use for any future sexual encounters, to prevent the spread of the STI
- o Compliance with treatment that is given to you at the clinic
- o Contacting previous sexual partners to advise them to get tested – it is their right to know as they have the responsibility to treat it and not spread it

LESSON FOUR: TUBERCULOSIS (TB)

AIM: The aim of this lesson is to increase awareness of Tuberculosis (TB) which is increasingly becoming a risk for all South Africans, irrespective of race, gender or age.

TIME: 60 minutes (1 activity)

LESSON OBJECTIVES:

On completion of this lesson, participants should demonstrate enhanced understanding of:

- What TB is and what causes TB
- How to prevent TB
- How to identify symptoms of TB
- How to access testing and treatment
- How TB is treated, and where to go for treatment



PURPOSE OF ACTIVITIES

- To provide participants with the opportunity to demonstrate what they know about TB
- To assess the level of baseline knowledge of participants and fill in the gaps about TB
- To provide a non-threatening way to learn
- To demonstrate interesting, informal, and safe ways for discussion and debate on TB

ACTIVITY: ASK YOUR HEALTH WORKER

REQUIREMENTS: Leader's Resource – TB Q&A

TIME: 45 minutes

PLANNING NOTES:

- Invite a practitioner from a local TB or health clinic to co-lead this session with you, and to provide an opportunity for participants to become familiar with information about the clinic. Knowing a friendly face at a local clinic may make it easier for them to go there when they need health services.
- Read Leader's Resource: TB Q&A
- Cut out the questions only.
- Fold the strips and place them in a container.

Obtain pamphlets on TB from a local health department or a family planning clinic. Display the pamphlets and information from the clinic in a prominent area of the room.

PROCEDURE

1. Introduce the lesson, and explain that it is important for participants to know the risks of being infected with TB, how TB is transmitted, and what to do if you have symptoms of TB.
2. Acknowledge that participants may already have knowledge and access to information about TB.
3. Explain that it is for this very reason that they will be given the opportunity to demonstrate what they know, as well as learn some new information about TB.
4. Divide the group into two and have the first group carry their chairs, and sit in a circle.
5. The second group should sit in a circle around the first group.
6. Place one chair in the middle of the inner circle.

7. The inner circle are 'health workers' and the outer circle are 'community members visiting the clinic'.
8. Members of the outer circle will be given pieces of paper with questions or concerns relating to TB.
9. Members of the outer circle will have turns to go into the centre and choose a 'health worker' to address their concern about TB.
10. If the chosen health worker cannot answer their concern, they can discuss with other health workers to provide an answer.
11. Conclude the activity using the **discussion questions** provided.

DISCUSSION QUESTIONS:

- What is the difference between MDR and XDR TB?
- What is DOTS and why do we use this strategy?
- What is the link between HIV and TB?

KEY LEARNINGS:

- TB is a preventable and curable disease.
- South Africa is estimated by the World Health Organization (WHO) to have the world's third largest TB burden, and an exceptionally high rate of multi-drug resistant (MDR) and extensively drug-resistant (XDR) TB cases.
- Drug resistant strains are expensive and difficult to treat, and pose a greater risk of infection to people in close contact with the infected person. These strains more often can lead to treatment failure and death.
- TB cases become almost five times more severe than it should be, due to HIV and AIDS.
- Antiretroviral Drugs (ARVs) do not cure TB and TB drugs do not cure HIV.
- This is why it is very important that people should know their HIV status, and get tested for TB and HIV regularly.

PULMONARY TUBERCULOSIS

What is TB?

Tuberculosis (TB) is caused by bacteria (*Mycobacterium tuberculosis*) that most often affects the lungs. Tuberculosis is curable and preventable.



Who Can Get TB?

- Tuberculosis mostly affects young adults in their most productive years. However all age groups are at risk. Over 99.5% of cases and deaths are in developing countries.
- People who are infected with HIV are 26 to 31 times more likely to become sick with TB. Risk of active TB is also greater in persons suffering from conditions that impair the immune system.
- Over half a million children (0 –14 years old) fell ill with TB and 60 000 HIV-negative children died from the disease in 2013.
- Tobacco use greatly increases the risk of contracting and dying from TB. More than 20% of TB cases worldwide are attributed to smoking.



What are the signs of TB?

- A cough that lasts for more than two weeks
- Fever
- Night sweats
- Weight loss
- Chest pain
- Weakness
- Symptoms may be mild for many months. This can lead to delays in seeking care, and result in transmissions of the bacteria to others.
- People with TB can infect up to 10 – 15 other people through close contact over the course of a year.

Without proper treatment up to two thirds of people with TB will die.

Treatment for TB

- **TB is a treatable and curable disease.**
- Active, drug-sensitive TB is treated with a standard six-month course of four antimicrobial drugs that are provided with information, supervision and support to the patient by a health worker or trained volunteer. Without such supervision and support, treatment adherence can be difficult and the disease can spread. The vast majority of TB cases can be cured when medicines are provided and taken properly.
- Between 2000 and 2013, an estimated 37 million lives were saved through TB diagnosis and treatment.



How is TB Spread?

TB is spread from person to person through the air. When people with TB cough, sneeze or spit, they project the TB germs into the air. A person only needs to inhale a few of these germs to become infected.



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MODULE TWO: WOMEN'S HEALTH AND EMPOWERMENT



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ABBREVIATIONS

AIDS:	Acquired Immune Deficiency Syndrome	MTCT:	Mother to Child Transmission
ANC:	Anti-natal Care	NCD:	Non-Communicable disease
ARVs:	Antiretrovirals	PEP:	Post Exposure Prophylaxis
CEDAW:	Convention on the Elimination of all forms of Discrimination Against Women	STIs:	Sexual Transmitted Infections
GI:	Gastrointestinal cancer	STDs:	Sexual Transmitted Diseases
HEAIDS:	Higher Education and Training HIV/AIDS Programme	TB:	Tuberculosis
HIV:	Human Immuno-Deficiency Virus	TVETs:	Technical and Vocational Education and Training Colleges
HPV:	Human Papillomavirus Infection	UDHR:	Universal Declaration of Human Rights
IUD:	Intrauterine Device	UNAIDS:	United Nations Programme on HIV/AIDS
IUS:	Intrauterine System	WHO:	World Health Organization

CRUCIAL CONVERSATIONS



Hey girlfriend!
Whatsup?

My mum's driving me crazy! She's nagging me to go to the clinic to see a 'woman' doctor. She says I need to take care of my health as a woman. I was like "mom, you know I'm young, fit and healthy right? You know it's more important for you to take care of YOUR health right?"



Hey your mum's right you know, we have to take care of ourselves.

Why, what's so special about women's health?



Remember the saying "prevention is better than cure?" Getting checked early can help you stop diseases like cancer, diabetes and osteoporosis in the very beginning, when they're easier to treat. Screening tests can spot illnesses even before you have symptoms.

How do you know so much about this?



Well last year I went to the First Things First activation, remember? Well that's where I got some hectic information...gave me a wakeup call. There is one next week – you should go!

AFTER SBU ATTENDED THE FIRST THINGS FIRST ACTIVATION



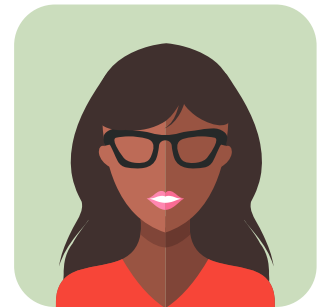
Yo! You should have come!



Where?



To the activation thingy YOU told ME about.



But I went last year. I have sooooo much work. I don't have the time to go to the same things over and over.



But it wasn't the same hey. It was cool, but yoh, it was scary. I feel like there's goggas inside my body and I don't know about them (shudders). What if there's germ thingys eating up all my good cells?



OK, OK, cool your jets now. What did they say to make YOU worried? You don't care about anything!



Well, they like showed us a video about all the screening tests and told us about symptoms to look out for. I feel like I have all the symptoms they talked about. It's crazy man – I didn't sleep last night. I'm only 19...I have a *ish load of living to do.



OK, you got me now – so spill. What did they say?



I brought you the handout. Check it out!



AIM

The Women's Health and Empowerment module aims to encourage women to utilise sexual and reproductive health services, empower young women to protect themselves from infection and gender based violence, and motivate them to maintain optimal levels of health and well-being.

OVERALL OBJECTIVES OF THE MODULE

By the end of this module, participants will:

- Know the top ten women's health issues
- Understand why screening tests are important
- Be familiar with various types of cancers faced specifically by women
- Be informed of all relevant issues regarding family planning, including the benefits of family planning vs. consequences of unplanned pregnancies
- Have explored various issues and scenarios around women's health, empowerment, risk, and responsibility
- Understand the dynamics of gender based violence
- Know the link between personality and power in relationships
- Know how human rights are linked to women's rights and women's health
- Be equipped to facilitate the programme amongst peers as well as target groups of young women
- Be able to provide comprehensive, relevant, and up to date information, knowledge, and skills to empower others to improve informed decision making on a range of issues facing young women

MODULE CONTENT

The module has 4 lessons which focus on 3 targeted areas:

- Overview of Women's Health, screening tests for women and specifically women's cancers
- Family Planning
- Gender based violence
- Personality and power

NOTES

LESSON ONE: WHAT'S SO SPECIAL ABOUT WOMEN'S HEALTH?

AIM: To review the top ten issues related to Women's Health, including regular screening and the various cancers that women are at risk for.

TIME: 120 minutes

LESSON OBJECTIVES:

On completion of this lesson, participants should:

- Know the top ten health issues for women as identified by the World Health Organisation (WHO)
- Be familiar with screening tests available for both communicable (diseases that can be passed from one to another) and non-communicable diseases
- Be familiar with the various cancers that women are susceptible to

PURPOSE OF ACTIVITIES:

- To provide information on relevant issues pertaining to women's health
- To provide opportunities for participants to demonstrate existing knowledge
- To use methods that are interesting and have high impact and enable participants to make associations between situations that are familiar and commonly occur amongst the target group and risks and consequences in terms of women's health

ACTIVITY: BUZZ GROUPS / RAPID FIRE QUIZ

OBJECTIVE: To create opportunities for participants to become informed of key issues related to women's health.

REQUIREMENTS: Crucial conversation script, Flip chart paper, markers, slides

TIME: 60 minutes

PLANNING NOTES

Prepare/review slides for:

- The top ten women's health issues
- Screening tests that women should do
- Cancers and symptoms that women should not ignore

PROCEDURE

1. Invite 2 participants to read the 'crucial conversation' between Sbu and Boni.
2. Introduce the subject of 'women's health' and ask, "So what's so special about women's health?"
3. Note all points on the flipchart.
4. Present slides, and facilitate discussion on the top ten women's health issues as identified by the World Health Organisation.
5. Facilitate a buzz group on the various health issues that women need to screen for, and the screening tests for each.
6. Present slides and be sure to only fill in gaps.
7. Ask participants to read 'symptoms women ignore' and myths and facts for 5 minutes, in preparation for a rapid fire quiz.
8. After 5 minutes facilitate a rapid fire quiz, and ask those who answer correctly to stand.
9. Inform them that they are in line for an award at the end of training.
10. Facilitate the popcorn exercise i.e. people to randomly 'pop up' and say one thing they learnt from the lesson.
11. Summarise the lesson using the **discussion questions** provided.

DISCUSSION QUESTIONS:

- Ask participants if they have any further questions about women's health?
- Did new information change the way they think about women's health?
- Did they learn anything new?
- How did the information impact on them?

WOMEN'S HEALTH ISSUES

WHAT'S SO SPECIAL ABOUT WOMEN'S HEALTH?

According to the World Health Organization (Dr. Flavia Bustreo, Assistant Director General for Family, Women's and Children's Health), even though the world has made so much progress towards recognising women's rights, "women still face many health problems". Dr. Bustreo pointed out the following issues as being amongst the top ten women's health issues:

CANCER: Two of the most common cancers that affect women are breast and cervical cancers. She says that every year around half a million women die from cervical cancer and half a million from breast cancer. However, if you find out early, it can be treated and you can prevent it from spreading. Most importantly, you can stay alive!

REPRODUCTIVE HEALTH: "In 2015, in too many countries, 'women's empowerment' remains a pipedream ..." says **Dr Flavia Bustreo**.

That means that women are still struggling, and in many places have no rights.

Sexual and reproductive health problems make up one third of health issues for women between the ages of 15 and 44 years (WHO 2015). This is because most women still do not have enough power to negotiate safer sex. This is especially so in poorer countries, like many parts of South Africa, where poverty causes women to be dependent on men to provide food, shelter, and support for their children.

HIV: Almost 30 years into the AIDS epidemic, it is young women who bear the brunt of new HIV infections. Too many young women still struggle to protect themselves against sexual transmission of HIV and to get the treatment they require. This also means they can more easily contract Tuberculosis, one of the leading causes of death in low-income countries of women 20–59 years.



Women are more than half of all people living with HIV/AIDS worldwide.



Worldwide, HIV/AIDS is the number 1 cause of death for women of reproductive age.



HIV/AIDS was responsible for 60,000 maternal deaths worldwide in 2008.



In Sub-Saharan Africa:

- 60% of the infected population are women.
- 72% of infections among young people age 15-24 are women.
- Among 15-24 year olds, young women are eight times as likely to become infected as young men.



It's time for a woman-centered approach to HIV/AIDS. Take action at www.genderhealth.org

OCTOBER 2012 — PHOTO FOR ILLUSTRATIVE PURPOSES ONLY.

SEXUALLY TRANSMITTED INFECTIONS: There is a lot of education and information about the importance of protecting against HIV and Human Papillomavirus (HPV) infection (the world's most common STI). But it is also very important to do a better job of preventing and treating diseases like gonorrhoea, chlamydia and syphilis. These are all STIs. There are more than 200,000 stillbirths and babies dying in the womb every year, as well as deaths of over 90 000 new-borns due to untreated syphilis.

VIOLENCE AGAINST WOMEN: Women are typically not as physically strong as men. Therefore, men whose male hormones can make them more aggressive, often get their way using different forms of violence, both physical and sexual violence. Today, one in three women under 50 has experienced physical and/or sexual violence, which affects their physical and mental health in the short and long-term. It is important to be aware and alert to violence, so we can help prevent it, as well as provide support to people who experience it.

MENTAL HEALTH: Because of the various problems that many women around the world experience, especially women who have no power, there is evidence that women are more likely than men to experience anxiety and depression. Therefore, suicide is a leading cause of death for women under 60. Helping to make them aware of mental health issues, and giving them the confidence to get help is vital.



MATERNAL HEALTH: In 2013, almost 300 000 women around the world died from complications during pregnancy and childbirth. Most of these deaths could have been prevented, if these women had received family planning services and some basic care during pregnancy and childbirth.



BEING YOUNG: Young women face a number of sexual and reproductive health challenges; STIs, HIV, and pregnancy. About 13 million girls (under 20) give birth every year. Complications from those pregnancies and childbirth are a leading cause of death for those young mothers. Many die because of attempts at unsafe or backstreet abortions.

GETTING OLDER: Women who have been stay-at-home mothers, as they reach old age, may have fewer pensions and benefits, less access to health care, and social services than their male partners. Together, with the greater risk of poverty and other conditions of old age, older women also have a higher risk of abuse and generally poor health. Therefore, it is important for young women to have a good start in life. Even though use of services, especially those for sexual and reproductive health, has increased in some countries, we are not there yet. Too many women are still missing out on the opportunity to get educated, support themselves, and obtain the health services they need, when they need them.



NONCOMMUNICABLE DISEASES:

A **non-communicable disease** (NCD) is a medical condition or **disease** that is **not** infectious, and therefore cannot be transmitted. NCDs can refer to chronic diseases which last for long periods of time and progress slowly. In 2012, the WHO recorded some 4.7 million deaths of women who died from non-communicable diseases before they reached the age of 70. Most of them were in low and middle income countries. Examples of NCDs are cancer, diabetes, high blood pressure, and obesity, more than 50% of women throughout the world are overweight. Helping girls and women adopt healthy lifestyles early on is key to a long and healthy life.

Young women in South Africa are at greater risk of being infected with HIV than young men.

A third of young women in South Africa have unplanned pregnancies before the age of 20.

Almost a quarter (24.1%) of all new HIV infections occurred in young females aged 15–24 years.

Females are more likely to have sex with older partners. Such relationships can also lead to disempowerment, sexual coercion and violence.

South African females have a one-in-eight chance of developing cancer. One in 29 women will develop breast cancer, and one in 41 women will develop cancer of the cervix.

7% of girls have their first sexual experience before they are 15 years old, 34% between 16–17 years, 32 % have sex for the first time when aged 18–19, and 21% of women aged between 20 and 24.

Research indicates that in comparison to boys and men, girls and women have a heightened physiological vulnerability to HIV through sexual intercourse (Yi, et al in 2013).

Survey (NCS) found that in 2011, 39% of sexually active girls aged 16–19 had had sexual partners five or more years older than themselves.

WHY ARE SCREENING TESTS IMPORTANT?

Screening means prevention. It's that simple. There are different types of prevention:

PRIMARY PREVENTION means that checking on your health regularly can help to prevent you from getting serious life threatening or less serious illnesses.

SECONDARY PREVENTION means that if you screen and find you've already been infected with some virus, or have developed a life threatening condition, you can take treatment and manage your lifestyle to prevent it from becoming more serious.

TERTIARY PREVENTION means that when you've caught it too late and it's already serious, you can take treatment to manage pain and side-effects.



What should your *wellness exam* include?
Use this list to check in with your provider:

- ☒ **Breast Cancer Screening**
Screenings help detect breast cancer in its earliest, most treatable stage. Visit plannedparenthood.org/breasthealth or cancer.org for more information on early detection.
- ☒ **Pelvic Exam**
Pelvic exams help protect against infertility and other health problems, detect signs of infection, and promote healthy pregnancies.
- ☒ **Pap Test and HPV Test**
These tests screen for warning signs of cervical cancer.
- ☒ **Pregnancy**
Thinking about getting pregnant at some point? Talk with your provider about pre-pregnancy health and planning for children.
- ☒ **Birth Control**
Birth control is not one-size-fits-all. Visit plannedparenthood.org/mymethod or bedsider.org before your appointment to get ready to talk with your provider about your options.
- ☒ **STDs**
Getting tested for infections protects your sexual health and the health of your partner(s). Visit plannedparenthood.org/thecheck to figure out if you should ask your provider for STD testing.
- ☒ **And more**
For information on women's bone health, heart health, diabetes, and colorectal health at every age, visit the interactive screening chart at womenshealth.gov/whw/.

PLANNED PARENTHOOD

WHAT CAN YOU SCREEN FOR?

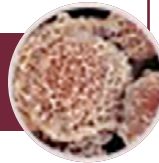
If you're a young woman in your 20s or 30s, you should go for a breast exam at least every 3 years. If you have risk factors, including a family history of cancer, you should go every year. The earlier you find breast cancer, the better your chance of a cure. Left untreated, the cancer will grow and spread to the lymph nodes and vital organs like the lungs and brain.

BREAST CANCER



If you have regular Pap smears, cervical cancer is easy to prevent. The cervix is a narrow passageway between the uterus (where a baby grows) and the vagina (the birth canal). A Pap smear can detect abnormal cells on the cervix, which can be removed before they ever turn into cancer. The main cause of cervical cancer is the Human Papillomavirus (HPV).

CERVICAL CANCER



Mammograms are x-rays that can often find a lump before you ever feel it. For older women, a mammogram is recommended more often every two years. If you have higher risk factors your doctor may recommend more screenings, more often.

MAMMOGRAMS



During a Pap smear, your doctor will scrape a sample of cells from your cervix and send them to a lab for analysis. It is strongly recommended that you should get your first Pap smear by age 21, and every two years after that. Once you are sexually active, you can get tested for HPV. Both types of tests are very effective in finding cervical cancer early enough to cure it.

PAP SMEAR



Blood pressure readings have two numbers. The first (systolic) is the pressure of your blood (how forcefully it pumps) when your heart beats. The second (diastolic) is the pressure between beats. Normal adult blood pressure is below 120/80. High blood pressure, also called hypertension, is 140/90 or above. This can cause life-threatening heart attacks or strokes without any warning.

HIGH BLOOD PRESSURE



Many people with diabetes don't know they have it. Diabetes can cause heart or kidney disease, stroke, blindness from damage to the blood vessels of the eye, and other serious problems. You can control diabetes with diet, exercise, weight loss, and medication, especially when you find it early. Type 2 diabetes is the most common form of the disease. Type 1 diabetes is usually found in children and young adults.

TYPE 2 DIABETES



High cholesterol can cause plaque (a fatty deposit) to clog your arteries. Plaque can build up for many years without symptoms, eventually causing a heart attack or stroke. Changing your lifestyle (making safer choices in terms of diet, exercise, and substance use) can lower your risk.

HIGH CHOLESTEROL



WOMEN'S CANCER: SYMPTOMS WOMEN IGNORE

NO.1 - UNEXPLAINED WEIGHT LOSS

Many of us would be pleased to lose weight without trying, but when a woman loses weight without diet or exercise this should be checked out.

NO. 2 - BLOATING

Many women experience bloating as a normal part of their monthly cycle. But if bloating includes every day and lasts for several weeks, you need to see a doctor. Signs of ovarian cancer include bloating and other digestive issues, abdominal or pelvic pain, feeling full quickly (even if you have not eaten much), and needing to pass urine urgently.

NO 3. - BREAST CHANGES

If there's redness and thickening of the skin on the breast, a rash that stays for a long time, changes in the nipple, or discharge when you are not breastfeeding, it is important to go to the doctor or clinic for a breast exam.

NO. 4 - BETWEEN-PERIOD BLEEDING OR OTHER UNUSUAL BLEEDING

If you normally have regular periods, bleeding in between periods needs to be checked out. An early sign of endometrial cancer is often spotting between periods.

NO. 5 - SKIN CHANGES

Moles that are changing, irregular in shape or colour, are common signs of skin cancer. But other skin changes can be signs too, including changes in skin pigmentation, bleeding, and peeling or scaling skin. Skin cancer can be aggressive, so it is important to act quickly after noticing changes.

NO. 6 - DIFFICULTY SWALLOWING

Difficulty swallowing may be a symptom of a gastrointestinal (GI) cancer, such as cancer of the oesophagus. It's important to go to a doctor or clinic about your symptoms.

NO. 7 - ABDOMINAL PAIN AND DEPRESSION

When depression is coupled with pain in the abdomen, it may be a symptom of cancer. It may sound strange but cancer of the pancreas causes a change in mood as well as pain in the abdomen. Therefore, it cannot be ignored.

NO. 8 - PAIN

Any unexplained pain may be a sign of cancer. Most of the time it is not, but pain that does not go away and has no known cause should be checked by a doctor.

NO. 9 - CHANGES IN LYMPH NODES

Enlarged lumps under your armpit, in your neck, or groin area may be a sign of possible cancer. If the lump is growing bigger and has been present for over a month, it needs to be investigated. It may be due to an infection, but it could be a sign of something more serious.

NO. 10 - FATIGUE

Tiredness (especially in young people) is a symptom of many illnesses and medical conditions, but it can also be a sign of some cancers such as leukaemia or some colon or stomach cancers.

NO. 11 - PERSISTENT COUGH

If you do not have a cold, allergies, or the flu yet you have a cough lasting more than three to four weeks, consult your doctor, especially if you smoke. Your doctor will examine your throat, check your lung function, and possibly order x-rays.

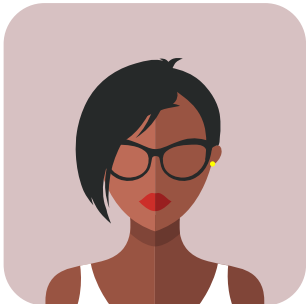
MYTH vs FACT	
I am too young to get breast cancer	 Many women who are under 40 are diagnosed with breast cancer
Alcohol is not linked to breast cancer	 Alcohol use increases the risk of breast cancer
Only women with a family history of breast cancer are at risk	 All women are at risk, but family history increases the risk
I have never had children, so I can't get breast cancer	 Women who have never had children, or only had them after 30, have increased risk of breast cancer

Zama and Anna walking out of class

Anna



Zama



Anna:	Hey Zama, Tobego says that tonight's the night! We need to do it now.
Zama:	What? Tonight?
Anna:	I told Gogo that I'm going to Zinzi's party tonight, but I'm not. We're going to Tobego's house tonight. He lives on his own. I'm excited, but scared too.
Zama:	Anna I don't know, hey. He's so old. You're only 16! And he's 35! He could be your dad...
Anna:	C'mon Zama, you're just jealous. He's so cool. He picks me up from school in his swag car and buys me cool presents and gives me lots of money. He even gives my parents money all the time. And I feel safe with him because he protects me. He says I belong to him and no one else can have me. He says he wants to teach me how to do it so I can practice before we get married.
Zama:	What if he hurts you?
Anna:	Why would he hurt me? He will never, ever. He says I am the most beautiful girl and he loves me. And I love him.
Zama:	Well, just saying- I think you need to be careful. I'm too scared to have sex. I'll only do it when I'm grown up.
Anna:	You do that! You're such a baby. That's why the guys don't like you.
Zama:	What do you mean the guys don't like me, did they say that?
Anna:	Well duhhh! You're all like...into your books and don't go out at night and all sorts.
Zama:	But Anna, you're really clever. You do well without even working. I have to work really hard just to pass. I can't help it, but I feel so ashamed that you guys talk about me. I hate that. I just wanna fit in and be like everyone else.
Anna:	Well you know what to do then girlfriend, it ain't rocket science. Anyway, I'll see you, I gotta go, tell you all about it tomorrow.

1. Identify all issues in this scenario that place Anna at risk.
2. What are the benefits for Anna?
3. Comment on the statement: "He says I belong to him and no one else can have me".
4. What are the risks for Zama in this situation?
5. What are the benefits for Zama in this situation?

6 MONTHS LATER

Zama hasn't seen Anna for 2 weeks. Also a few weeks ago, she didn't attend classes for a whole week. On her way home, she stops by Anna's house. She finds Anna on her bed her eyes swollen and red from crying.

Alarmed, she asks:

Zama:	What's wrong? Are you okay?
Anna:	No I'm not!! I'm miserable... I just want to die, I want to die. (sobbing)
Zama:	What happened? Are you hurt?
Anna:	Go away!!! Just go away Zama, you know don't you? You just came to say "I told you so"!
Zama:	What are you talking about? I'm worried about you. I haven't even seen you for months. You've been avoiding me. At first I was cross because you just dumped me for your boyfriend, but now I'm worried. You're my friend and I love you. Please tell me what's wrong. Maybe I can help?
Anna:	No one can help. I don't know where to start.
Zama:	It's ok. just tell me everything.
Anna:	Well, since I started sleeping with Tobego, he changed. He's stopped me from being with my friends, he said I'm not allowed. And then a few weeks ago he saw me talking to Zoe and he just dragged me to the car. I was so ashamed. Then he took me to his house and beat me. (sobbing) He punched me on my ear and now it feels weird like something's wrong inside...and...he slapped and kicked me in my stomach... and then forced himself on me. I couldn't bear to have sex but he didn't care. He said he paid for it. I couldn't even go to school. And when my mother saw my bruises she said I must have done something to make him angry.
Zama:	What?! Oh my God Anna, that's awful. We need to report it.

Anna:	No I can't! I'm scared. And now... (sobbing) and now it's even worse. Last week I had sores in my private parts and I had to go to the clinic. I was also vomiting. They said I have an infection and they also did a pregnancy test- I'm PREGNANT!!!
Zama:	What? You guys didn't use a condom? Are you crazy?
Anna:	He never did. He said you can't eat the wrapping without the sweet. And when I said I was scared of HIV he became angry and asked if I was sleeping around or accusing him of something. So I just let him to do it.
Zama:	But you're just a kid!! Does he know about the baby? Does your mother know?
Anna:	No, I'm not telling anyone. If I do they'll make me keep it. And then they'll make me leave school... and Mr Mopanyane said I'm in line for the maths award(head in hands), what am I going to do?
Zama:	So what does that mean? What are you going to do?
Anna:	I have to get rid of it... I know someone my cousin used.
Zama:	Anna...I'm really scared for you. Why don't you just go to the clinic?
Anna:	Because everyone knows EVERYONE! I just can't.
Zama:	And Tobego? You need help. You need to tell someone.
Anna:	OMG! It's too much...I can't do it, I can't. Maybe I should just....

1. Identify the issues facing Anna in this scenario.
2. Each group will be allocated one issue by the facilitator. Use the decision making tool (hand-out) to evaluate the choices attached to each issue.

ACTIVITY: CRUCIAL CONVERSATIONS ROLE-PLAY

OBJECTIVE:	To provide a real life simulation to enable participants to identify with the issues under discussion
REQUIREMENTS:	Crucial conversation script, flip chart paper, markers, slides
TIME:	60 minutes

PLANNING NOTES

- Prepare to role play the Anna and Zama scenario (crucial conversation)
- Prepare flipchart with questions for the crucial conversation exercises
- Prepare/review slides on early sexual debut and termination of pregnancy (TOP)

PROCEDURE

1. Explain that as an introduction to the next few lessons, the facilitators will role-play a 'crucial conversation' that highlights various important issues, that will be covered in the rest of the module.
2. Divide participants into groups of around 5 or 6.
3. The facilitation team must enact part one of the crucial conversation between Zama and Anna.
4. Invite groups to observe the role play, and answer the questions on the prepared flipchart (15 minutes).
5. Present slides on risks associated with **early sexual debut** and **sex with older men**.
6. Enact part two of the CC.
7. Invite groups to observe the role play, and complete the exercise on the prepared flipchart (15 minutes).
8. Present slides on **unplanned pregnancy** and **termination of pregnancy**.
9. Explain that the scenario is an introduction to issues that would be covered later in the module.
10. In the plenary, lead a discussion using the **discussion questions** provided.

DISCUSSION QUESTIONS:

- Ask participants how they experienced the scenario.
- What stood out for them the most?
- How did the scenario impact on them?
- Discuss ideas on what can be done to educate communities about the issues highlighted in the scenario.

PROVIDE GUIDANCE AS FOLLOWS:

- Ask: How can I help?
- Refrain from giving advice.
- Assist to list options and consequences, and allow her to make her own choice.
- Offer to accompany her to clinic.
- Listen with warmth and empathy.
- Respect her decisions and choices.
- Be non-judgemental.

LESSON TWO: FAMILY PLANNING

AIM: To explore issues around family, particularly in relation to women, ways of preventing unwanted pregnancy and contraception.

TIME: 30 minutes

LESSON OBJECTIVES:

On completion of this lesson, participants should demonstrate enhanced understanding of:

- What is family planning?
 - What are the advantages and disadvantages of family planning?
 - Who provides family planning/contraceptives?
 - What can you expect from family planning interventions?
 - Methods of contraception
-

PURPOSE OF ACTIVITIES:

- To correct misconceptions
 - To create opportunities for participatory learning
 - To enable participants to become informed about all relevant aspects of family planning
-

ACTIVITY: SMALL GROUP SESSION

OBJECTIVE:	To review baseline knowledge To enable participants to become informed about all relevant aspects of family planning
REQUIREMENTS:	Flip chart paper, markers
TIME:	15 minutes

PLANNING NOTES

- Familiarise yourself with the content of the notes on family planning
 - Write up questions for groups on flip charts
-

PROCEDURE

1. Introduce the lesson to participants, and explain that they will be divided into groups to review issues around family planning.
 2. Divide participants into groups using a creative technique that doubles up as an energiser (refer to energisers in the Leader's Resource).
 3. Questions for groups:
 - o What is family planning?
 - o What are the advantages and disadvantages of family planning?
 - o Who provides family planning/contraceptives?
 - o What can you expect from family planning interventions?
 - o Methods of contraception?
 4. Show slides on family planning.
 5. Facilitate a plenary discussion using the **discussion questions** provided.
-

DISCUSSION QUESTIONS

- Ask participants how they personally view the issue of family planning.
- Is family planning a woman's issue? Why discuss it under women's health and empowerment?
- Are there any outstanding questions?

THE AIM OF FAMILY PLANNING AND CONTRACEPTION

The purpose of family planning is to help prospective parents plan how many children they want, if any, and how close together they want them. Contraceptive methods, when correctly used, help ensure that these goals are achieved. The goal is to reduce risks and create benefits for women and communities as a whole.

REDUCING RISK

By preventing unintended pregnancy, the overall risk to mothers and infants is reduced. Older and younger mothers are particularly at risk during pregnancy and childbirth. By spacing pregnancies, women reduce their risk of complications and death. By preventing the rate of unwanted pregnancies, women also reduce the need to seek out dangerous backstreet abortions.

Mothers who have more than 4 children can be at higher risk of maternal mortality, so by planning the number of children in the family, this risk is reduced. The ability to make informed choices about family planning empowers women and subsequently strengthens communities. By choosing smaller families, each child can have added attention, benefiting the child's development. Where families have fewer siblings, all the children are more likely to stay in school.

Teenagers who fall pregnant are at risk of health problems, and their babies are more likely to be born prematurely, or suffer neonatal mortality. These adolescents are also likely to leave school, which has an impact on their future, their family, and their communities.

BENEFITS

When women can make informed decisions about their reproductive health, and choose when to fall pregnant, they are more likely to complete their education and add value to the communities where they live and work.

An added benefit to utilising a family planning method, such as male and female condoms, is that it helps to reduce STI's including HIV transmission. Proper family planning for women with HIV can help ensure fewer HIV infected babies.

Many communities and countries are not able to sustain the population growth with enough resources. By controlling the population there is less impact on the resources that are available to communities for development.

WHERE TO GET FAMILY PLANNING SERVICES IN SA

South Africa has many family planning clinics, as well as clinics resourced with health care providers who have extensive skills and knowledge about family planning and contraception.

Doctors, gynaecologists, midwives, and nurses can help with various types of contraception, and provide advice on family planning, with the clinical professionals handling more complex matters like sterilisations.

Even community health workers can provide basic assistance with family planning like counselling and basic contraception like condoms.

WHAT YOU NEED TO KNOW ABOUT CONTRACEPTION

Often, contraception can be seen as a 'woman's job' with men not always taking an active role. For men, contraception is still limited to condoms and vasectomies.

Although women often wish to manage their reproductive health, many do not use contraception. Reasons are mixed, but gender based barriers, social stigma, religious beliefs, limited choice and access, fear of side effects or actual side effects, as well as issues like poor quality of service or biased providers that make them uncomfortable can prevent these women from accessing contraception.

How to use a Female Condom



1 Check the expiry date and make sure the package is sealed with no air escaping from it.



2 Open the package carefully, make sure not to damage the condom. Do not use teeth or nails for this.



3 Rub condom to spread lubricant.



4 Hold ring and squeeze into figure eight.



5 Insert as far as it will go.



6 Do not twist condom.



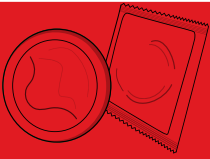
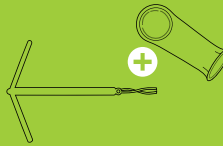
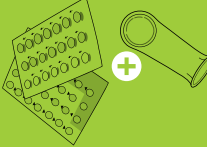
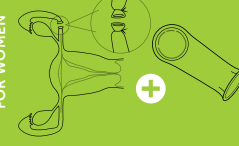
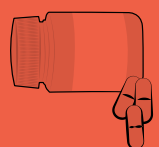
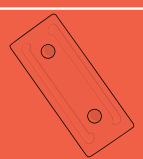
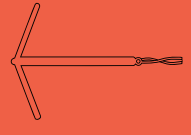
7 During sex guide penis into the condom.



8 To remove, squeeze and twist outer ring, and pull out.



9 Wrap the condom in toilet paper and throw away in the rubbish bin. Do not flush it down the toilet.

	MALE CONDOM	FEMALE CONDOM	COPPER INTRAUTERINE DEVICE (Cu IUD)	INJECTABLES	IMPLANT	ORAL CONTRACEPTIVE PILL	VOLUNTARY STERILISATION FOR WOMEN	VOLUNTARY STERILISATION FOR MEN	POST-EXPOSURE PROPHYLAXIS (PEP)	EMERGENCY CONTRACEPTIVE PILL	Emergency Cu IUD
What is it?	The male condom is a thin sheath of latex that fits over the erect penis. 	The female condom is a thin sheath that fits inside the vagina before having sex. 	The Cu IUD is a small, thin T-shaped device that is inserted into the womb (uterus). It is also known as the 'loop'. 	The injectable contains a hormone that prevents the release of an egg from the ovary. You need to go for the injection every two or three months depending on the type used. 	Hormonal implants are small, thin rods, about the size of a matchstick inserted under the skin by a health care provider, and can be removed at a later stage. 	The pill comes in a 28- or 21-day pack. The pills contain hormones that regulate your system to prevent pregnancy. Different types of the pill are available at your local clinic. 	Female sterilisation is an permanent form of contraception. It involves the cutting and tying of the fallopian tubes to prevent any future pregnancy. 	Male sterilisation is the permanent tying of the sperm tubes. It is quick and mostly painless. It is done at a clinic. It is also known as a 'Vasectomy'. 	PEP is a 28-day course of drugs that prevent HIV which helps prevent HIV from developing in the body. 	The emergency pill (morning after pill) is used to prevent an unplanned pregnancy after unprotected sex. 	The emergency Cu IUD may prevent an egg from being fertilised or from implanting in the womb. It prevents pregnancy but not HIV or other STIs. 
How does it work?	It prevents sperm and egg from meeting. You can use a water-based lubricant with a condom to increase the enjoyment of sex, but do not use any oil-based lubricant. Lubricants prevent a condom from breaking or slipping. A new condom must be used for every round of sex. If a condom breaks or tears, you should use emergency contraception immediately after sex. You can also ask for post exposure prophylaxis (PEP) at the clinic or hospital.	It prevents sperm and eggs from passing into or from the vagina. A new female condom must be used with each round of sex. If a condom breaks or tears, you should use emergency contraception immediately after sex. You must use a condom every time you have sex to prevent HIV and STIs.	This Highly effective long-lasting method can prevent pregnancy for up to ten years. Does not cause weight gain. It does not affect the enjoyment of sex.	It is a highly effective method. Only needs to be repeated every 2-3 months depending on the type of injection. Can be used by breast feeding mothers. It does not affect the enjoyment of sex.	This highly effective long-lasting method works for up to ten years, depending on the type of implant. It does not affect the enjoyment of sex.	This effective method can help to make your menstrual period more regular. It does not affect the enjoyment of sex.	The procedure means that eggs are not released into the uterus and pregnancy is prevented. It is safe and legal for women over the age of 18 to be sterilised. Sterilisation is safe for women who have had children, who have not had children. You must also use a condom to prevent HIV and STIs. Is a quick and painless procedure done at a clinic or hospital.	The sperm tubes which carry sperm from each testis are tied so that sperm does not get into the man's semen. Condoms still need to be used for three months after the operation, or for the next fifteen to twenty times that the man has sex to prevent pregnancy. You must also use a condom every time you have sex to prevent HIV and STIs.	PEP must be taken within 72 hours (three days) after sex or rape. The drugs help the body's immune system to stop the virus from multiplying. It is available from clinics and hospitals. You DO NOT have to report the rape to the police to receive PEP.	It prevents an egg from being released, so that it cannot get fertilised. It must be taken as soon as possible and within 72 hours of having sex. The sooner you take it, the more effective it will be. You should have an HIV test at the same time. If you are negative you should start PEP. If you are positive you should have a follow-up test after six weeks.	Having an IUD inserted prevents pregnancy but not HIV or other STIs.
What are the advantages?	Condoms protect women from getting pregnant, and protect men from HIV and STIs. They are free at clinics and can be bought at garages, shops and pharmacies.	A female condom can be put in just before sex, or up to eight hours before sex. Condoms protect women from getting pregnant and passing on HIV and STIs. They are free from clinics and hospitals or can be bought from pharmacies.	This highly effective long-lasting method can prevent pregnancy for up to ten years. Does not cause weight gain. It does not affect the enjoyment of sex.	It is a highly effective method. Only needs to be repeated every 2-3 months depending on the type of injection. Can be used by breast feeding mothers. It does not affect the enjoyment of sex.	This highly effective long-lasting method works for up to ten years, depending on the type of implant. It does not affect the enjoyment of sex.	This effective method can help to make your menstrual period more regular. It does not affect the enjoyment of sex.	The procedure means that eggs are not released into the uterus and pregnancy is prevented. It is safe and legal for women over the age of 18 to be sterilised. Sterilisation is safe for women who have had children, who have not had children. You must also use a condom to prevent HIV and STIs. Is a quick and painless procedure done at a clinic or hospital.	The sperm tubes which carry sperm from each testis are tied so that sperm does not get into the man's semen. Condoms still need to be used for three months after the operation, or for the next fifteen to twenty times that the man has sex to prevent pregnancy. You must also use a condom every time you have sex to prevent HIV and STIs.	PEP must be taken within 72 hours (three days) after sex or rape. The drugs help the body's immune system to stop the virus from multiplying. It is available from clinics and hospitals. You DO NOT have to report the rape to the police to receive PEP.	It prevents an egg from being released, so that it cannot get fertilised. It must be taken as soon as possible and within 72 hours of having sex. The sooner you take it, the more effective it will be. You should have an HIV test at the same time. If you are negative you should start PEP. If you are positive you should have a follow-up test after six weeks.	This long-lasting method can be left in as a regular contraceptive method. It does not affect the enjoyment of sex.
What are the disadvantages?	Some people have an allergic reaction which causes itching, burning, or swelling. If you have any problems with your condom, ask your healthcare provider.	Side effects may include cramping, spotting, pain during and after insertion of the Cu IUD. Can be pushed out (expelled) from the uterus into the vagina, in the first few months of use.	This highly effective long-lasting method can prevent pregnancy for up to ten years. Does not cause weight gain. It does not affect the enjoyment of sex.	It is a highly effective method. Only needs to be repeated every 2-3 months depending on the type of injection. Can be used by breast feeding mothers. It does not affect the enjoyment of sex.	This highly effective long-lasting method works for up to ten years, depending on the type of implant. It does not affect the enjoyment of sex.	This effective method can help to make your menstrual period more regular. It does not affect the enjoyment of sex.	The procedure means that eggs are not released into the uterus and pregnancy is prevented. It is safe and legal for women over the age of 18 to be sterilised. Sterilisation is safe for women who have had children, who have not had children. You must also use a condom to prevent HIV and STIs. Is a quick and painless procedure done at a clinic or hospital.	The sperm tubes which carry sperm from each testis are tied so that sperm does not get into the man's semen. Condoms still need to be used for three months after the operation, or for the next fifteen to twenty times that the man has sex to prevent pregnancy. You must also use a condom every time you have sex to prevent HIV and STIs.	PEP must be taken within 72 hours (three days) after sex or rape. The drugs help the body's immune system to stop the virus from multiplying. It is available from clinics and hospitals. You DO NOT have to report the rape to the police to receive PEP.	It prevents an egg from being released, so that it cannot get fertilised. It must be taken as soon as possible and within 72 hours of having sex. The sooner you take it, the more effective it will be. You should have an HIV test at the same time. If you are negative you should start PEP. If you are positive you should have a follow-up test after six weeks.	This long-lasting method can be left in as a regular contraceptive method. It does not affect the enjoyment of sex.

LESSON THREE: GENDER BASED VIOLENCE (GBV)

AIM: To increase an understanding of what gender based violence is, and what the underlying causes are.

TIME: 60 minutes

LESSON OBJECTIVES:

On completion of this lesson, participants should demonstrate enhanced understanding of:

- What gender based violence is(GBV)
 - Different types of GBV
 - What to do in cases of GBV
 - The relationship between personality types and power in relationships
 - How to prevent GBV
-

PURPOSE OF ACTIVITIES:

- To review baseline knowledge
- To correct misconceptions
- To create opportunities for participatory learning
- To provide triggers for discussion around significant issues relating to GBV

ACTIVITY: FROZEN STATUES – POWER AND RELATIONSHIPS

OBJECTIVE: To highlight various key issues pertaining to women's health and empowerment

REQUIREMENTS: Flip chart paper, markers

TIME: 20 minutes

PLANNING NOTES

- Familiarise yourself with how to conduct the activity.
 - Anticipate issues that may emerge, and research all aspects thoroughly in preparation.
 - Prepare folded pieces of paper, each with a specific problem related to women's health and empowerment written on it.
-

PROCEDURE

1. Divide the participants into groups of 5 or 6.
2. Ask groups to pick one of the folded pieces of paper.
3. Give the groups five minutes to use their bodies to create a statue(that does not talk or move), which represents this problem.
4. One by one, ask the groups to show their statue to the rest of the group. When each statue is shown, ask the rest of the group the following questions:
 - o What is this problem being shown in this statue?
 - o Does this happen often in our community?
 - o Why does this happen?
 - o What can we do when we face this kind of problem?
5. After all of the statues have been shown, summarise the discussion by presenting slides on the challenges women face in their relationships, and strategies for overcoming them.
6. Facilitate a plenary discussion using the **discussion questions** provided.

DISCUSSION QUESTIONS

- Are there any ways that they can think of to avoid this problem?
- Which of the strategies that have been discussed in this session do they think they could use to help them cope?

SUMMARY OF KEY LEARNINGS

- Some problems can be avoided if we can see them coming, but some cannot.
- Thinking ahead about how to deal with problems can help us to cope with them when we need to face them.
- Having a personal plan and a supportive network of other people can help us to get through these problems more easily.
- Understanding our rights is important.
- Health and safety are a priority.

ACTIVITY: BEHIND CLOSED DOORS

OBJECTIVE:	To increase understanding of what gender based violence is, and how and where it happens
REQUIREMENTS:	Flip chart paper, markers
TIME:	20 minutes

PLANNING NOTES

- Familiarise yourself with how to conduct the activity.
- Anticipate issues that may emerge, and research all aspects thoroughly in preparation.
- Prepare flipchart with questions for groups written in advance.

PROCEDURE

1. Write the words 'Gender Based Violence' (GBV) on the flipchart.
2. Ask participants what they think gender based violence is. Accept all contributions and make a note of them. Ensure that the following topics come up: violence against women, violence against girls, domestic violence and rape. Clarify if there are any misconceptions. Reiterate that gender based violence is any form of violence directed at women, and that this includes threats of violence and other abuse that is not physical.
3. Divide the participants into five groups. Allocate one of the following settings to each group: the bedroom, the home, the education/ training institution, the workplace, and the local community.
4. Give the groups 10 minutes to discuss the following questions, in relation to their allocated place:
 - Does gender based violence happen in this place?
 - What do we see happening?
 - Why does this happen?
 - What are the results of this violence?
5. Groups must report back to the larger group about their discussion. Answer any questions, and address any myths or misconceptions.
6. Show slide on wheel of violence.
7. Facilitate a plenary discussion using the **discussion questions** provided.

DISCUSSION QUESTIONS:

- Are there any ways that they can think of to avoid this problem?
- What can we do about this kind of violence?
- Whose responsibility is it to act against violence and abuse?

KEY LEARNINGS

- Gender based violence is not just about individuals who are the perpetrators and survivors or victims of violence, but it is about the whole of society.
- The primary cause of gender based violence is the belief that women are not equal to men.
- To address this, we need to understand the concept of masculinity and male culture.
- Violence compromises the health, dignity, security, and independence of people affected, and can have long lasting consequences.
- Silence about gender based violence does not protect anybody; instead, it makes the situation worse as it is seen to be acceptable. Talking about this violence is the only way to make more people aware of it and to bring about change

ACTIVITY: THE GOOD WIFE

OBJECTIVE:	To address attitudes and biases that promote GBV
REQUIREMENTS:	A flipchart and pens; coloured board (3 per person: 1 red, 1 green, 1 blue)
TIME:	30 minutes

PLANNING NOTES

- Familiarise yourself with how to conduct the activity.
- Anticipate issues that may emerge and research all aspects thoroughly in preparation.
- Prepare written statements in advance

STATEMENTS FOR THE GAME

- If a woman behaves well and respects her partner, she won't get beaten up by him.
- Alcoholism is a disease that leads men to be violent, and it's not their fault.
- Women who choose to be breadwinners put themselves at risk, as they are more likely to be abused.
- It's in the biological make-up of men to be strong, aggressive, and violent.
- Women who are abused by their partners must persevere and work at the relationship to make it better.
- In a marriage withholding sex is just not acceptable.



PROCEDURE

1. Explain to the participants that you are going to call out statements. If they agree with what you say, they should hold up a 'GREEN CARD'. If they disagree, they should hold up a 'RED CARD'. If they are unsure, they should hold up a 'BLUE CARD'.
2. Call out a statement from the list and ask the participants to indicate their view by holding up a card.
3. Ask them randomly why they agree or disagree, or are not sure.
4. Ask them what the consequences of having that opinion might be. Encourage the participants to think about how what they think might affect others in their family, including husbands, partners, brothers, sisters, and children.
5. Facilitate a plenary discussion using the **discussion questions** provided.

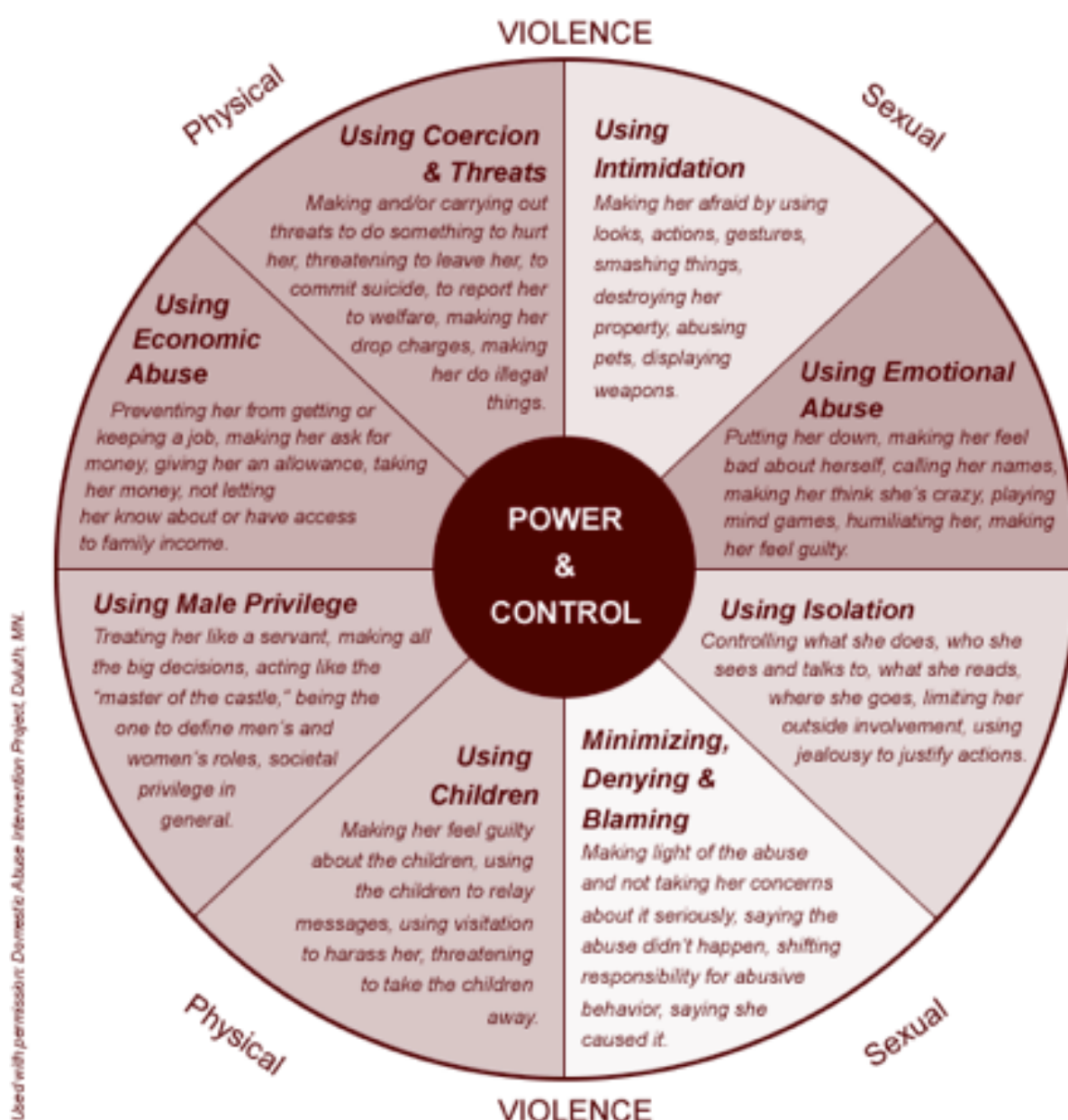
DISCUSSION QUESTIONS

- Is there a relationship between personality and power?
- If you're 'born with a certain personality' is it possible to change?
- What kind of power is 'good power'?
- What type of power is 'bad power'?
- What are the risks of being disempowered?

KEY LEARNINGS

- Violence is used as a way to control people in intimate relationships, such as wives, girlfriends, husbands or casual partners.
- Sometimes culture, religion, or traditional practices are used as an excuse to justify intimate partner violence.
- Silence and myths about gender based violence allow this type of violence to continue.
- When young people see violence as acceptable, they may also grow up to be violent.
- How we see and accept or challenge violence affects others around us.
- It is up to each person individually to make changes in their own environment, if they would like to see a world with less violence.
- Remind the group about the steps they can take if they or somebody they know has been abused.
- Ask if there are any further questions or comments, and thank the participants for their contributions.

THE WHEEL OF VIOLENCE JUST KEEPS ON TURNING

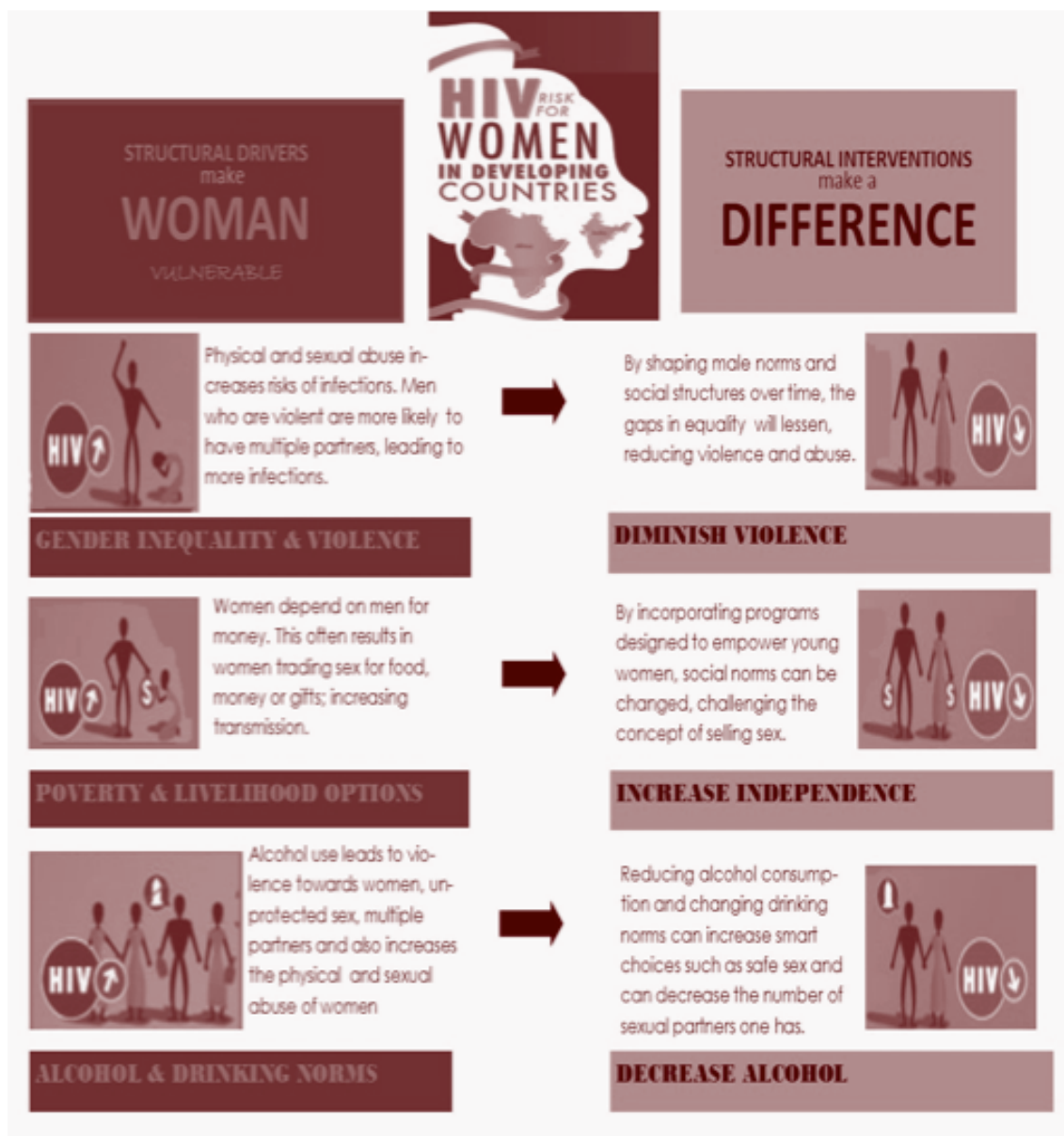


MY PERSONALITY PUTS ME AT RISK

THE PASSIVE PERSON	THE AGGRESSIVE PERSON	THE ASSERTIVE PERSON
Is afraid to speak up	Interrupts and 'talks over' others	Speaks openly
Speaks softly	Speaks loudly	Uses a conversational tone
Avoids looking at people	Glares and stares at others	Makes good eye contact
Shows little or no expression	Intimidates by using expressions	Shows expression which matches the message
Slouches and withdraws	Stands rigidly, crosses arms, invades the personal space of others	Relaxes and adopts an open stance and expressions
Isolates self from groups	Control groups	Participates in groups
Agrees with others despite personal feelings	Only considers own feelings and/or makes demands of others	Keeps to the point
Values self-less than others	Values self-more than others	Values self-equal to others
Hurts self to avoid hurting others	Hurts others to avoid being hurt	Tries not to hurt any one [including self]
Does not reach goals and may not even know goals	Reaches goals but hurts others in the process	Usually reaches goals without hurting others
Lies to keep the peace	Sniffs out lies and uses them as valid reasons to hurt	Is open and transparent
"You're OK. I'm not."	"I'm OK. You're not."	"I'm OK. You're OK,"

USING YOUR POWER

PERSONAL POWER - ASSERTIVE	ABUSIVE POWER - AGGRESSIVE	ABDICATED POWER - PASSIVE	MANIPULATIVE POWER - DECEPTIVE
- This is 'good power'. It is appropriate and acceptable. - Honest - Responsible - Open-minded - Appropriate choices - Say what you need - Say what you mean - Mean what you say - Doer - Flexible - Goal Getter Confident - Compassionate - Positive - Risk-Taker	- This kind of power hurts other people. Abusive power is forcing your will on another person. - Controlling - Pushy - Rude - Violent - Bossy - Threatening - Acting superior - Interrupting - Cruel - Angry - Mean - Teasing - Insensitive	- You hurt yourself when you don't take personal responsibility. Passive power is giving your power to another person. - No eye contact - Hide - Withdraw - Don't give your opinion - Silent - Mumble - indecisive - Walk with your head down - Feel sorry for yourself	- This kind of power hurts people and makes them dislike you. Manipulative power is controlling another person. - Using emotional blackmail - Being sneaky or dishonest - Spreading rumors - Making people feel guilty - Mean - Teasing - Silent treatment - Nothing is ever your fault



ACTIVITY: GET UP, STAND UP

OBJECTIVE:

- To demonstrate the use of the Personal STOP model for acting assertively.
- To identify aspects of communication that will contribute to the effectiveness of the response.
- To provide opportunities for participants to practice, using a structured model.
- To enable participants to practice 'thinking on their feet' when developing a response to a difficult situation using the Personal STOP model.

REQUIREMENTS:

Flip chart and pens to record plenary discussion

TIME:

45 minutes

PLANNING NOTES

- Familiarise yourself with how to conduct the activity
- Make 6 copies of Handout: Personal STOP Model of Assertion
- Prepare slides on the assertive model, the rights and responsibilities and the STOP model

PROCEDURE

- Present slide on the Assertive Model Diagram (Resource Sheet: Assertive Details).
- Present the Rights and Responsibilities table displaying only the 'rights' and blocking out the 'responsibilities'. Allow group to come up with responsibilities associated with each right.

3. Present the Personal STOP Model of Assertion explaining each step as follows:
 - o State the action or behaviour that led to the conflict. Use “I” statements rather than “you” statements, for example, “when I saw/ gave/ said...”
 - o Tell the person how it made you feel without blaming them, for example, “I felt angry/ disappointed/ upset/ hurt ...”
 - o Offer a suggestion of how you would prefer the person to act, for example, “In that situation I would prefer that...”
 - o Propose a set of positive consequences or outcomes that could result, for example, “If that happens I/we will/ can/ be...”
4. The whole group will use one example from the scenario (breaking the shell) to write an individual response using the Personal STOP model (e.g. refusing to have sex).
5. When all four steps are written, ask volunteers to demonstrate STOP model script, and discuss the possible success or failure of this method.

TIPS

- Assertion should not be confused with aggression, which seeks to win at the expense of another person’s rights and feelings.
- While anger may be present while acting assertively, it is likely to be handled better and dissipated through positive action.
- Being assertive or not, is a choice. You don’t have to be assertive all the time.

Non-assertive Action	
Aggressive action Respects own rights and feelings Ignores others rights and feelings	Submissive Action Ignores own rights and feelings Respects others rights and feelings
Assertive Action Respects own rights and feelings	
Right to:	Associated responsibility to:
1. Make your own decisions	Allow others to make their own decisions
2. Be treated with respect	Treat others with respect
3. Refuse requests by others	Refuse courteously and assertively
4. Make mistakes	Ensure mistakes don’t harm others
5. Change your mind	Act reasonably
6. Take time to consider requests	Allow others this courtesy
7. Make reasonable requests	Do not impose upon others
8. Have personal opinions	Respect the opinions of others
9. Control your destiny	Allow others to control their own destiny
10. Express your feelings	Consider the feelings of others

KEY INFORMATION

South Africa has high levels of gender based violence, sexual assault, and intimate partner violence which contributes to the increased risk of HIV infection for women. Physical, sexual, and psychological abuse are common problems. Some of the factors that are associated with this abuse include low education levels, alcohol and drug abuse, the use of violence to resolve conflict, witnessing a mother's abuse, and having more than one partner. Reasons for conflict are mainly associated with attempts by men to control women, their sexuality, and their households.

South Africa has produced an environment in which violence has flourished, and where it has become part of the South African male identity. Gender based violence is any form of violence or abuse that is directed against a person on the basis of their gender. The term 'gender' does not refer to biological sex (the physical differences between men and women) but refers to the different roles, attributes and behaviours that are seen as appropriate for men and women by society.

In South Africa, gender based violence is most often violence against women and girls, based on women's lower status in society. Although the term 'violence against women' is common, the term 'gender based violence' takes into account that this violence is based on social beliefs about gender and is related to ideas about power and control. It is important to note that men are not biologically more violent than women, but it is male socialisation (the way that men are taught to behave) and peer pressure to conform to gender roles that makes some men believe that violence is an acceptable way to solve conflict in their relationships with women. Gender based violence both reflects and reinforces the unequal power relationships between men and women and compromises the health, dignity, security and independence of its survivors. Silence about abuse and violence allows this situation to continue, and reinforces the idea that women cannot speak for themselves or speak out about their experiences and their needs. This silence also creates a stigma that prevents women from speaking out about abuse, and can lead women to keeping silent and protecting their abusive partner.

PHYSICAL ABUSE

Physical abuse includes slapping or smacking, grabbing or pushing, kicking, or throwing objects at another person. Other forms of abuse include any physical threats (to hit or throw objects at a person) as well as emotional abuse, where a man might ignore a woman, or leave the house, or threaten to leave the relationship or stay away for the night or weekends.

VERBAL ABUSE

Verbal abuse includes any shouting, swearing at, or using dirty language and rude names.

PSYCHOLOGICAL ABUSE AND OTHER ABUSE

Psychological abuse, emotional abuse or mental abuse, are forms of abuse where one person says or does things that result in psychological trauma, anxiety, depression, or extreme stress for another person, usually somebody who has less power than them. This includes threats, verbal aggression, dominant behaviours and bullying, stalking or jealous behaviours. Withheld economic support, where a person refuses or threatens to refuse to continue financial or material support is also a form of abuse. Some types of abuse and violence against women are perpetrated by other women. Some women commit abuse as a way to ensure their own survival and security; for example, older women may be abusive towards their daughters-in-law, women may use violence against their domestic workers to protect and assert their position as wives, or women in the workplace may undermine each other.

What to do if you are abused or assaulted or somebody you know has been abused or assaulted:

- Get to a safe place as soon as possible.
- Tell somebody who you trust who can help you through this difficult time.
- Follow the steps for handling violence in your community. This might involve first reporting it to the family or community leaders, who should then report it to the police.
- Any survivor of abuse can apply for a protection order at the nearest police station or magistrate's court, which orders a person to stop the abuse. An interim protection order can be issued at any time of the day or night. The protection order prevents the person who has abused or assaulted you from doing so again, or from coming near you, or from getting anybody else to hurt you. The application form for this court order is called a J480.
- If you decide to report the violence to the police, you can give a statement of what happened, have the event recorded in the occurrence book, and lay a criminal charge against the abuser. The case will be investigated by a detective at the police station. If you are afraid that you will get hurt again, tell the police, so that they can take steps to protect you.
- If you have been physically abused, you can ask a health care practitioner to fill in a J88 form, which details your injuries. This form can help to win a criminal case against the person who has assaulted or abused you.
- Police are not allowed to refuse to investigate your case if you report it. However, if there is too little evidence after an investigation by the police, a prosecutor may decide not to prosecute your case.
- People who use violence are likely to repeat this behaviour. If you are in a relationship with somebody violent, it is better to get out before it is too late.
- Remember that being beaten or abused is NEVER your own fault. Violence is not an acceptable response to conflict.

SEXUAL ABUSE AND RAPE

Sexual abuse includes rape or any attempt to rape another person. Rape means being forced to have sex against your will. It is most common that males rape females, but men and boys can also be raped, if a man forces his penis into a man's or a boy's anus. A woman forcing a boy to have sex with her is also sexual assault or abuse. Rape includes any forced penetration through oral sex, anal sex or vaginal sex. Being forced to touch somebody or to watch other people have sex is sexual abuse.

Men may use drugs, beating, threats, or other force, like telling women that they have 'a right' to have sex with them, in order to rape women. Nobody has a right to have sex with another person against their will. If a woman is forced to have sex against her will by her boyfriend or husband, this is still rape.

Most people in South Africa who are raped know the person who raped them, and it usually happens in a place they know. Young women are often raped by boyfriends or would-be boyfriends or by elderly sugar daddies, customers, or relatives. Women of all ages have a right to say 'no' to sex at any time with anybody, be it a boyfriend, husband, employer, or relative. Any sex without consent is rape, whether the woman has fainted, is asleep from drugs, has been knocked out, says 'no', fights, or can't move with fear.

Financial /Economic Abuse is a common tactic used by abusers to gain power and control in a relationship. The forms of financial abuse may be subtle or overt but in general, include tactics to limit partner's access to assets or conceal information and accessibility to the family finances. This type of abuse is less commonly understood, and yet one of the most powerful methods of keeping a survivor trapped in an abusive relationship, and deeply diminishes her ability to stay safe after leaving an abusive relationship.

Forms of financial abuse:

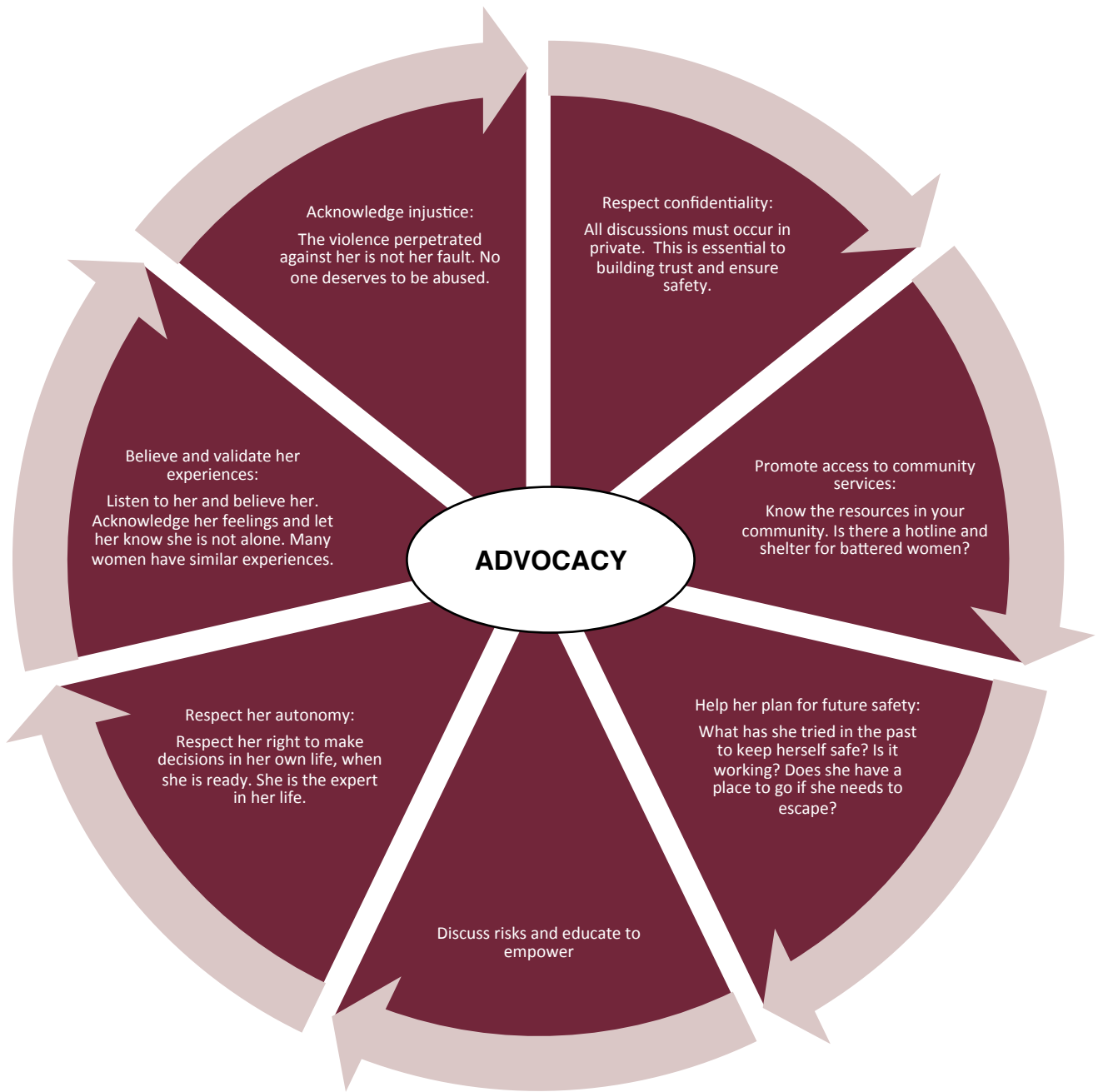
- Forbidding the victim to work
- Sabotaging work or employment opportunities by stalking or harassing the victim at the workplace or causing the victim to lose her job by physically battering prior to important meetings or interviews
- Controlling how money is spent
- Not allowing victim to access bank accounts
- Withholding money or an allowance
- Forcing victim to engage in fraudulent dealings
- Running up huge debt on joint accounts
- Withholding funds for the victim or children to obtain basic needs such as food and medicine
- Refusing or evading to pay child support or manipulating the divorce process by drawing it out by hiding or not disclosing assets
- Stealing victims' identity, property, or inheritance
- Hiding assets

What to do when financially abused, protecting yourself before and after you leave:

- Transfer your assets into a separate bank account
- Keep a copy of all your important papers, including bank statements, birth and marriage certificates, and documentation of jointly held assets
- As soon as you leave, change all your pin codes and passwords, avoid using birthdays or easily identifiable codes
- Call issuers of joint accounts and have your name removed, it will protect you from having to pay debt incurred after you leave
- Work on rehabilitating your credit score



HOW PEER EDUCATORS CAN HELP



Developed by the Domestic Violence Project of Kenosha Wisconsin.

WOMEN'S RIGHTS ARE HUMAN RIGHTS

Human rights are those rights that belong to every individual; man or women, girl or boy, infant or elder, simply because she or he is a human being. They embody the basic standards, without which people cannot realise their inherent human dignity. Human rights are those enshrined in a country's constitution or bill of rights that citizens or residents of that particular country are entitled to.

Human rights play an important role by setting minimum standards for how individuals and institutions should treat people. They also empower people to take action to demand and defend their rights and the rights of others. Young women are often the ones denied basic rights by virtue of many factors, including age and gender.

WHERE ARE OUR HUMAN RIGHTS OUTLINED?

The foundation document of human rights is the Universal Declaration of Human Rights, which was adopted by the United Nations in 1948. Over the years, the United Nations have developed other human rights conventions or treaties, which are legally binding to those countries that have ratified them. When countries that are members of the United Nations ratify a convention, they agree to abide by its provisions, to change the laws of the country to conform to the convention, and to report on its progress in doing so.

WHAT ABOUT WOMEN'S HUMAN RIGHTS?

Some of the conventions that have been developed address populations who experience particular discrimination (e.g. the Convention on the Elimination of all forms of Discrimination Against Women), one of the most important and widely ratified conventions we can learn about is the Convention on the Elimination of all forms of Discrimination Against Women, which has been signed by 180 countries including South Africa. CEDAW, as it is commonly referred to, was adopted in 1979 by the United Nations and is often described as an international bill

of rights for women. It defines what constitutes discrimination against women, and sets up an agenda for national action to end such discrimination.

Within the Convention, discrimination against women is defined as "...any distinction, exclusion or restriction made on the basis of sex which has the effect or purpose of impairing or nullifying the recognition, enjoyment or exercise by women, irrespective of their marital status, on a basis of equality of men and women, of human rights and fundamental freedoms in the political, economic, social, cultural, civil or any other field." By accepting the Convention, countries that are members to the United Nations commit themselves to undertake a series of measures to end discrimination against women in all forms, including:

- to incorporate the principle of equality of men and women in their legal system
- abolish all discriminatory laws and adopt appropriate ones prohibiting discrimination against women
- to establish tribunals and other public institutions to ensure the effective protection of women against discrimination
- to ensure elimination of all acts of discrimination against women by persons, organisations or enterprises

WHAT ARE THE FUNDAMENTAL PRINCIPLES BEHIND HUMAN RIGHTS?

I. EQUALITY:

The basis of human rights is that "all human beings are born free and equal in dignity and rights" Universal Declaration of Human Rights Article 1.

II. UNIVERSALITY:

Certain moral and ethical values are shared in all regions of the world, and governments and communities should recognise and uphold them. The universality of rights does not mean, however, that they cannot change or that they are experienced in the same manner by all people.



III. NONDISCRIMINATION:

The Universal Declaration of Human Rights and subsequent international human rights law afford the same rights and responsibilities equally to all women and men, boys and girls, by virtue of their humanity, and regardless of any role or relationship they may have.

IV. INDIVISIBILITY:

Human rights should be viewed as an indivisible body, including civil, political, social, economic, cultural, and collective rights. You cannot be denied a right because someone decides that it is “less important” or “non-essential.”

V. INTERDEPENDENCE:

Human rights concerns appear in all spheres of life; home, school, workplace, courts, and markets. Human rights violations

are interconnected, loss of one right detracts from other rights. Similarly, the promotion of human rights in one area supports other human rights.

VI. RESPONSIBILITY:

A. GOVERNMENT RESPONSIBILITY: Human rights are not gifts bestowed at the pleasure of governments. Nor should governments withhold them or apply them to some people but not to others. When they do so, they must be held accountable.

B. INDIVIDUAL RESPONSIBILITY: Every individual has a responsibility to teach human rights, to respect human rights, and to challenge institutions and individuals that abuse them.

C. OTHER RESPONSIBLE ENTITIES: Every part of society, including corporations, nongovernmental organisations, foundations, and educational institutions, also shares responsibility for the promotion and protection of human rights.

USEFUL INFORMATION

It is a good idea to familiarise yourself with the laws relevant to abusive and gender-based violence.

These include:

Domestic Violence Act, 1995 Criminal Law (Sexual offences and Related Matters) Act, 2007

All of these Acts are available online at www.acts.co.za and at www.acts.co.za and at www.justice.gov.za

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<http://health365.com.au/author/hayden-horner>

<http://www.who.int/mediacentre/factsheets/fs351/en/> (Family Planning fact sheets, May 2015)

<http://www.eqi.org>

Universal Declaration of Human Rights – www.unhcr.ch/udhr/ (the declaration is available in over 300 languages from this website)

International human rights law and conventions - www.un.org/law/

CEDAW and the Optional Protocol - www.un.org/womenwatch/daw/cedaw/

United Nations Population Fund human rights resource - www.unfpa.org/rights

MODULE THREE: MEN'S HEALTH



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ABBREVIATIONS

AIDS:	Acquired Immune Deficiency Syndrome	MMC:	Male Medical Circumcision
ARVs:	Antiretrovirals	STIs:	Sexual Transmitted Infections
HBV:	Hepatitis B Virus	STDs:	Sexual Transmitted Diseases
HCT:	HIV counselling and testing	TB:	Tuberculosis
HCV:	Hepatitis C Virus	TVETs:	Technical and Vocational Education and Training Colleges
HEAIDS:	Higher Education and Training HIV/AIDS Programme	UNAIDS:	United Nations Programme on HIV/AIDS
HIV:	Human Immuno-Deficiency Virus		
HPV:	Human Papillomavirus Virus		

CRUCIAL CONVERSATIONS

Philani is very stressed. He is sitting in the café deep in thought when he sees Dumi walking up to him.



High five man, I aced my test. What's up with you? Girl problems?

Eish man, I don't feel so good



What's cutting bro? You can tell me the vibe.

I got this burny feeling when I pass urine and eish it's very sore down there



Too much shagging hey? (laughs). Okay okay, no jokes. There's campus clinic man - go check it out

Haibo no way. Can I go show my stuff to a woman bro?



She's a nurse man- she seen everything already.

2 WEEKS LATER



Hey wassup – you sorted my man?

I'm freaking out bro, I'm freaking out





Like Philani most young men believe they are invincible (will live forever) because society conditions our minds to accept that “men are strong” and “men are powerful”. From a cultural point of view this idea is reinforced from a young age. Young boys are led to believe they should grow up to be the heads of the households, in charge of decisions, protect women and children and become leaders in the workplace. This creates the mindset that good health is guaranteed as an essential part of youth, and that illness, weakness and death are associated with old age. But is this really true.

AIM

The Men's Health module aims to encourage men to utilise health services including sexual and reproductive health services, empower them on how to protect themselves from infection and maintain optimal levels of health and well-being and motivate them to become involved in the prevention of physical and sexual violence

OVERALL OBJECTIVES OF THE MODULE

By the end of this module, participants will:

- Be familiar with the specific health challenges faced by young men in South Africa including communicable and non-communicable diseases (cancer, HIV, TB and STIs)
- Understand the benefits of condom use in preventing transmission of HIV and unplanned pregnancies
- Know all relevant information pertaining to Male Medical Circumcision
- Understand the social and cultural issues that impact men's values and attitudes towards women
- Understand the dynamics of gender based violence
- Be able to provide comprehensive, relevant and up to date information, knowledge and skills to empower others to improve informed decision making on a range of issues facing young men

MODULE CONTENT

The module has 5 lessons which focus on 3 targeted areas viz:

- Overview of Men's Health, screening tests for men and specifically men's cancers
- Condom use
- Male medical circumcision
- Challenges of being a man in South Africa
- Gender-based violence

LESSON ONE: WHAT'S SO SPECIAL ABOUT MEN'S HEALTH?

AIM: To review the priority issues related to Men's Health including regular screening and the various health issues that men are at risk for.

TIME: 180 minutes

LESSON OBJECTIVES:

On completion of this lesson, participants should:

- Know the various health issues for men as identified by global research evidence
 - Be familiar with screening tests available for both communicable (diseases that can be passed from one to another) and non-communicable diseases
 - Be familiar with the various cancers that men are susceptible to
-

PURPOSE OF ACTIVITIES:

- To provide information on relevant issues pertaining to men's health
 - To provide opportunities for participants to demonstrate existing knowledge
 - To use participatory methods for high impact to enable participants to realise how particular issues and situations have associated risks and consequences in terms of men's health
-

ACTIVITY: BUZZ GROUPS / RAPID FIRE QUIZ

REQUIREMENTS: Crucial conversation script, Flip chart paper, markers, slides

TIME: 60 minutes

PLANNING NOTES

Prepare/review slides for:

- The top ten men's health issues
 - Screening tests that men should do
 - Cancers and symptoms that men should not ignore
-

PROCEDURE

1. Invite 2 participants to read the 'crucial conversation' between Philani and Dumi
2. Introduce the subject of "men's health" and ask "so what's so special about women's health?"
3. Note all points on the flipchart
4. Present slides and facilitate discussion on the priorities in respect of men's health issues as identified by global research
5. Facilitate a buzz group on the various health issues that men need to screen for and the screening tests for each
6. Present slides and be sure to only fill in gaps
7. Ask participants to read "what you need to know about testicular cancer" and for 5 minutes in preparation for a rapid fire quiz
8. After 5 minutes facilitate a rapid fire quiz and ask those who answer correctly to stand
9. Inform them that they are in line for an award at the end of training
10. Facilitate the popcorn exercise i.e. people to randomly "pop up" and one reason some men don't like to test for HIV
11. Summarise the lesson using the **discussion questions** provided

DISCUSSION QUESTIONS:

- Ask participants if they have any further questions about men's Health?
- Did new information change the way they think about men's Health?
- Did you learn anything new?
- How did the information impact on them?

MEN'S HEALTH FACTS

4.4

Men live 4.4 years less than Women.

11

The last 11 years will be in poor health.



Men are more likely to die from heart disease at earlier ages.



Men are at a significantly higher risk of dying from liver disease.



80% of spinal cord injuries occur in young Men.



70% of developmental and learning disabilities affect Boys.



Men have an increased risk of dying from diabetes.



Men make up 56% of the workforce.



Yet 94% of all workforce fatalities are Men.



The top 3 reasons for reduced lifespan in Men are cardiovascular disease, suicide, and motor vehicle accidents.



Globally, alcohol kills almost 6 times more Men than Women.

Only 30% of a Man's overall health is determined by his genetics:

Having a Y chromosome is not an excuse for unhealthy lifestyle choices.



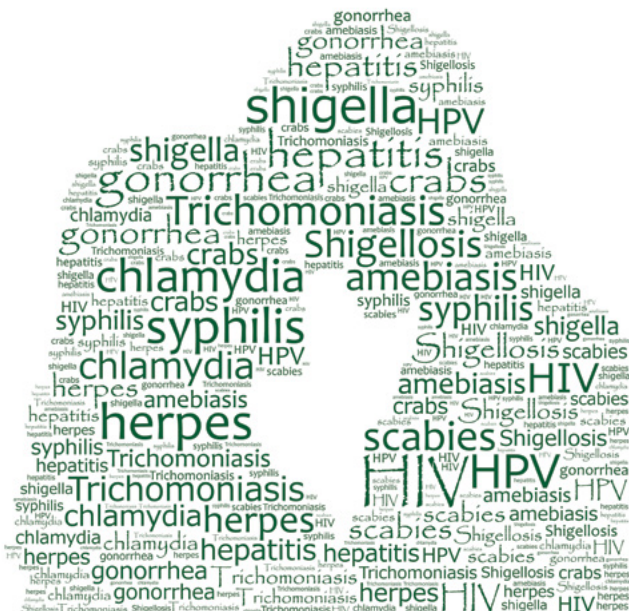
All statistics sourced from "A Roadmap to Men's Health: Current Status, Research, Policy & Practice 2018" published by the Men's Health Initiative of British Columbia.

WHAT'S SO SPECIAL ABOUT MEN'S HEALTH?

The following definition of a male health issue has been adapted from one proposed by the Department of *Health and Ageing, Australia*:

“A male health issue can be linked to physiological (body), psychological (mind), social (community), cultural (beliefs and traditions) or environmental factors (living conditions) that have a specific impact on boys or men which needs action or lifestyle changes from men to achieve improvements in health or well-being at either individual or population level ...”

This definition is meaningful to the lives of South African men because it acknowledges that there are more factors that contribute to men's health than the biological; that the health of men and boys cannot be separated from their ideas of what it means to be a man, or from the influences of the wider world of (for example) work or relationships. The definition emphasises that men need to act to bring about change or improvement.



The module focuses on issues such as male cancers, condom use, male medical circumcision, HIV and STI screening and prevention and gender based intimate partner violence. The importance of focusing on men is centred on the following norms/situations that exist in our communities:

- Male health and wellness does not receive enough focus
- Trends in health-seeking behaviour among men is low
- Men as decision-makers influence behaviour such as simultaneous multiple partners
- Men and women report men's resistance to condom use and HIV testing
- Traditional ideas about manhood impact significantly on behaviour and lifestyle management
- Alcohol and substance abuse influence attitude, intimate partner violence and sexual behaviour.
- There is a general lack of male involvement in family health issues

“Men’s health is the ‘Cinderella’ of public health, being generally overlooked and hidden in plain sight” says Peter Baker, Director, Global Action on Men’s Health.

Peter Baker is Director of Global Action on Men's Health: Men's health: a global problem requiring global solutions HEALTH MAY/JUNE 2016.

WHY IS IT NECESSARY TO DISCUSS MEN HEALTH?

Research from all over the world shows that men have poorer health outcomes than women. Men are far more likely than women to be at risk from an unhealthy lifestyle (including smoking and alcohol abuse), to be exposed to physical and chemical risks in the work environment, and to use health services ineffectively, particularly primary care and screening. Men are particularly reluctant to seek help for health problems. It seems clear that, across the world, men believe that the 'male role' prevents one from getting help and taking care of oneself.

The reasons are almost certainly linked to the fact that men are, in most cultures, the dominant sex and the main perpetrators of violence. But tackling issues such as male alcohol addiction and depression would help to reduce violence. Better male health would reduce the burden on partners and children who depend on men's incomes or who could end up becoming caregivers, at the cost of their own income or education. It would also reduce the burden on country's economies caused by lost productivity and health costs. There is an increasing body of research showing that 'gender sensitive' and outreach health interventions aimed at men can improve outcomes.

HEALTH ISSUES FOR MEN

HIGH CHOLESTEROL LEVELS (HEART DISEASE AND STROKE)

CHOLESTEROL is fat-like substance that's found in all cells of the body. Your body needs some cholesterol to make hormones, vitamin D, and substances that help you digest foods. Your body makes all the cholesterol it needs. Your blood cholesterol level has a lot to do with your chances of getting heart disease. High blood cholesterol is one of the major risk factors for heart disease. When there is too much cholesterol (a fat-like substance) in your blood, it builds up in the walls of the arteries. Over time, this build-up causes "hardening of the arteries" so that they become narrowed and blood flow to the heart is slowed down or blocked. The blood carries oxygen to the heart and brain, and if enough blood and oxygen cannot reach your heart, you may suffer chest pain. If the blood supply to a portion of the heart is completely cut off by a blockage, the result is a heart attack. Blocked blood flow to the brain causes the brain to be starved of oxygen which could result in a stroke. High blood cholesterol itself does not cause symptoms, so many people are unaware that their cholesterol level is too high. We have good cholesterol and bad cholesterol. LDL (bad) cholesterol is the one that causes narrowing of the

arteries and blockage in the arteries. HDL (good) cholesterol helps keep cholesterol from building up in the arteries.

Young men are at high risk for heart disease and stroke!

Even in young females, girls' arteries look healthier than boys'. Experts believe women have naturally higher levels of good cholesterol (HDL). Men have to work harder to reduce their risk for heart disease and stroke.

- Get your cholesterol checked, beginning at age 20 and every five years.
- Control your blood pressure and cholesterol, if they're high.
- If you smoke, try to stop.
- Increase your physical activity level to 30 minutes per day, most days of the week.
- Eat more fruits and vegetables and less saturated or trans fats.

DEPRESSION AND SUICIDE: MEN ARE AT RISK

Depression is about emotions- feeling low, miserable and unhappy. It isn't just a bad mood. It causes a disturbance in the body and mind that affects your whole self and overall health. When you are depressed, it causes the brain chemicals and stress hormones to go out of balance. Sleep, appetite, and energy levels are disturbed. Research indicates that far more men present with symptoms of depression than do women and that men with depression are more likely to develop heart conditions. That may just be because men are taught from children to hide sad or unhappy feelings, to not talk about them and to just "deal with it". So often what happens is that when men are stressed or sad they show anger and aggression because that shows that they are strong and powerful. They may also cope in other ways, like drinking too much. Men are also less likely to seek help for depression. The results can be tragic. Women attempt suicide more often, but men are more successful at completing it. "Suicide is the eighth leading cause of death among all men; for young men it's higher." Females generally respond much better to being treated either with counselling and therapy or with medication, or both.

If you think you might be depressed, reach out to your friends, peer educators or someone close to you, and seek help.

DIABETES: THE SILENT HEALTH THREAT FOR MEN

The worrying fact about something like Diabetes is that it is silent and does not show symptoms for a long period. Over the years, blood sugar levels creep higher, eventually spilling into the urine. This results in the urge to go to the toilet to urinate more often. There is also the symptom of feeling thirsty all the time.

The most common symptoms of type 2 diabetes include:

Excessive thirst	✓
Frequent or increased urination, especially at night	✓
Excessive hunger	✓
Fatigue	✓
Blurry vision	✓
Sores or cuts that won't heal	✓

Type 1 diabetes signs and symptoms can come on quickly and may include:

Increased thirst	✓
Frequent urination	✓
Bedwetting in children who previously didn't wet the bed during the night	✓
Extreme hunger	✓
Unintended weight loss	✓
Irritability and other mood changes	✓
Fatigue and weakness	✓
Blurred vision	✓
In females, a vaginal yeast infection	✓

If you have these symptoms, check it out immediately!

Current trends predict that boys born in 2000 have an alarming one-in-three chance of developing diabetes in their lifetimes. Obesity also feeds diabetes. "The combination of diabetes and obesity increase the incidence of heart disease risk".

Exercise, combined with a healthy diet, can prevent type 2 diabetes. In one major study in the United States, it was found that determined efforts to lose weight (for those who needed to) and 30 minutes a day of physical activity reduced the chance of diabetes by more than 50% in men at high risk.

EAT 5, MOVE 10, SLEEP 8

EAT 5. Eat five servings of fruit and vegetables a day to boost your heart health.



<http://www.popsugar.com/fitness/How-Many-Servings-Fruits-Veggies-Do-You-Eat-Daily-3202734?ref=0>

MOVE 10. Add at least 10 minutes of moderately intense physical activity to what you do every day. It doesn't have to be elaborate — take the stairs, take a walk, just get moving. As you become more active, you can try to increase your total amount of activity a day.



<http://www.wellblog.it/il-workout-in-2-mosse-per-perdere-peso-e-tonificare-i-muscoli/>

SLEEP 8. Quality sleep is good for your heart. It can be a challenge to make time for good sleep, but it's important. At regular intervals aim for at least two weeks to get eight hours of good, quality sleep each night.

OBESITY

Obesity is when someone is very overweight. Being overweight or obese increases our chances of chronic disease, such as cancer, hypertension, cholesterol and diabetes.

South Africa has the highest overweight and obesity rate in sub-Saharan Africa. 1 in 4 girls and 1 in 5 boys between the ages of 2 – 14 years are overweight or obese.

This is no longer just an adult problem, it might seem strange to think of being overweight as a chronic disease, but carrying extra weight is not good for our health. The more extra weight we carry the worse it is for our health and the more difficult it is to lose weight and to exercise.

Obesity is associated with a number of diseases including type 2 diabetes, heart disease, stroke, hypertension (high blood pressure), joint pain and certain cancers.

SEXUALLY TRANSMITTED INFECTIONS

There is a lot of education and information about the importance of protecting against HIV and human papillomavirus (HPV) infection (the world's most common STI). But it is also very very important to do a better job of preventing and treating diseases like gonorrhoea, chlamydia and syphilis. These are all STIs. There are more than 200,000 stillbirths and babies dying in the womb every year, as well as deaths of over 90 000 new-borns because of not treating syphilis.



CANCER

LUNG CANCER: STILL A HEALTH THREAT TO MEN

Cancer of the lung is a terrible disease: ugly, aggressive, and spreads early, usually before it grows large enough to cause symptoms or even show up on an X-ray. By the time it's found, lung cancer is often advanced and difficult to cure. Generally, lung cancer is more common amongst men. Less than half of men who are diagnosed are alive a year later. This may be due to the fact that tobacco use is more common among men.

Cancer is a word used to describe many different kinds of diseases, where the cells change their form and multiply out of control. If not stopped, cancer can spread to all parts of the body. The things that you do in your daily life, such as smoking tobacco, what you eat and how much you exercise all play a role in putting you at higher or lower risk of getting cancer. Being overweight or obese greatly increases your chances of developing cancer.

Tobacco is the single biggest cause of cancer. Drinking a lot of alcohol has also been shown to increase the risk of throat, mouth, liver and breast cancers. Lung, testicular and prostate cancer are three cancers that most commonly affect men.

Risk factors for cancer include tobacco use, alcohol use, an unhealthy diet and physical inactivity; as well as infections from hepatitis B (HBV), hepatitis C virus (HCV) and Human Papilloma Virus (HPV).

Normal human cells die and are replaced over time, but cancer cells do not die off on their own, and instead they divide and grow, and infest the body.

Treatment for cancer is therefore often very harsh, to kill these hardy cancerous cells. Many cancers have a high chance of being cured if they are picked up early and treated immediately. Cancer treatment usually requires more than one type of treatment, such as surgery, radiotherapy, and chemotherapy.

Testicular cancer is more common amongst younger men aged 18 – 35. Young men can do a self-check once a month after a bath or shower. Testicular cancer is curable when detected early.

PROSTATE CANCER is one of the leading cancers in men. Prostate cancer can be prevented by having regular prostate checks to detect and treat any changes early. Early diagnosis increases your chances of successfully treating and beating the cancer. Symptoms are similar to those caused by other less serious conditions but if you experience any of the following you should visit your clinic:

- Blood in the urine
- Needing to pee a lot, especially at night
- Weak or interrupted urine flow
- Pain or a burning feeling while peeing
- Unable to pee
- Constant pain in the lower back, pelvis, or upper thighs

Men's Health

WHAT YOU NEED TO KNOW ABOUT TESTICULAR CANCER

TC = Testicular Cancer

Cases of reported testicular cancer: *

0-19 years	10	40-49 years	5
20-29 years	50	50+ years	11
30-39 years	26		

Risk Factors

- Babies born with undescended testicles
- Congenital (born with) abnormalities of a testicle, penis, kidney and inguinal hernias (hernia in groin area)
- A family history of testicular cancer
- Using marijuana (dagga or cannabis)

MYTH vs FACT

MYTH Riding a bike frequently causes TC	FACT Cycling can cause a swollen prostate but has no link to TC
MYTH TC means no more sexual desire or sex drive	FACT Only 2-3% of TC patients have tumours in both testicles. The 97-99% left with one testicle will still have sex drive
MYTH Only older men are at risk to get TC	FACT The majority of TC patients are between 15-39
MYTH Big testicles make you more prone to this cancer	FACT There is no evidence to suggest size is a risk factor

Signs & Symptoms

- A painless swelling or lump in one or both testicles
- Pain or feeling of heaviness in a testicle
- An ache in the lower abdomen, back or groin

Self Exam

- Examine your testicles after taking a warm bath or shower
- Stand in front of a mirror & look for any signs of swelling
- Support your testicles with one hand & feel each one with your other hand
- Roll the testicles between the thumb & fingers, feeling for any lumps - it's normal to feel a soft chord-like structure on the top and back of each testicle (lumps can be as small as a grain of rice & are usually quite firm)

Did you know?

TC is one of the most common cancers in males ages 15-39

IF DIAGNOSED EARLY

TC can be treated effectively via:

- Surgery
- Chemotherapy
- Radiation

Consult your health practitioner if any abnormalities are detected

CAN/SA
Cancer Association of South Africa
Research • Educate • Support

Toll-free 0800 22 66 22 www.cansa.org.za

*SA Statistics as per National Cancer Registry (NCR) 2010

About 95 percent of anal cancers, 70 percent of oropharyngeal cancers and 35 percent of penile cancers are caused by HPV type 16.

HIV

MEN AND TESTING

The Department of Health has an ambitious target of 10 million people undergoing HCT every year. We are not doing too badly; In 2012 the Department reached this target, with 10.6 million South Africans testing as part of campaigns led by the President and the Deputy President.

Over the 15 months of the HCT campaign, 14-million people have been counselled and more than 12-million have tested for HIV in the public sector. In addition, 1.5-million were tested in the private sector,” SANAC said. “This reflects a sixfold increase in the number of people testing for HIV over the previous year.

As awareness spreads we are seeing that a growing number of men are going for testing: from 19.9% in 2008 to 37.5% in 2012. This demonstrates that national efforts need to be targeted at increasing the uptake of HCT amongst men, and need to continue to communicate the need for regular testing.

The fact that men are reluctant to participate in HCT means that men are over 30% more likely to die of HIV-related infections than women.

This is a tragic and unnecessary loss to their families and loved ones. Research also shows that, of the people living with HIV in South Africa, 55% are women, yet women make up 68% of the people who start ARV treatment. Many more men than women who do go on ARVs die. This gender difference is because men living with HIV are more likely to be at an advanced disease stage when they start treatment and the treatment does not have time to take effect. Men are losing out on life saving treatment,

and our children are losing their fathers and families are losing their men unnecessarily.

WHY SOME MEN DON'T TEST

Some of the reasons that men give for not wanting to test:

- Fear that a positive diagnosis will force them to change their lifestyle, including alcohol consumption and many sexual partners.
- Reluctance to engage with young female health workers at the clinics.
- Fear that health workers will gossip about them.
- Concern that people in their community will gossip about them if they are seen at the clinic
- Fear that if they are diagnosed positive they will lose respect from other men in their communities
- Belief that real men do not show weakness and get sick
- They don't need to, because their sexual partners test regularly, and as long as these women are HIV -, the man is too. This is known as using the partner's status as a 'proxy' for their own.
- Preference for traditional healers

Many of these fears come from some men's ideas about masculinity and their belief in the qualities and behaviours they need to adopt to be a 'real' man:

- 'It's not cool to be sick and going to a clinic will make people think I'm weak
- 'I'm a man, I'm strong, HIV can't touch me!'
- 'These HIV people say I must reduce the number of lovers I have, and that would damage my image'
- 'What? I must go to the clinic and talk about my private life with a young, female nurse?'
- 'This illness kills people, if I have it, it will take away my power and status as a man'

How to do a Monthly Testicular Self-Exam



Best done after a warm shower or bath, when your scrotum is relaxed. If possible, stand in front of a mirror. Check for any swelling on the scrotal skin.



Examine each testicle with both hands. Place your index and middle fingers under the testicle with the thumbs placed on top. Firm but gently roll the testicle between your thumbs and fingers to feel for any irregularities on the surface or texture of the testicle.



Find the epididymis, a soft rope-like structure on the back of the testicle. If you are familiar with this structure, you won't mistake it for a suspicious lump.



IF YOU NOTICE ANY CHANGES OR IRREGULARITIES, THEN TELL YOUR DOCTOR IMMEDIATELY.



Testicular Self-Exam Instructions and Reminders

For our free mobile app: go to www.BallChecker.com
For monthly text reminders: text @selfexam to 81010

LESSON TWO: CONDOM USE

AIM: To highlight the benefits of correct and consistent condom use
TIME: 30 minutes

LESSON OBJECTIVES:

On completion of this lesson, participants should:

- Know the benefits of condom use for men and women
 - Know the steps to follow for correct and effective condom use
 - Be familiar and comfortable with the various terms related to condom use
-

PURPOSE OF ACTIVITIES:

- To familiarise participants with how to use a condom
 - To provide an opportunity to review the benefits of correct and consistent condom use
-

ACTIVITY: CONDOM RACE

OBJECTIVE: To provide young people with knowledge about the correct use of condoms and comfort in using words related to condom use

REQUIREMENTS: A flipchart and pens;
A copy of the 25 Steps to follow when using a male condom
Large poster board cards (x 25 A4 boards), and markers

TIME: 30 minutes

PLANNING NOTES

- Familiarise yourself with how to conduct the activity
 - Be sure you know the correct order of the steps
 - Write each step of condom use on a separate large card, one card for each step.
 - There are 10 steps in all. If there are more cards than participants, distribute them as evenly as possible.
 - Prepare/review slide for lesson.
-

PROCEDURE

1. Introduce the lesson using the introductory slide.
 2. Explain that the activity that follows will provide a fun way of exploring how to use condoms correctly.
 3. Explain that you have prepared cards for all participants and that each card lists a different step in the process of using condoms.
 4. Mix the cards up and pass them out to participants
 5. Ask them, as a group, to arrange the cards in order so that the steps in the process of using a condom correctly is sequential
 6. After the participants have established an order, have the entire group read through the steps
 7. Everyone must become comfortable in saying words like condom, penis, erection, ejaculation, etc.
 8. Process the activity, using the **discussion questions** below.
-

DISCUSSION QUESTIONS:

- Was anyone uncomfortable saying some of these words out loud? Which words?
- Why were people uncomfortable? Do they feel more comfortable now?
- What kind of message is sent when educators are uncomfortable with words?
- What steps are missing (e.g., check expiration date)?

KEY LEARNINGS

- Condoms are the only contraceptive that can prevent both pregnancy and transmission of HIV and some STIs
- Condoms are effective in reducing the risk of HIV, STIs and unwanted pregnancies when used correctly and consistently with all partners
- Choice™ condoms, undergo stringent tests by the South African Bureau of Standards to ensure that they are safe and reliable before being released to the public
- Random condoms from each batch are selected by the South African Bureau of standards and placed through a number of tests
- These include performance tests (burst, volume and pressure; freedom from holes; and package integrity) and design tests (length; width; thickness; lubricant quantity; and package material and markings)
- Only after the condoms pass these tests are they released to the public
- Condoms can only be used once and should then be safely thrown away by wrapping in toilet paper and throwing it into a dustbin



LESSON THREE: MALE MEDICAL CIRCUMCISION (MMC)

AIM: To highlight the benefits of male medical circumcision
TIME: 20 minutes

LESSON OBJECTIVES:

On completion of this lesson, participants should:

- Have gained factual information about male medical circumcision (MMC)
- Know the benefits of MMC
- Be motivated to consider MMC for themselves and family members if as yet uncircumcised
- Be able to educate others on MMC

PURPOSE OF ACTIVITIES:

- To familiarise participants with MMC
- To provide an opportunity to review the benefits of MMC

ACTIVITY: ADVOCACY PANEL – MALE MEDICAL CIRCUMCISION

OBJECTIVE: To identify concerns men may have about MMC and to explore sound factual information for enhancing knowledge and understanding, and making MMC an attractive option for men.

REQUIREMENTS: Flip chart paper and markers,
3 copies of Q&A guide and FACT SHEET (Leader's Resource) for group 1
3 copies of the QUESTIONS ONLY in the Q&A Guide for group 2

TIME: 60 minutes

PLANNING NOTES

- Familiarise yourself with how to conduct the activity
- Study the Fact Sheet related to MMC, and review the questions and answers on the Q&A Guide.
- Prepare/review a slide/s introducing MMC
- Plan seating to accommodate a panel and an audience

PROCEDURE

1. Explain to participants that this exercise is a role play involving the audience and a panel of Healthcare workers who are advocates for MMC. The job of the panel is to prepare and persuade the community that it is beneficial for the men in the community to undergo MMC
2. Ask participants what they know about MMC, and write responses on the flip chart.
3. Divide participants into two groups: group 1 and group 2
4. For the first round, give group 1 (panel members) the set of QUESTIONS ONLY and request them to take 15 minutes to review the FACT SHEET and prepare their answers
5. Give group 2 (the community members) the questions and answers in the Q&A guide and request them to review these for 15 minutes.
6. Position group 1 in front of the room, facing the community members
7. The community members then pose the questions to the panel
8. Based on what they have learnt from the FACT SHEET, and the answers they prepared, the panel answers the questions the best they can
9. Ask the 'community members' to give feedback on the quality of the answers given by the panel
10. For each question:
 - Was the information correct?
 - Was information missing, and if so what was missing?
11. Process the activity, using the **discussion questions** below.

DISCUSSION QUESTIONS:

- Was anyone uncomfortable saying some of these words out loud? Which words?
 - Ask participants to reflect on the reasons they would or would not consider MMC either for themselves or family members
 - Ask the group if they have any further questions for clarity or concluding comments, respond to these and thank them for their contributions
-

KEY LEARNINGS

- Male circumcision reduces the risk of HIV infection by 60%
- There are many reasons why men (and boys) get circumcised. In some religions, boys are circumcised soon after birth, e.g. Jewish and Islamic religions
- In some cultures, young boys are circumcised as part of the rites of passage from boyhood to manhood
- Adult men may also undergo medical male circumcision, which is the topic of this lesson
- For more detailed information, refer to the FACT SHEET in this section. For a more intensive lesson focused on male participants only, use the MEN'S HEALTH AND EMPOWERMENT PROGRAMME.

Medical Male Circumcision

Cut your risk of HIV by 60%!

Additional benefits

- ✓ reduced risk of sexually-transmitted infections
- ✓ makes penis easier to clean
- ✓ reduced risk of penile cancer
- ✓ reduced risk of cervical cancer for female partners

FREE health service

For more info, call or send a 'please call me' to **082 807 2373**

THE SOUTH AFRICAN NATIONAL AIDS SOCIETY

<http://www.tbhivcare.org/?page=news&id=105>

LESSON FOUR: CHALLENGES OF BEING A MAN IN SA

AIM: To highlight key issues related to gender stereotyping specific to South African society

TIME: 60 minutes

LESSON OBJECTIVES:

On completion of this lesson, participants should:

- Understand how socialisation influences attitudes
- Be aware of how gender stereotypes are created
- Be aware of their own attitudes and biases pertaining to gender based roles, rights and responsibilities
- Understand the link between gender stereotyping, inequality and discrimination

PURPOSE OF ACTIVITIES:

- To familiarise participants with how gender roles and responsibilities are shaped by culture and society
- To provide an opportunity for participants to reflect on their own attitudes and biases
- To demonstrate the impact of gender stereotyping on the health and wellbeing of communities
- To provide an opportunity for participants to reflect on changes that they need to make
- To identify and practice skills that are required to make the required changes in communities

ACTIVITY: ACT LIKE A MAN/ACT LIKE A WOMAN

OBJECTIVE: To demonstrate how gender roles are shaped by society

REQUIREMENTS: Flip chart paper and marker pens;

TIME: 60 minutes

PLANNING NOTES

- Familiarise yourself with how to conduct the activity
- On a flipchart write the heading: ACT LIKE A MAN
- On another flipchart write the heading: ACT LIKE A WOMAN
- Prepare/review a slide/s with the summary of learnings

PROCEDURE

1. Ask participants if they have ever been told to 'act like a man' or 'act like a woman'. Ask a few participants to share why it was said and how it made them feel
2. Ask participants to work in pairs to role play for 2 minutes what it means to 'act like a man'. One person can act out the role of the male and the other the role of the female. Afterwards, ask a few participants to share how it felt to act like a man. How did the female partner feel?
3. Use the flipchart: ACT LIKE A MAN. As a group, brainstorm how society expects men to behave. Write participants' responses inside the box. Add messages from the list below:
 - Be tough
 - Don't cry
 - Speak loudly
 - Yell at people
 - Don't show your feelings
 - Take care of other people
 - Be the protector
 - Don't show fear
 - Stay in control
 - Don't back down

- Be the boss
 - Make the rules
 - Be an authority figure
 - Earn money/Be the breadwinner
 - Have more than one girlfriend / spouse
 - Have sex when you want it /Get sexual pleasure from women
 - Travel to find work
4. Facilitate a discussion about the male gender role using the **discussion questions** provided.
-

DISCUSSION QUESTIONS:

- How do you feel about this 'gender box' that society puts males into?
 - Why is it limiting for a male to remain in his box and be expected to always behave in these ways?
 - Which feelings are males not supposed to show? Does this mean that men don't have these feelings?
 - How does this gender role affect a man's relationship with his partner and children?
 - How has it affected your relationship?
5. Use the flipchart: ACT LIKE A WOMAN. As a group, brainstorm how society expects females to behave. Write participants' responses inside the box. Add messages from the following list:
- Be passive
 - Don't argue/" backchat"
 - Be quiet and stay in the background
 - Speak softly
 - Be gentle
 - Listen to others particularly the males in the family/community
 - Be the caregiver/nurturer
 - Be the homemaker
 - Cook for the family
 - Act sexy, but not too sexy
 - Be smart, but not too smart
 - Be faithful
 - Be obedient and loyal
 - Follow the man's lead
 - Provide your man with sexual pleasure in order to keep him
 - Don't complain or refuse sex
 - Don't initiate sex
6. Facilitate a discussion about the female gender role using the discussion questions provided
-

DISCUSSION QUESTIONS:

- How do you feel about this gender box that society puts females into?
 - Why is it limiting for a female to remain in her box and be expected to always behave in these ways?
 - Which feelings are females not supposed to show? Does this mean that women don't have these feelings?
 - How does this gender role affect a women's relationship with her partner and children?
 - How has it affected your relationship with females in your life?
 - Ask participants to take 5 minutes for self-reflection and ask themselves the following questions: What would make it easier for men and women to act outside their gender boxes? What can you do to support this? What one thing can you do to change how you act towards men/women?
7. Present slides to summarise key learnings
-

KEY LEARNINGS

Gender: The way society defines the, roles, behaviours, activities, and attributes of men, women and people who are transgender. These definitions are not fixed, but change over time and from society to society.

Gender roles: Society's ideas of what it means to be a man or woman, and the different roles that men and women should play.

Gender stereotypes: Simplistic generalisations about the differences, characteristics and roles that men and women should play. Most stereotypes are built on inaccurate information about others.

Point out that men and women are not born with these characteristics or behaviours. They learn how society expects a man or woman to act from the spoken and unspoken messages they get from their family, community and society as a whole. But these gender stereotypes affect our lives in the following ways:

- Gender stereotyping creates divisions between men and women and this can lead to discrimination and inequality
- Men are led to believe that they are superior to women and that they should therefore have access to different rights, resources and power
- Women are made to believe that they are inferior to men and they 'buy' into their weaker female role. They often lack self-confidence and are highly vulnerable to violence. They have less control than men over their sex lives, and are at higher risk of getting HIV and AIDS.
- Men are afraid to show their feelings in case they are seen as weak. They feel the constant need to prove that they are 'real men'. Many use sex or violence to prove this.
- Ridicule, threats and violence are used to keep women and men in their gender roles.
- Violence is used as a way to control people in intimate relationships. This might be wives, girlfriends, husbands or casual partners.
- Sometimes culture, religion or traditional practices are used as an excuse to justify intimate partner violence
- Silence and myths about gender based violence allow this type of violence to continue
- When young people see violence as acceptable, they may also grow up to be violent
- How we see and accept or challenge violence affects others around us
- It is up to each person individually to make changes in their own environment if they would like to see a world with less violence.

Remind the group about the steps they can take if they or somebody they know has been abused.

Ask if there are any further questions or comments, and thank the participants for their contributions.



LESSON FIVE: GENDER- BASED VIOLENCE

AIM: To explore the links between gender and violence, and ways in which we can overcome obstacles and break the cycle of violence

TIME: 30 minutes

LESSON OBJECTIVES:

On completion of this lesson, participants should:

- Know the triggers for violence that are created through socialisation
 - Understand the risks, dangers and consequences of violent behaviour
 - Develop insight into the thoughts and feelings that trigger violent behaviour by males and ways to transform the behaviour
-

PURPOSE OF ACTIVITIES:

- To provide information on relevant issues pertaining to gender-based violence
 - To provide opportunities for participants to open up and discuss feelings and emotions related to this topic
 - To use methods for high impact to enable participants to realise how particular issues and situations have associated risks and consequences in terms of health and wellbeing of communities in general
-

ACTIVITY: SMALL GROUP DISCUSSION – PAUL Z’S STORY

OBJECTIVE: To provide a real life situation that participants can identify with to how the triggers for gender-based violence are created

REQUIREMENTS: Flip chart and pens
Copies of Paul’s story x6

TIME: 30 minutes

PLANNING NOTES

- Familiarise yourself with how to conduct the activity
 - Prepare copies of “Paul’s Story”
 - The story may be trigger emotional responses for people who have experienced violence or other severe trauma
 - Exercise special sensitivity when facilitating the lesson
 - Start the discussion by acknowledging that it can be quite difficult to witness the pain that the storyteller reveals
 - It is also valuable to suggest ways that people can take care of themselves during the session. For instance, some may need to step away from the group for a moment to gather their thoughts, take time out.
-

PROCEDURE

1. Hand out copies of Paul’s story (see below), and read to participants:
-

KEY LEARNINGS

- Each of us is a potential perpetrator of violence
- Gender-based violence is a significant public health issue throughout South Africa, affecting countless women, men and Children
- Change is possible in spite of circumstances
- It takes a great deal of courage to take responsibility for our own actions that have hurt others.

CAUSES OF VIOLENCE

- **ALCOHOL AND DRUG USE/ABUSE**

- o often lead to violent behaviour:
- o can cause stress in families when money is spent on alcohol and not on necessities, which in turn causes arguments and lead to violence.
- o Can sometimes cause men to become violent
- o can become an addiction and people will do anything to make sure they get their 'fix'

- **NEGATIVE MALE NORMS**

- o Men who have been brought up to believe that they are superior to women
- o Growing up in a culture where men hold authority and women are seen as inferior and under the control of men
- o When boys who are raised to think they are superior to women, they can become abusive when this power is challenged. They are also more likely to believe that it is ok to beat a woman or force her to have sex with him even if she does not want to

- **LOW SELF-ESTEEM**

- o Develops when a person does not think they are valued or important
- o Usually causes people to be deeply unhappy about themselves
- o Creates insecurity which may lead people to behave in ways that give them power, such as abusive and violent behaviour

- **ANGER MANAGEMENT**

- o Can be the cause of violent and aggressive behaviour
- o Some men are socialised to believe that anger is a natural and acceptable masculine response
- o Extreme anger is a learnt behaviour
- o We all have to learn to control our anger and manage conflict appropriately

- **JEALOUSY**

- o Is a frequent cause of violence
- o May cause a man to resort to violence if he feels threatened by his partner's real or imagined attraction to another man
- o Can be self-destructive and lead to things like murder or suicide



MEN'S HEALTH FACTS

4.4

Men live 4.4 years less than Women.

11

The last 11 years will be in poor health.



Men are more likely to die from heart disease at earlier ages.



Men are at a significantly higher risk of dying from liver disease.



80% of spinal cord injuries occur in young Men.



70% of developmental and learning disabilities affect Boys.



Men have an increased risk of dying from diabetes.



Men make up 56% of the workforce.



Yet 94% of all workforce fatalities are Men.



The top 3 reasons for reduced lifespan in Men are cardiovascular disease, suicide, and motor vehicle accidents.



Globally, alcohol kills almost 6 times more Men than Women.

Only 30% of a Man's overall health is determined by his genetics:

Having a Y chromosome is not an excuse for unhealthy lifestyle choices.



EAT 5. Eat five servings of fruit and vegetables a day to boost your heart health.

MOVE 10. Add at least 10 minutes of moderately intense physical activity to what you do every day. It doesn't have to be elaborate — take the stairs, take a walk, just get moving. As you become more active, you can try to increase your total amount of activity a day.

SLEEP 8. Quality sleep is good for your heart. It can be a challenge to make time for good sleep, but it's important. At regular intervals aim for at least two weeks to get eight hours of good, quality sleep each night.



FACT SHEET: HEALTH ISSUES FOR MEN

HIGH CHOLESTEROL LEVELS (HEART DISEASE AND STROKE)

CHOLESTEROL is fat-like substance that's found in all cells of the body. Your body needs some cholesterol to make hormones, vitamin D, and substances that help you digest foods. Your body makes all the cholesterol it needs. Your blood cholesterol level has a lot to do with your chances of getting heart disease. High blood cholesterol is one of the major risk factors for heart disease. When there is too much cholesterol (a fat-like substance) in your blood, it builds up in the walls of the arteries. Over time, this build-up causes "hardening of the arteries" so that they become narrowed and blood flow to the heart is slowed down or blocked. The blood carries oxygen to the heart and brain, and if enough blood and oxygen cannot reach your heart, you may suffer chest pain. If the blood supply to a portion of the heart is completely cut off by a blockage, the result is a heart attack. Blocked blood flow to the brain causes the brain to be starved of oxygen which could result in a stroke. High blood cholesterol itself does not cause symptoms, so many people are unaware that their cholesterol level is too high. We have good cholesterol and bad cholesterol. LDL (bad) cholesterol is the one that causes narrowing of the arteries and blockage in the arteries. HDL (good) cholesterol helps keep cholesterol from building up in the arteries.

Young men are at high risk for heart disease and stroke!

Even in young females, girls' arteries look healthier than boys'. Experts believe women have naturally higher levels of good cholesterol (HDL). Men have to work harder to reduce their risk for heart disease and stroke.

- Get your cholesterol checked, beginning at age 20 and every five years.
- Control your blood pressure and cholesterol, if they're high.
- If you smoke, try to stop.
- Increase your physical activity level to 30 minutes per day, most days of the week.
- Eat more fruits and vegetables and less saturated or trans fats.

DEPRESSION AND SUICIDE: MEN ARE AT RISK

Depression is about emotions- feeling low, miserable and unhappy. It isn't just a bad mood. It causes a disturbance in the body and mind that affects your whole self and overall health. When you are depressed, it causes the brain chemicals and stress hormones to go out of balance. Sleep, appetite, and energy levels are disturbed. Research indicates that far more men present with symptoms of depression than do women and that men with depression are more likely to develop heart conditions.

That may just be because men are taught from children to hide sad or unhappy feelings, to not talk about them and to just "deal with it". So often what happens is that when men are stressed or sad they show anger and aggression because that shows that they are strong and powerful. They may also cope in other ways, like drinking too much. Men are also less likely to seek help for depression. The results can be tragic. Women attempt suicide more often, but men are more successful at completing it. "Suicide is the eighth leading cause of death among all men; for young men it's higher." Females generally respond much better to being treated either with counselling and therapy or with medication, or both.)

**If you think you might be depressed,
reach out to your friends, peer educators or
someone close to you, and seek help.**

DIABETES: THE SILENT HEALTH THREAT FOR MEN

The worrying fact about something like Diabetes is that it is silent and does not show symptoms for a long period. Over the years, blood sugar levels creep higher, eventually spilling into the urine. This results in the urge to go to the toilet to urinate more often. There is also the symptom of feeling thirsty all the time.

The most common symptoms of type 2 diabetes include:

- excessive thirst.
- frequent or increased urination, especially at night.
- excessive hunger.
- fatigue.
- blurry vision
- sores or cuts that won't heal

Type 1 diabetes signs and symptoms can come on quickly and may include:

- Increased thirst
- Frequent urination
- Bedwetting in children who previously didn't wet the bed during the night
- Extreme hunger
- Unintended weight loss
- Irritability and other mood changes
- Fatigue and weakness
- Blurred vision

**If you have these symptoms, check it out
immediately!**

**Current trends predict that boys born in 2000
have an alarming one-in-three chance of
developing diabetes in their lifetimes. Obesity
also feeds diabetes. The combination of
diabetes and obesity increase the incidence of
heart disease risk.**

Exercise, combined with a healthy diet,

**can prevent type 2 diabetes.
In one major study in the United States,
it was found that determined
efforts to lose weight (for those who needed
to) and 30 minutes a day of physical activity
reduced the chance of diabetes by more than
50% in men at high risk.**

OBESITY

Obesity is when someone is very overweight. Being overweight or obese increases our chances of chronic disease, such as cancer, hypertension, cholesterol and diabetes.

**South Africa has the highest
overweight and obesity rate
in sub-Saharan Africa. 1 in 4
girls and 1 in 5 boys
between the ages of 2 – 14 years
are overweight or obese.**

This is no longer just an adult problem, it might seem strange to think of being overweight as a chronic disease, but carrying extra weight is not good for our health. The more extra weight we carry the worse it is for our health and the more difficult it is to lose weight and to exercise.

Obesity is associated with a number of diseases including type 2 diabetes, heart disease, stroke, hypertension (high blood pressure), joint pain and certain cancers.

CANCER

LUNG CANCER: STILL A HEALTH THREAT TO MEN

Cancer is where the cells change their form and multiply out of control. If not stopped, cancer can spread to all parts of the body. The things that you do in your daily life, such as smoking tobacco, what you eat and how much you exercise all play a role in putting you at higher or lower risk of getting cancer. Being overweight or obese greatly increases your chances of developing cancer.

**Lung, testicular, and prostate cancer
are three cancers that most
commonly affect men.**

Risk factors for cancer include tobacco use, alcohol use, an unhealthy diet and physical inactivity; as well as infections from hepatitis B (HBV), hepatitis C virus (HCV) and Human Papilloma Virus (HPV).

**Normal human cells die and are replaced over
time, but cancer cells do not die off on their
own, and instead they divide and grow, and
infest the body.**

Treatment for cancer is therefore often very harsh, to kill these hardy cancerous cells. Many cancers have a high chance of being cured if they are picked up early and treated immediately. Cancer treatment usually requires more than one type of treatment, such as surgery, radiotherapy, and chemotherapy.

**Testicular cancer is more common amongst
younger men aged 18 – 35. Young men can do a
self-check once a month after a bath or shower.
Testicular cancer is curable when detected early.**

PROSTATE CANCER is one of the leading cancers in men. Prostate cancer can be prevented by having regular prostate checks to detect and treat any changes early. Early diagnosis increases your chances of successfully treating and beating the cancer. Symptoms are similar to those caused by other less serious conditions but if you experience any of the following you should visit your clinic:

- Blood in the urine
- Needing to pee a lot, especially at night
- Weak or interrupted urine flow
- Pain or a burning feeling while peeing
- Unable to pee
- Constant pain in the lower back, pelvis, or upper thighs

**About 95 percent of anal cancers, 70 percent of
oropharyngeal cancers and 35 percent of penile
cancers are caused by HPV type 16.**

WHAT YOU NEED TO KNOW ABOUT TESTICULAR CANCER

TC = Testicular Cancer

Cases of reported testicular cancer: *

0-19 years	10	40-49 years	5
20-29 years	50	50+ years	11
30-39 years	26		

Risk Factors

- Babies born with undescended testicles
- Congenital (born with) abnormalities of a testicle, penis, kidney and inguinal hernias (hernia in groin area)
- A family history of testicular cancer
- Using marijuana (dagga or cannabis)

Did you know?

TC is one of the most common cancers in males ages 15-39

IF DIAGNOSED EARLY

TC can be treated effectively via:

- Surgery
- Chemotherapy
- Radiation

MYTH vs FACT

Riding a bike frequently causes TC



Cycling can cause a swollen prostate but has no link to TC

TC means no more sexual desire or sex drive



Only 2-3% of TC patients have tumours in both testicles. The 97-98% left with one testicle will still have sex drive

Only older men are at risk to get TC



The majority of TC patients are between 15-39

Big testicles make you more prone to this cancer



There is no evidence to suggest size is a risk factor

Signs & Symptoms

- A painless swelling or lump in one or both testicles
- Pain or feeling of heaviness in a testicle
- An ache in the lower abdomen, back or groin

Self Exam

- Examine your testicles after taking a warm bath or shower
- Stand in front of a mirror & look for any signs of swelling
- Support your testicles with one hand & feel each one with your other hand
- Roll the testicles between the thumb & fingers, feeling for any lumps - it's normal to feel a soft chord-like structure on the top and back of each testicle (lumps can be as small as a grain of rice & are usually quite firm)



Consult your health practitioner if any abnormalities are detected



Research • Educate • Support



Toll-free 0800 22 66 22 www.cansa.org.za

How to do a Monthly Testicular Self-Exam



Best done after a warm shower or bath, when your scrotum is relaxed. If possible, stand in front of a mirror. Check for any swelling on the scrotal skin.



Examine each testicle with both hands. Place your index and middle fingers under the testicle with the thumbs placed on top. Firm but gently roll the testicle between your thumbs and fingers to feel for any irregularities on the surface or texture of the testicle.



Find the epididymis, a soft rope-like structure on the back of the testicle. If you are familiar with this structure, you won't mistake it for a suspicious lump.



IF YOU NOTICE ANY CHANGES OR IRREGULARITIES, THEN TELL YOUR DOCTOR IMMEDIATELY.



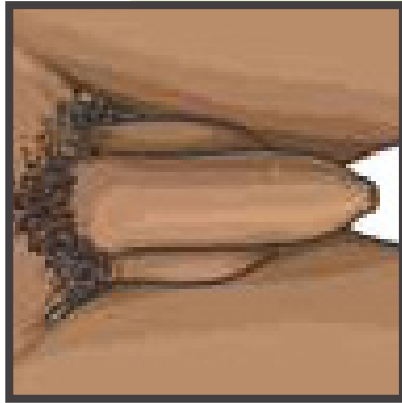
Testicular Self-Exam Instructions and Reminders

For our free mobile app: go to www.BallChecker.com

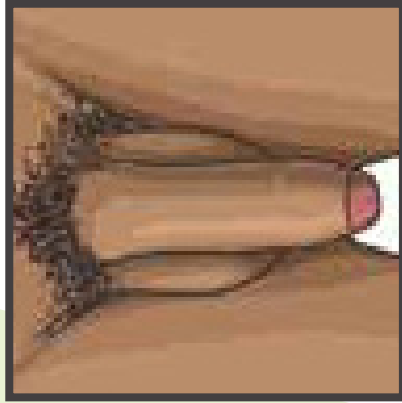
For monthly text reminders: text @selfexam to 81010

MEDICAL MALE CIRCUMCISION

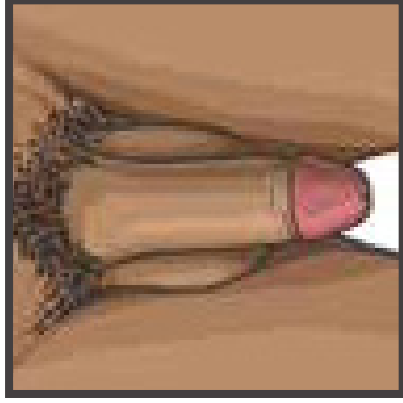
Medical male circumcision is the full removal of the penis foreskin, fully exposing the head of the penis. It is a simple and safe procedure that is performed at community health centres and district hospitals by a qualified doctor or nurse. Medical male circumcision (MMC) is different from traditional circumcision, which is a cultural practice that may involve partial removal of the foreskin, or a small slit in the foreskin; and which may not have the same health benefits as the medical procedure. MMC helps to reduce the chance of men contracting HIV through having vaginal sex by up to 60%. The foreskin on the penis contains small cells that trap the HIV virus. The inner skin of the foreskin is prone to tearing during sex, and it is more likely to attract HIV than any other skin. If the foreskin is removed, there is less chance of men contracting HIV. Removing the foreskin also makes the penis easier to keep clean, and this lowers the risk of men getting other sexually transmitted infections and genital ulcers. It also reduces the risk of cancer of the penis. Medical male circumcision provides no direct protection for women against HIV. The main health benefit of medical male circumcision for women is better hygiene, as a man who is circumcised is less likely to have bacteria or viruses trapped under the foreskin, and this reduces the chance of a woman getting other sexually transmitted infections through unprotected sex. Medical male circumcision also reduces a woman's risk of cervical cancer by removing the human papillomavirus (HPV) that is often carried in the foreskin.



Uncircumcised



Partially Circumcised



Fully Circumcised

How to use a Male Condom



1 Check the expiry date and make sure the package is sealed with no air escaping from it.



2 The penis must be erect before you roll the condom on.



3 Open the package, make sure not to damage the condom. Do not use teeth or nails for this.



4 For an uncircumcised penis, make sure the foreskin is pulled back.



5 Make sure the condom is the right way out in order to "unroll".



6 Pinch the air out of the tip of the condom as this may cause a bubble which can burst while having sex.



7 Unroll the condom as far as it will go to cover the shaft of the penis.



8 Use only water based lubricants such as KY Gel. Non-water based lubes may cause condom breakage. Only apply lube after the condom is on - never before.



9 When pulling out hold the condom at the base and pull out while you are still erect to stop it from slipping off your penis.



10 Wrap the condom in toilet paper and throw away in the rubbish bin. Do not flush it down the toilet.

1. What is Male Medical Circumcision (MMC)?

Answer: Male Medical Circumcision is the complete removal of the foreskin.

2. Why should a man undergo MMC?

Answer:

- It is the best option for your sexual and reproductive health.
- It reduces the risk of HIV infection.
- It is the best method to reduce your risk of sexually transmitted infections such as syphilis and gonorrhea.
- It reduces your risk of penile cancer and your partner's risk of cervical cancer.
- Infections (STIs) and diseases.
- It ensures better hygiene.

3. What male circumcision does not do?

Answer:

- Does not prevent pregnancy.
- Does not benefit the partner of an HIV-positive man.
- Does not reduce the risk of HIV infection during anal sex.

4. Where can a man go for a male medical circumcision?

Answer:

Male Medical Circumcision takes place in local clinics, community health centres and district hospitals. A specially trained doctor or nurse will do the procedure.

5. What happens to a man when he decides to have a male medical circumcision?

Answer:

At the health facility the man will be counselled and will be examined by the nurse or doctor to make sure that a circumcision can be done on him. It will be recommended that he has an HIV test. He will be given a local anaesthetic. The procedure takes about 30 minutes. The wound is stitched and he will be told how to keep the wound clean. Then he can go home.

6. How long is the healing process?

Answer:

The wound takes about six weeks to heal. The man may not have sex or masturbate during this time otherwise the wound will take longer to heal.

7. Does a man still need to use a condom after circumcision?

Answer:

Yes, the man must use a condom to reduce unwanted pregnancy and to reduce the transmission of HIV and STI's. The man's sexual ability will be the same as before the circumcision.

8. If a man has already been circumcised as part of his culture or religion, does he need another circumcision?

Answer:

Many cultures and religions only partially circumcise the penis, however, for full health benefits a medical circumcision is recommended.

9. Who can go for a medical male circumcision?

Answer:

Provided informed consent for the procedure is obtained by the medical practitioner, parents can have their male babies and children circumcised at various health care facilities. Men from 15 to 49 years can also visit their nearest clinic to undergo the procedure.

10. Can a man be circumcised if he is HIV-positive?

Answer:

Yes, HIV-positive men can be circumcised, however this does not protect your partner from contracting HIV and you should continue to use condoms at all times (even after being circumcised).

TIPS

- This Lesson can be facilitated for mixed groups. However, should the group be males only, a more intensified version can be found in the Programme – MEN'S HEALTH AND EMPOWERMENT.
- The group discussion and/or counselling must be appropriate and sensitive to the group circumstances. This includes culture, language, sexual orientation, religion and age. It may work best to counsel men in groups that share some commonalities.
- There should be no discrimination against clients on the basis of HIV status, colour, sex, language, religion, sexual orientation or sexual behaviour.
- Participants are encouraged to share their views, ask questions and share information from the session with

MALE MEDICAL CIRCUMCISION BENEFITS

- Improves hygiene as it is easier to keep the penis clean and free of germs
- Reduces the risk of HIV infection from a woman to a man
- Reduces the risk of some STIs such as herpes, chancroid, human papilloma virus and syphilis
- Reduces the risk of penile cancer
- Reduces the risk of women getting cervical cancer as the Human Papilloma virus that is mostly associated with cervical cancer is more common amongst uncircumcised men

HOW DOES MMC REDUCE HIV RISK?

- The skin inside the foreskin is soft and prone to bruising and tearing during sex and this provides an entry point for HIV.
- There are cells underneath the foreskin (Langerhans cells) that attract the HI virus. Therefore by removing the foreskin, these cells are also removed.

PARTIAL CIRCUMCISION OR NO CIRCUMCISION HAS NO HEALTH BENEFITS

- The foreskin covers the head of the penis, and it is more difficult to keep clean.
- The skin inside the foreskin is soft and prone to bruising and small cuts during sex, which allows an entry point for HIV and can cause discomfort during sex.
- Higher risk of STIs and HIV.

IMPORTANT:

- Being circumcised does not mean that one can or should stop using condoms. Medical male circumcision does not provide 100% protection against HIV it only reduces the risk of HIV infection. There is still a 40% chance that men who are circumcised can get HIV.
- Medical male circumcision (MMC) does not reduce the risk of HIV infection if you have anal sex.
- Medical male circumcision does not reduce the risk of HIV infection to your female partner if a man is already HIV-positive.
- Medical male circumcision does not prevent unwanted pregnancies.

HANDOUT: PAUL'S STORY

I was the first born child in my family. My father being a boxer, he taught me how to defend myself from other boys. The first sport I played was karate; then as I grow up I lifted irons, body building. There was this girl I loved very much. It was still my early years of dating and I was still learning all these things, how to treat a woman. One night I found her with another guy. I hit her in front of that guy and as I was hitting her I said words like: *"You see what you made me to do. You see the monster you created in me. Now you'll have to live with it!"*

I thought I was the victim. Then the relationship ended. Things were back to normal for a while, until I met this young beautiful girl. Her love for me was something she couldn't hide. I thought to myself: "No, she's still very young, I can't take advantage of her. She does not know what she's doing." Everyone could see that she loved me and in a way I loved her too. That one night happened: she got pregnant. She wanted to do abortion and it was against our belief; I begged her not to. We must dance to the music we made.

The truth is, I was scared too. I saw her maybe once a week. Things were no longer the same. She cheated on me now while pregnant, which made me doubt that the baby she's carrying was mine. I was very angry. I caught her one night and the anger I permitted in me busted out. I hit her, repeating those words: *"You see what you made me to do. You see the monster you created in me. Now you'll have to live with it!"*

I started ordering her around: *"As long as you still pregnant with my child, you'll do what I tell you."* I used power to try and make her do what I wanted, but my power was not meant for that, but to protect. We were created to take care of women and children and not exercise our power or strength on them. I had a choice but I chose to be abusive, a decision I regret. I am taking care of our baby... but it's just not enough to raise a child without a mother. Taking a child to the clinic alone and having to face people staring at you and whispering "where's the mother?" It would have been better if she had walked out on us just by herself, and not because of my actions.

We all have a choice and we are what we choose to be. Be careful of the choices you make!

QUESTIONS		GROUP
1.	How might Paul's own background have contributed to his choice to use violence?	
2.	When Paul found his girlfriend with another man, what other ways might he have responded?	
3.	Think about the words "You see what you made me to do." Who is really responsible for Paul's behaviour?	
4.	What steps could Paul and his girlfriend have taken to avoid an unplanned pregnancy? Whose responsibility was it to take these steps?	
5.	Paul admits that he was scared when he found out his girlfriend was pregnant. Discuss what his fears might have been, and how he could have dealt with them in a positive way.	
6.	How do you think his girlfriend felt when Paul started seeing her less and less during her pregnancy?	
7.	What challenges could Paul be facing as a result of his choice to be a single father? How might his peers and his community help or hinder him?	
8.	What are the responsibilities of a father to a child? How is Paul acting to change himself and provide a good life for his child?	

NOTES

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ABBREVIATIONS

AIDS:	Acquired Immune Deficiency Syndrome	HIV:	Human Immuno-Deficiency Virus
ARVs:	Antiretrovirals	HPV:	Human Papillomavirus Virus
HBV:	Hepatitis B Virus	LGBTI:	Lesbian, Gay, Bisexual, Transgender, Intersex
HCT:	HIV Counselling and Testing	STIs:	Sexual Transmitted Infections
HCV:	Hepatitis C Virus	STDs:	Sexual Transmitted Diseases
HEAIDS:	Higher Education and Training HIV/AIDS Programme	TB:	Tuberculosis

CRUCIAL CONVERSATIONS



Hi, what's up? You need some help?

Hi...er...it's okay,
I see you're busy.



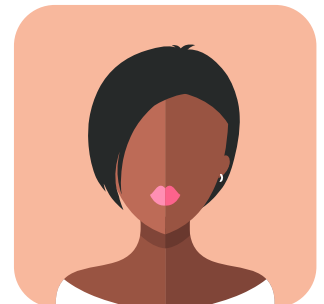
No, no, please come in. I'm done.
What can I help with?

It's okay. I changed my mind



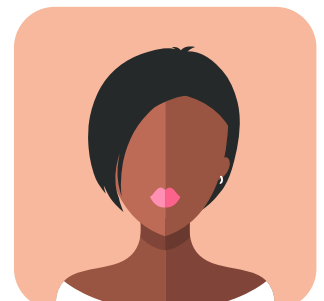
Hey, hey, hey...Look, you seem really nervous
and scared. And that tells me something's really
troubling you and you need help right? It's okay, I
can try. You're safe here. what's your name?

It's Akona



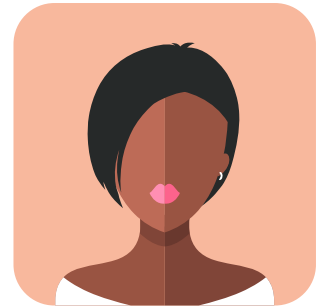
Akona, I get how difficult it is to talk to someone you don't
know well or at all. Especially if it's personal. But hey,
sometimes it's easier when it's someone you don't know.
And you came here right? I can't force you but it will be a
shame for you to go away without being helped in any way

Well ...do you guys like write
everything down? And talk
to everyone in the office?



Well, I do need to make notes but
I signed a pledge so I won't blab.
Unless it's something that is life
threatening. Then I don't have a choice

Please I wanna talk but can you not write my name down. I'm so confused right now. I need to tell someone. Anyone. You know you guys spoke about all that gay stuff last week?



Yeah, so you were at the talk?



Yeah. I think I'm lesbian...but I don't know for sure. What do I do? I'm scared. I feel sick. My dad will die...no I can't be...it's impossible



Hey, easy now. Sit down and let's just talk. What's been happening to make you feel this way?



Well, I kinda feel "turned on" by other women. I feel different from all my girlfriends, like I don't fit in sometimes. When they're checking out the guys, I find myself checking out other women. Am I normal? Am I sick?



The good news is that you're normal. There are many others who feel that way? Didn't you get that at the talk?



No, I didn't stay. I felt like everyone would know. So I left. But I don't think I am - look at me. I don't have like big muscles or anything. I don't even look like a boy! I'm a girl! Everyone says I'm beautiful and the guys - they just fall like flies. But I feel nothing. I never have, even in school. Omg! I just wanna die...what if anyone finds out? I heard they gang raped a lesbian from Res last year. I'm so scared...I can't breathe... omg! This can't be happening



AIM

To review accurate and relevant information about each of the key health risks pertaining to LGBTI persons and address stigma and discrimination of the LGBTI community.

OVERALL MODULE OBJECTIVES

By the end of this module, participants will:

- Be aware of general and specific issues pertaining to health needs/challenges and risks faced by the LGBTI community
- Be sensitised to LGBTI issues (coming out, coming to terms with sexual identity and sexual orientation)
- Have addressed myths and misconceptions to mitigate stigma and discrimination of gay and lesbian people and those belonging to other sexual minority groups
- Will understand the health consequences of prejudice and discriminatory behaviour such as hate crime

MODULE CONTENT

The module has 4 lessons which focus on 3 targeted areas:

- Human sexuality
- Overview of LGBTI health
- Myths and misconceptions related to LGBTI
- Stigma and discrimination

NOTES

LESSON ONE: INTRODUCTION TO SEXUALITY

AIM: To review where LGBTI issues fit within the broader context of human sexuality
TIME: 20 minutes

LESSON OBJECTIVES:

On completion of this lesson, participants should:

- Know the meaning of human sexuality
 - Be familiar with terminology related to human sexuality
 - Understand where LGBTI sexuality fits within the context of human sexuality
-

PURPOSE OF ACTIVITIES:

- To provide information on relevant issues pertaining to human sexuality
 - To provide opportunities for participants to demonstrate existing knowledge
 - To use participatory methods to encourage participants to talk about sexuality issues
-

ACTIVITY: SMALL GROUP TASK

OBJECTIVE: To review where LGBTI issues fit within the broader context of human sexuality
REQUIREMENTS: A flipchart and pens
Copies of the human sexuality handout for groups
Slides on human sexuality
TIME: 30 minutes

PLANNING NOTES:

- Familiarise yourself with all aspects of the topic
 - Prepare slides
 - Make copies of handout
-

PROCEDURE:

1. Introduce the lesson
 2. Divide participants into groups
 3. Distribute copies of handout on human sexuality (leader's resource) to groups
 4. Instruct them to complete the diagram in the handout by filling in the different categories of human sexuality (10 mins)
 5. Go through diagram in plenary inviting different groups to input on aspects that are missing
 6. Present slides on human sexuality
 7. Process the activity, using the **discussion questions** below.
-

DISCUSSION QUESTIONS:

- Was anyone uncomfortable saying some of these words out loud? Which words?
- Why were people uncomfortable?
- How does socialisation impact on sexuality?
- Did you learn anything new?
- How do you feel about what you learned?

KEY LEARNING

- Sexuality is a central aspect of being human throughout life and encompasses sex, gender identities and roles, sexual orientation, eroticism, pleasure, intimacy and reproduction. Sexuality is experienced and expressed in thoughts, fantasies, desires, beliefs, attitudes, values, behaviours, practices, roles and relationships
- Biological sex refers to the scientific concept that categorises individuals as either male or female.
- Biologically male: has a penis, testicles, XY chromosomes, and produces testosterone
- Biologically female: has a vagina, ovaries, XX chromosomes, and produces oestrogen
- Intersex people are born with chromosomes, external genitalia, and/or internal reproductive systems that are not considered “typical” for either males or females.
- Sexual Orientation is the way in which one expresses oneself, sexually in relation to others. The three sexual orientations are: homosexual, bisexual and heterosexual (straight).
- Sexuality includes 5 interrelated, components: biological sex, gender identity, social gender role, sexual orientation, and sexual practice or behaviour.
- A person’s gender role is largely culturally and socially determined.
- Gender identity is how someone identifies with women or men, i.e. a person’s sense of being either male or female.
- Transvestites refer to men, usually heterosexuals, who enjoy wearing female clothes and adopt traditionally female character traits. Transvestites generally self-identify as men and have no interest in being women.

WHY HUMAN SEXUALITY?

Why is it important to understand human sexuality and sexual behaviour? Many of the main individual health risk factors of both heterosexual and homosexual individuals are associated with sexual behaviour. Human behaviour, particularly sexual behaviour, is complex. Therefore, an introductory understanding of human sexuality will provide the necessary background required to further understand sexual orientation.

DEFINITION OF HUMAN SEXUALITY

We all need to understand ourselves completely and holistically as sexual, social beings. To begin with, we must be clear about what we mean when we talk about human sexuality. According to the World Health Organisation (WHO) the working definition of sexuality is: ‘a central aspect of being human throughout life and encompasses sex, gender identities and roles, sexual orientation, eroticism, pleasure, intimacy and reproduction. Sexuality is experienced and expressed in thoughts, fantasies, desires, beliefs, attitudes, values, behaviours, practices, roles and relationships. While sexuality can include all of these dimensions, not all of them are always experienced or expressed. Sexuality is influenced by the interaction of biological, psychological, social, economic, political, cultural, ethical, legal, historical and religious and spiritual factors.’

Human sexuality can be broadly broken down into five concepts: biological sex, gender identity, sexual orientation, sexual identity, and sexual behaviour.

Society has a tendency to build little boxes for people and they are placed there based on society’s definitions. These often don’t take into account the individual in society and the fact that we are all unique beings with unique identities. Just as we have a unique finger print, we have a unique social and sexual identity. One can never know someone’s sexual identity based on their biological sex, gender identity, gender role, sexual orientation, or sexual behaviour.

WHAT IS BIOLOGICAL SEX?

Biological sex refers to the scientific concept that categorises individuals based on certain characteristics like their chromosomes, internal and external genitalia, and their hormonal profile. A person’s sex is usually categorised as either male or female. So when you’re completing documentation/application forms and it has a section that says “Sex”, this asks not whether you have sex or not but refers to whether you are identified as **male or female**. We’re all born with a particular biological gender identity – that moment when parents wait with bated breath and the doctor or midwife holds up the baby and proclaims “It’s a boy” or “it’s a girl”

Biologically Male: has a penis, testicles, XY chromosomes, and produces testosterone

Biologically Female: has a vagina, ovaries, XX chromosomes, and produces oestrogen

And then we have what is referred to as a “social emergency” simply because there is no third box on your application form. It is estimated that 1 in 2000 babies are born “intersex” where it is difficult to clearly see whether their genitalia are male or female.

WHAT IS INTERSEX?

Intersex people are born with chromosomes, external genitalia, and/or internal reproductive systems that are not considered “typical” for either males or females. For example, instead of an XX or XY set of chromosomes, a person may be born with an XXY set. Or instead of a penis or vagina, a person may be born with ambiguous genitalia. Intersex is not uncommon in South Africa. When discovered at birth in most Westernised countries, unnecessary cosmetic surgery is performed on the majority of intersex babies to ensure that they conform to either a male or female biological identity. These surgeries often require multiple follow-up repair surgeries and have many complications. Adult intersex people are often haunted by a lifetime of these traumatic procedures, robbing them of sexual sensations, and denying them the ability to feel present and safe in their own bodies. These ‘corrective’ surgeries remove any choice intersex individuals have in deciding their biological gender which can leave many mentally and emotionally traumatised.

Intersexuality challenges the notion that human sex is either male or female. Intersexuality shows that biological human sex is actually a spectrum with male on one end, a variety of intersexualities in the middle, and female on the other end. It is very important not to confuse the term intersex (a biological concept) with being transgender or bisexual (a gender identity concept), being gay (a sexual orientation), or expressing transvestite behaviours/practices.

SEXUAL ORIENTATION

Sexual Orientation is the way in which one expresses oneself, sexually, in relation to others. The lasting emotional, romantic, sexual or intimate feelings one has for individuals of a specific gender at all levels (psychological, physical, intellectual and spiritual). The three sexual orientations are: **homosexual, bisexual and heterosexual (straight)**. The homosexual orientation has two identities, that of **gay man or lesbian woman**. A gay man is a man who has romantic, sexual and intimate feelings for (or a love relationship with) another man (or men) and *identifies with being gay*. A lesbian woman (some do not like the word, so prefer to be called gay women) is a woman who has romantic, sexual and intimate feelings for (or a love relationship with) another woman (or women) and *identifies with being lesbian (or a gay woman)*. A bisexual is an individual who has romantic, sexual, intimate feelings for (or a love relationship with) someone of the same sex and/or with someone of the opposite sex. *Note, being bisexual doesn't mean that they will have these feelings at the same time or with an equal amount of attraction to both sexes. Or that these individuals are promiscuous.* It is important to note here that one's sexual orientation is not necessarily associated with one's biological sex, gender identity, gender role, or even sexual practice.

Lastly, it is important to state that being bisexual is in no way related to being intersex or transgender.

SEXUAL PRACTICES

This refers to a range of behaviours between members of the same or opposite sex. Once again this relationship is not clear cut or as simple. For example, the term MSM (men who have sex with men) became popular. This broad term applies to heterosexually, homosexually, and bisexually self-identified men who sometimes or often engage in sexual activity with other men. This broad term shows how complicated human sexuality is.

According to the World Health Organisation (WHO, 2002) sexuality may be defined as a ‘central aspect of being human throughout life and includes sex, gender identities and roles, sexual orientation, pleasure, intimacy and reproduction’.

‘Sexuality is experienced and expressed in thoughts, fantasies, desires, beliefs, attitudes, values, behaviours, practices, roles and relationships. Sexuality is influenced by the interaction of biological, psychological, social, economic, political, cultural, ethical, legal, historical and religious and spiritual factors.’ Within this broad working definition is a more specific definition of sexuality or sexual identity, which includes **5 interrelated, yet distinct, components** that contribute to an awareness of oneself as a sexual being. These 5 dimensions include **biological sex, gender identity, social gender role, sexual orientation, and sexual practice or behaviour** (see diagram below for a schematic overview of these dimensions). The relationship between these dimensions remains complex and largely unclear. The more we learn the more we realise that, when it comes to human sexuality, there are essentially no basic rules or prescriptions that can apply to all. What follows is a description of each dimension.

SOCIAL GENDER ROLE

A person's gender role is largely culturally and socially determined. As such, men are typically expected to be masculine (e.g., boys don't cry, and men must be strong, sporty, competitive, rational and, in many cases, aggressive), while women are typically expected to be feminine (e.g., girls must play with dolls, and women must be submissive, emotional, nurturing and, in many cases, domesticated). These cultural norms, standards, and expectations are usually adopted at a very early stage via one's family (starting with the ‘innocent’ gesture that baby girls should wear pink and baby boys should wear blue), and reinforced later on through the media, education, religious institutions, one's peers, and various other social networks.

GENDER IDENTITY

Gender identity is how someone identifies with women or men, i.e. a person's sense of being either male or female. While most people's gender matches their biological sex, this is not always the case. For instance someone may be born biologically male, yet have a female gender identity.

It is important to understand gender presentation in this context. There are people whose gender identity cannot be classified as the above. Transsexuals are people whose gender does not match their sex. For example a person who is biologically male but feels like a female. Transsexuals often have a feeling of being "born in the wrong body". The terms used in this context are MTF (male to female) or FTM (female to male).

TRANVESTITES

Transvestites refer to men, usually heterosexuals, who enjoy wearing female clothes and adopt traditionally female character traits for personal satisfaction, which may take the form of sexual arousal and/or gratification, but may be of a non-sexual nature. *Transvestites* generally self-identify as men and have no interest in being women.



LESSON TWO: WHAT'S SO SPECIAL ABOUT LGBTI HEALTH?

AIM: To review the issues related to LGBTI health including regular screening and the various health conditions that LGBTI persons are at risk for.

TIME: 30 minutes

LESSON OBJECTIVES:

On completion of this lesson, participants should:

- Know the health issues for LGBTI persons as identified by the World Health Organisation (WHO)
- Be familiar with screening tests available for both communicable (diseases that can be passed from one to another) and non-communicable diseases
- Be familiar with the various health challenges that LGBTI persons are susceptible to

PURPOSE OF ACTIVITIES:

- To provide information on relevant issues pertaining to LGBTI health
- To provide opportunities for participants to demonstrate existing knowledge
- To use methods that are interesting and enable participants to understand the risks and consequences of issues such as stigma and discrimination in terms of LGBTI health

ACTIVITIES: GROUP DISCUSSION / BRAINSTORM / SLIDES

OBJECTIVE: To create opportunities for participants to become informed of key issues related to LGBTI health

REQUIREMENTS: Flip chart paper, markers, slides

TIME: 30 minutes

PLANNING NOTES:

Prepare/review slides for:

- The priority health issues for LGBTI persons
- HIV and STIs and the LGBTI community
- Health services available for LGBTI individuals
- Screening tests that LGBTI persons should do
- Specific issues pertaining to lesbian health

PROCEDURE:

1. Introduce the subject of LGBTI health and ask "so what's so special about LGBTI health?"
2. Note all points on the flipchart
3. Present slides and be sure to only fill in gaps
4. Facilitate the popcorn exercise i.e. people to randomly "pop up" and say one thing they learnt from the lesson
5. Summarise the lesson using the **discussion questions** provided

DISCUSSION QUESTIONS:

- Ask participants if they have any further questions about LGBTI Health?
- Did they learn anything new?
- Did new information change the way they think about LGBTI health/community?
- How did the information impact on them?

KEY LEARNING

- There are many reasons and that prevent LGBTI individuals from accessing testing, prevention and treatment services, and support from friends and family.
- Compared to other men, gay, bisexual and other men who have sex with men are additionally affected by:
 - o Higher rates of HIV and other sexually transmitted diseases (STDs)
 - o Tobacco and drug use
 - o Depression
- Other factors that can negatively impact your health and ability to receive appropriate care:
 - o Homophobia
 - o Stigma (negative and usually unfair beliefs)
 - o Discrimination (unfairly treating a person or group of people differently)
 - o Lack of access to culturally and orientation-appropriate medical and support services
 - o Heightened concerns about confidentiality
 - o Fear of losing one's job
 - o Fear of talking about your sexual practices or orientation



WHAT'S SO SPECIAL ABOUT LGBTI HEALTH?

There are many reasons that prevent LGBTI individuals from accessing testing, prevention and treatment services, and support from friends and family:

- The large percentage of gay, bisexual, and other men who have sex with men who have HIV and STDs means that, as a group, they have a higher chance of being exposed to these diseases
- Too many men don't know their HIV or STD status (if they have a disease or not), which means they may not get medical care and are more likely to unknowingly spread these diseases to their sexual partners
- Hence, compared to other men, gay, bisexual and other men who have sex with men are additionally affected by:
 - o Higher rates of HIV and other sexually transmitted diseases (STDs);
 - o Tobacco and drug use;
 - o Depression.

The CDC recommends sexually active gay, bisexual, and other men who have sex with men test for:

- HIV (at least once a year)
- Syphilis
- Hepatitis B
- Chlamydia and gonorrhoea of the rectum if you've had receptive or penetrative anal sex or in the past year
- Chlamydia and gonorrhoea of the penis (urethra) if you have had penetrative anal sex (been on the "top") or received oral sex in the past year
- Gonorrhoea of the throat if you've given oral sex (your mouth on your partner's penis, vagina, or anus) in the past year

Other factors that can negatively impact your health and ability to receive appropriate care:

- Homophobia
- Stigma (negative and usually unfair beliefs)
- Discrimination (unfairly treating a person or group of people differently)
- Lack of access to culturally and orientation-appropriate medical and support services
- Heightened concerns about confidentiality
- Fear of losing your job
- Fear of talking about your sexual practices or orientation

WHY ARE SCREENING TESTS IMPORTANT?

Screening means prevention! It's that simple. There are different types of **prevention**:

PRIMARY PREVENTION means that checking on your health regularly can help to prevent you from getting serious life-threatening or less serious illnesses.

SECONDARY PREVENTION means that if you screen and find you've already been infected with some virus or have developed a life-threatening condition, you can get treatment and manage your lifestyle to prevent it from becoming more serious

TERTIARY PREVENTION means that when you've caught it too late and it's already serious, you can get treatment to manage pain and side-effects.

WHAT SERVICES ARE AVAILABLE TO LGBTI INDIVIDUALS?

- HIV Counselling and Testing (HCT)
- Sexually Transmitted Infection (STI) screenings and treatment
- Tuberculosis (TB) screenings
- Provision of Post Exposure Prophylaxis (PEP)
- Anti-Retroviral Treatment (ART)
- HIV management (CD4 and viral load)
- Pap smear
- Basic wound care and clean needles and syringes
- General medical, sexual health and safer sex consultations
- Individual and couples counselling
- Condoms and lubrication

Breast Cancer

- Lesbians are likely to have risk factors for breast cancer yet less likely to get screening exams. This combination means that lesbians may not be diagnosed early when the disease is most curable.

Depression/ Anxiety

- Lesbians may experience chronic stress from discrimination. This stress is worse for women who need to hide their orientation as well as for lesbians who have lost important emotional support because of their orientation. Living with this stress can cause depression and anxiety.

Heart Health

- Heart disease is the leading cause of death for women. Smoking and obesity are the biggest risk factors for heart disease among lesbians. All lesbians need yearly medical exams for high blood pressure, cholesterol problems, and diabetes. Health care providers can also offer tips on quitting smoking, increasing physical activity, and controlling weight.

Gynecological Cancer

- Lesbians have higher risks for certain types of gynaecological cancers compared to straight women. Having regular pelvic exams and pap tests can find cancers early and offer the best chance of cure.

Tobacco

- Research also shows that lesbians use tobacco more often than heterosexual women do. It is easy to get addicted to smoking, even if smoking if it's only done socially. Smoking has been associated with higher rates of cancers, heart disease, and emphysema — three major causes of death among women.

Alcohol

- Heavy drinking and binge drinking are more common among lesbians compared to other women. While one drink a day may be good for the heart, more than that can raise your risk of cancer, liver disease and other health problems.

Substance Use

- Lesbians may use drugs more often than heterosexual women. This can be due to stress from homophobia, sexism, and/or discrimination. Lesbians need support to find healthy ways to cope and reduce stress.

Intimate Partner Violence

- Contrary to stereotypes, some lesbians experience violence in their intimate relationships. However, health care providers do not ask lesbians about intimate partner violence as often as they ask heterosexual women. Lesbians need to be asked about violence and have access to welcoming counseling and shelters when needed.

Sexual Health

- Lesbians can get the same sexually transmitted infections (STDs) as heterosexual women. Lesbians can give each other STDs by skin-to-skin contact, mucus membrane contact, vaginal fluids, and menstrual blood. It is important for sexually active lesbians to be screened for STDs by a health care provider.

I am Gay: 5 things I fear



It shouldn't be like this...

*MSM is the acronym for men who have sex with men



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LESSON THREE: MYTHS AND MISCONCEPTIONS

ACTIVITY: STOP! WAIT A MINUTE..

- OBJECTIVE:** To address myths and misconceptions and stereotypes relating to the LGBTI community
To provide more information about LGBTI issues
- REQUIREMENTS:** Flip chart paper and marker pens
Handout: myth information
- TIME:** 30 minutes
-

PLANNING NOTES:

- Familiarise yourself with the correct information pertaining to myths and facts. Given the intensity of feelings about sexual orientation this can be a very challenging session to conduct. The facilitators must have:
 - Knowledge about same gender relationships - current findings, statistics and research
 - Self-awareness
 - Comfort with the subject matter
 - An attitude of acceptance regarding homosexual and bisexual orientations.
 - If this session is poorly facilitated, it could actually reinforce and/or increase feelings of homophobia amongst participants. As a first step in preparation, ask yourself the following questions:
 - How comfortable am I with the issue and the content?
 - How comfortable am I with my own sexual orientation?
 - How will I react if a participant "comes out"?
-

PROCEDURE:

- Divide participants into 3 small groups and number them 1 to 3
 - Allocate the following questions per group as follows:
 - All Groups: Describe a homosexual / describe a heterosexual
 - How does he/she look?
 - How does he/she dress?
 - What jobs are they good at?
 - What kinds of sexual behaviour do they engage in?
 - Describe the level of risk for HIV
 - Facilitate plenary discussion using **discussion questions** provided
-

DISCUSSION QUESTIONS:

- When did you choose your sexual orientation?
- Will you be offended if someone from the same gender "comes on to you"?
- How easy would it be if you are heterosexual and wanted to change your sexual orientation?
- How easy would it be to tell anyone if you did?

KEY LEARNINGS

For the peer education programme, destigmatising LGBTI students means:

- Establishing a safe and supportive environment
- Objecting to and eliminating jokes and humour that put down or portray LGBTI people in stereotypical ways
- Countering statements about sexual orientation or gender identity that are not relevant to decisions or evaluations being made awards, selections, nominations or appointments
- Inviting “out” professionals to conduct seminars and provide guest lectures on LGBTI topics and other topics of their expertise
- Not forcing LGBT people out of the closet or ‘coming out’ for them to others (The process of coming out is one of enlarging a series of concentric circles and is very individual. Initially the process should be in control of the individual until (and if) they consider it public knowledge)
- Not including sexual orientation or gender identity information in letters of reference or answering specific or implied questions without first clarifying how “out” the person chooses to be in the specific process in question. (Because your environment may be safe, it does not mean all environments are safe)
- Recruiting and contracting LGBTI peer educators
- Viewing LGBTI identity as a positive form of diversity that is desired in a multicultural setting
- Being sensitive to issues of oppression and appreciating the strength and struggle it takes to establish a positive LGBT identity
- Becoming knowledgeable, accepting, and comfortable with this topic before teaching it. If you cannot be, identify someone to teach in your place and carefully screen them.
- Considering carefully how the institutional community (lecturers, students, religious leaders, others) may feel about sexual orientation which is an extremely sensitive topic and working with them to avoid negative reactions.
- Preparing this topic carefully, but not being overly hesitant about addressing it. If you meet a lot of resistance, work to educate other stakeholders before undertaking work with the target group.

Read resource sheet: coming out

FREQUENTLY ASKED QUESTIONS ABOUT SEXUAL ORIENTATION AND GENDER IDENTITY

WHAT IS HOMOSEXUALITY?

Homosexuality is emotional, romantic, and sexual attraction to persons of the same sex. The term homosexual has medical roots from the turn of the last century (early 1900s) and most people now prefer the terms gay and lesbian instead. (Adapted from Buhrke & Douce, 1991)

IS BEING GAY NORMAL?

Being gay is perfectly normal. It is a complete package of feelings and relationships that make up a natural and satisfying identity. Homosexuality has existed throughout humanity's history. Anthropologists report that lesbians and gay men have been and are a part of every culture. Lesbian, gay, and bisexual people are part of every socioeconomic class, educational level, and race/ethnicity.

WHAT IS BISEXUALITY?

Bisexuality is emotional, romantic, and sexual attraction to people whose biological sex is different than and the same as one's own. Bisexuality is perfectly normal and is a complete package of feelings and relationships that make up a natural and satisfying identity. Bisexuality has existed throughout humanity's existence and in every culture. Lesbian, gay, and bisexual people are part of every socioeconomic class, educational level, and race/ethnicity.

ARE HOMOSEXUALITY AND BISEXUALITY MENTAL ILLNESSES?

Homosexuality and bisexuality are not mental illnesses. In 1973, the American Psychiatric Association removed homosexuality from the official listing of psychiatric disorders. In 1975, the American Psychological Association adopted a similar resolution. Position statements of the American Medical Association and the Society for Adolescent Medicine agree with these affirmations regarding sexual orientation. Studies show that people's sexual orientation has no bearing on their mental health and emotional stability. When forced to remain in the closet about one's homosexuality or bisexuality, a person may experience depression and other psychological problems. However these problems stem from a homophobic society and not from sexual orientation.



WHAT IS HOMOPHOBIA?

Homophobia is the irrational fear, disgust, or hatred of gays, lesbians, and/or bisexual people, or of homosexual feelings in oneself. It refers to the discomfort one feels with any behaviour, belief, or attitude (in self or others) that does not conform to traditional sex role stereotypes. Homophobia exhibits itself in the fear of knowing, befriending, or associating with gays, lesbians, or bisexual people; fear of being perceived as gay or lesbian; and/or fear of stepping out of accepted gender role behaviour.

WHAT IS HETEROSEXISM?

Heterosexism is the assumption that everyone is heterosexual. It is a form of oppression that targets gays, lesbians, and bisexual people. Heterosexism confers rights and privileges to heterosexual people that are denied to gay, lesbian, and bisexual people. This is revealed through personal behaviours (telling ‘faggot’ jokes, putting up graffiti, and/or offering verbal and physical harassment), and discriminatory policies, such as denial of health, retirement, and housing benefits. In addition, mainstream media provide few characterizations of gay, lesbian, or bisexual people, and these few are usually stereotypes.

DO GAY MEN, LESBIANS, BISEXUAL PEOPLE, AND TRANSGENDER PEOPLE HAVE LONG-LASTING RELATIONSHIPS?

Yes, a large portion of the LGBTI community has a preference for, or is involved in, a longstanding relationship. However, social rejection of homosexuality and of transgender people frequently causes these relationships to be invisible. How many gay, lesbian, bisexual, and transgender people are there?

The Kinsey Institute suggested that approximately 10 percent of the population may be gay, lesbian, bisexual, or transgender. However, the basis for the percentage is greatly disputed. Gay men, lesbians, and bisexual people are found in all walks of life, among all racial/ethnic groups, and at all socioeconomic levels. In addition, the number of transgender people is greatly disputed and largely unknown. The number is probably higher than experts estimate because the estimates are usually based on the number of people who undergo sexual reassignment surgery, and many transgender people do not pursue this surgery.

WHEN DO GAY, LESBIAN, BISEXUAL, AND TRANSGENDER PEOPLE FIRST KNOW?

People can realise their sexual orientation and gender identity at any point during their lives. Many simply grow up knowing, while some come to understand their identity and orientation later in life (in their teens or 20s, for example). It is important to note that nothing someone encounters in life can ‘make’ one gay, bisexual, lesbian, or transgender. Although events in a person’s life can be catalysts toward self-discovery, sexual experience is

not necessary for anyone to understand their sexual orientation. How does a heterosexual male know he is attracted to women before having sexual experience, or a heterosexual woman know that she is attracted to men? They just know. It is the same with gay, lesbian, and bisexual people. Similarly, a transgender person does not have to have lived as a gender to know that it is his/her correct gender.

WHERE DO GAY, LESBIAN, BISEXUAL, AND TRANSGENDER PEOPLE BELONG IN THE WORKPLACE?

Like straight people, LGBTI people belong anywhere that they can use their talents and abilities. Sexual orientation and gender identity have nothing to do with abilities, talents, or job performance. If forced to remain in the closet for fear of job discrimination, a person may experience depression and other psychological problems that could impair his/her ability to work effectively; but it is homophobia and transphobia—not sexual orientation or gender identity—that cause the problem.

WHY SHOULD PEOPLE BE INFORMED ABOUT GAY, LESBIAN, BISEXUAL, AND TRANSGENDER ISSUES?

Heterosexism and homophobia are the result of ignorance about sexual orientation and gender identity issues. Education about LGBTI issues can help combat fear and discrimination, enabling gay, lesbian, bisexual, and transgender people to be authentic and not to live a lie of false heterosexuality or gender identity in order to be safe. For LGBTI youth, who are more likely to experience depression and rejection by friends and/or family, acceptance and understanding can even be a matter of life and death, since the risk of suicide in LGBTI adolescents is two to three times greater than in their straight counterparts.

WHAT CAUSES HOMOSEXUALITY?

Perhaps a better question is “What determines sexual orientation (i.e., heterosexuality, bisexuality, and homosexuality)?” The factors that determine sexual orientation are complex. There is a growing understanding that human beings have a basic sexuality that can be expressed in a variety of relationships: homosexual, bisexual, and heterosexual, categories that are fluid and may overlap. Although the causes are not known, some researchers believe that one’s basic sexual orientation is predisposed at birth. While one’s orientation may not be recognised or acknowledged for many years, once established, it tends not to change.

AREN’T GAY AND BI MEN EFFEMINATE AND LESBIAN AND BI WOMEN MASCULINE?

Lesbian, gay, and bisexual people are as varied in their dress, mannerisms, and lifestyles as are heterosexual people. Despite this diversity, stereotypes persist about the effeminate man or masculine woman. Although some gay people reflect these characteristics, the overwhelming majority of lesbians and gay

men do not conform to these stereotypes. At the same time, many effeminate men and masculine women are straight.

WHAT IS THE DIFFERENCE BETWEEN SEXUAL PREFERENCE AND SEXUAL ORIENTATION?

Sexual orientation is not a choice. Preference implies choice. Being gay, lesbian, or bisexual is sexual orientation. Sexual preference is a term that might apply to a bisexual person who prefers people of a particular biological sex. However, it is important to understand that many bisexual people do not have a sexual preference.

WON'T GAY PARENTS MAKE THEIR KIDS GAY?

Research has shown that children of lesbian or gay parents are no more likely to become gay or lesbian than children of heterosexual parents. This simply supports the fact that nothing 'makes' a person gay, lesbian, or bisexual. Same-sex couples are just as capable of raising a child as are heterosexual couples.

WHAT DO TRANSGENDER, TRANSSEXUAL, AND CROSS-DRESSER MEAN? AND, ARE THEY THE SAME?

First of all, they are not at all the same. To understand these terms, we must first understand the difference between biological sex and gender identity. Biological sex is a physical condition, identified at birth by one's primary sex characteristics (penis and scrotum for males and vulva, labia, clitoris, and vagina for females). Gender identity is a combination of one's personal internal recognition of the gender that is one's own, the degree to which that internal recognition conforms or fails to conform to one's biological sex, and how one desires to be recognised by others: as male, female, or gender-queer. Transgender, transsexual, and crossdressing people may have any sexual orientation.

Transgender is an umbrella term referring to people whose gender identity differs from the social expectations for the biological sex identified as theirs at birth (using primary sex characteristics). Since these social expectations include gender roles (feminine women and masculine men), people who do not conform to prescribed gender roles may be considered part of the transgender community. A transgender person may or may not ever choose to become transsexual.

Transsexual refers to a person who experiences a mismatch of the body and the brain and sometimes undergoes medical treatment, including hormone therapy and sexual reassignment surgery, to change physical sex to match gender identity.

Crossdressers (formerly known as transvestites) are people who like to dress in the clothing of the gender identity opposite to that considered socially appropriate to their biological sex. Most crossdressers are content with their own biological sex and gender identity. Most crossdressers do not want to be the other biological sex or to be another gender.

WHAT DOES IT MEAN TO BE INTERSEX?

Intersex people (once called hermaphrodites) are people born with ambiguous genitalia or genitalia having characteristics of both sexes. Usually a doctor will immediately perform surgery to assign the infant's sex, usually removing male characteristics and 'creating' a female. Because this surgery is medically unnecessary, advocates today are asking that doctors and parents wait until the child is old enough to self-identify the appropriate biological sex and gender and also old enough to choose whether or not to have the surgery. Some who had surgery as infants later experience conflict with their assigned gender, similar to that experienced by transgender people. They may opt for hormone therapy and surgery to transition to the gender that they should have had. About one in every 2,000 people are born intersex.

WHAT DOES THE TERM 'QUEER' MEAN?

This word, once a derogatory term for gay men, bisexuals, lesbians, and transgender people, has recently been reclaimed by the gay, lesbian, bisexual, and transgender community as a term of pride. However, many have not chosen to reclaim the word and still see it as a derogatory, similar to the terms 'dyke' and 'fag.'

WHAT DOES THE TERM 'TRANSPHOBIA' MEAN?

Intense dislike of or prejudice against transsexual or transgender people.

THERE ARE ONLY TWO GENDERS, RIGHT?

Traditionally, gender has meant either 'male' or 'female.' Gender is the collection of behaviours, dress, attitudes, etc., culturally assigned to people according to their biological sex. However, there is really a range of genders, including male and female, but also including genderqueer or gender ambiguous, butch (man or woman), femme (man or woman), transgender (sometimes considered a gender), and many others.

14 WAYS HOMOPHOBIA AFFECTS EVERYONE

Homophobia and transphobia affect everyone; they have serious consequences for both LGBTI and straight youth.

WAYS THAT HOMOPHOBIA AFFECT LGBTI YOUTH

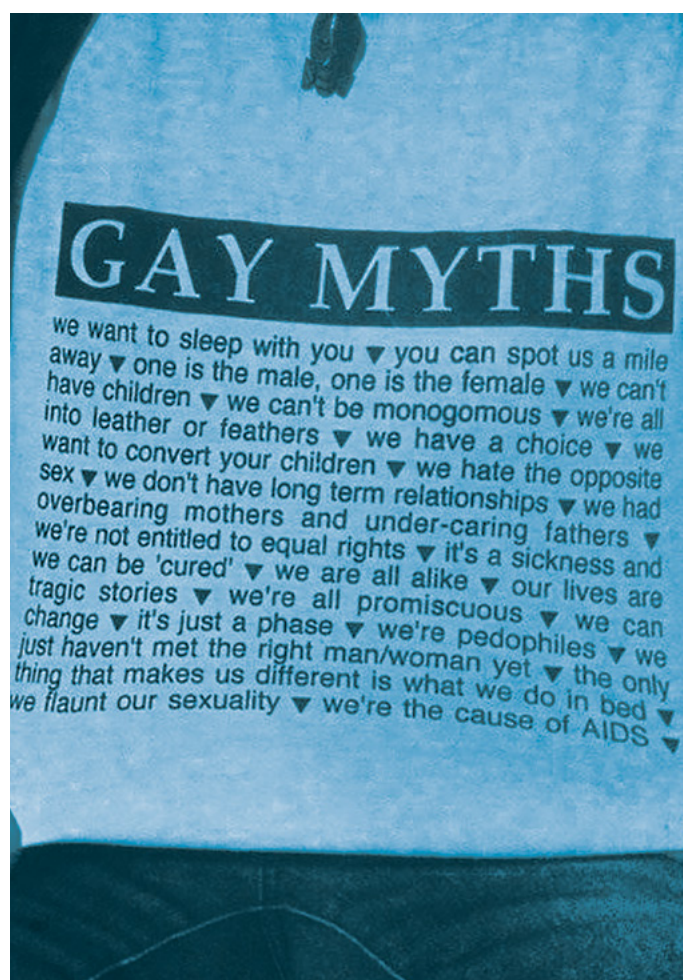
- Homophobia makes many LGBTI youth feel isolated, lonely, and ashamed.
- Homophobia creates an environment in which LGBTI youth face harassment and even violence in their schools, communities and/or homes.
- Homophobia makes some LGBTI youth 'act straight' to hide their sexual orientation or gender identity.
- Homophobia forces many gay, lesbian, bisexual, and questioning youth to become sexually active before they really want to, choosing partners of the opposite sex just to hide their sexual orientation or their questions. Transphobia forces many transgender youth to become sexually active before they really want to just so they can hide their gender identity.
- Homophobia and transphobia contribute to the self-doubt that makes many LGBTI youth turn to drugs and/or alcohol and other forms of high risk behaviour to numb their feelings.
- Homophobia and transphobia cause many LGBTI youth to drop out of school and/or run away.
- Homophobia and transphobia cause many LGBTI youth to think about and/or attempt suicide. Many of the youth who kill themselves are LGBTI.

HOW HOMOPHOBIA AND TRANSPHOBIA AFFECT STRAIGHT YOUTH

- Homophobia and transphobia pressure straight people to act unkindly or even cruelly towards LGBTI people and encourage bullying and cruelty toward anyone whose appearance or behaviour isn't sufficiently 'macho' or 'feminine' (from the viewpoint of the bully).
- Homophobia and transphobia force straight people to act 'straight,' limiting their individuality and self-expression. Straight youth often choose their clothes, hair colour/style,

friends, and even behaviour to 'prove' that they are not LGBTI

- Homophobia and transphobia can destroy family relationships. Some parents, sisters, brothers, and even grandparents break off their relationships with LGBTI family members.
- Homophobia and transphobia lead many straight youths to become sexually active before they really want to just to 'prove' they are straight.
- Homophobia, along with racism, sexism, and poverty, makes it harder to end the HIV epidemic.
- Homophobia and transphobia make it very hard for straight people and LGBTI people to be friends.
- Homophobia and transphobia make it nearly impossible for people to appreciate diversity and the wonderful variety that exists among all people.



LESSON FOUR: STIGMA AND DISCRIMINATION

ACTIVITY: SPACE INVASION

OBJECTIVE: To ensure that participants consider the perspective of groups that are discriminated against e.g. LGBTI population
REQUIREMENTS: A flipchart and pens, or a board to write on, and individual paper and pens for the participants
TIME: 45 minutes

PLANNING NOTES:

- Familiarise yourself with how to facilitate the exercise
 - On a large sheet of newsprint, write the following questions:
 - o Describe how you felt when you compared your items to items other people shared.
 - o What items did you not want to share and why?
 - o What feelings did you have about individual items of yours: sadness, pride, etc.?
-

PROCEDURE:

1. Remind participants that the ground rules established for the first activity still apply
 2. Divide participants into groups of five
 3. Ask them to go to the designated areas with their handbags, briefcases etc.
 4. Tell participants to share with the other members of the group the contents of their handbag, purse, and briefcase
 5. The other group members should write a list of the habits, likes, dislikes, and probable hopes or fears of the person whose belongings were shared, based on the contents of that person's handbag, purse, briefcase. The person can be described through the objects shared, such as 'This person cares about family because they have lots of photos of them' or 'This person is a substance abuser because there are 3 packets of cigarettes' or 'This person does not practice safer sex'.
 6. The person is then asked whether the group's observations are consistent with their habits/traits/feelings
 7. Provide a time check to the groups at the end of each 5-minute allocation of the group discussion time.
 8. Facilitate a plenary discussion using the **discussion questions** provided.
-

DISCUSSION QUESTIONS:

- Describe how you felt when you compared your items to items other people shared
 - What items did you not want to share and why?
 - What feelings did you have about individual items of yours: sadness, pride, etc.?
 - Why is it okay to invade the personal and private lives of LGBTI individuals?
-

KEY LEARNINGS

As peer educators, we expect students to share intimate aspects of their lives. It is important that this be respected. This may cause them to experience a range of emotion. It may, in addition, force them to confront issues sooner. Peer educators and service providers should respect their feelings and support them through the process.

Further, confidentiality of a peer's health information must be upheld. A peer's health information should not be shared inappropriately or without the his/her consent. In cases where disclosure is necessary and they refuse to provide consent such as in the case of TB contact tracing, the peer educator should inform them that their health information will be shared with those affected, so as to protect the community at large.

TIP

Remind participants that there are no right or wrong answers. They should not be forced to share.

Discrimination against LGBTI persons at the personal, familial, and societal levels has been associated with long-lasting physical and mental health problems, including high rates of psychiatric disorders, substance abuse, and suicide. Because of these links, LGBTI persons, whether “out” or not, and often from an early age, have significant needs for psychosocial support and health care. However, access to health care services is among the most daunting challenges the LGBTI community confronts. LGBTI persons who seek health care services of any kind encounter rejection, humiliation, and poor services, leading to significant health issues among LGBTI populations. HIV/AIDS health care trends illustrate the impact of stigma and discrimination against the LGBTI community. Globally, the burden of HIV infection among gay men and other men who have sex with men (MSM) and transgender persons is disproportionately high. MSM are 19 times more likely to have HIV when compared to men in the general population, and transgender individuals show an alarmingly high HIV prevalence rate, ranging from 8 percent to 68 percent. Gay men, MSM and transgender persons who are living with HIV often lack access to HIV education or fear entering the health care system for services, such as antiretroviral therapy, thereby delaying access to life-saving prevention, care and treatment services. LGBTI youth are at increased risk for being abandoned by their families and rejected, barred from accessing schools, all of which undermine their ability to learn and develop the skills that are necessary for a productive life as an adult.

SUPPORT AND MOBILIZE LGBT COMMUNITIES

Open and meaningful participation of LGBTI individuals and communities is essential to more equitable and effective development in all sectors, including health, education, economic growth, democracy, human rights, and governance. Inclusion of LGBTI persons in the development process ensures that the voices of marginalised citizens are heard, and promotes transparency in decision making that will substantially impact their well-being. It also ensures that governments and donors are accountable to the needs of the LGBTI community, including sub-sectors of this community that may face unique challenges (e.g., transgender and intersex persons).

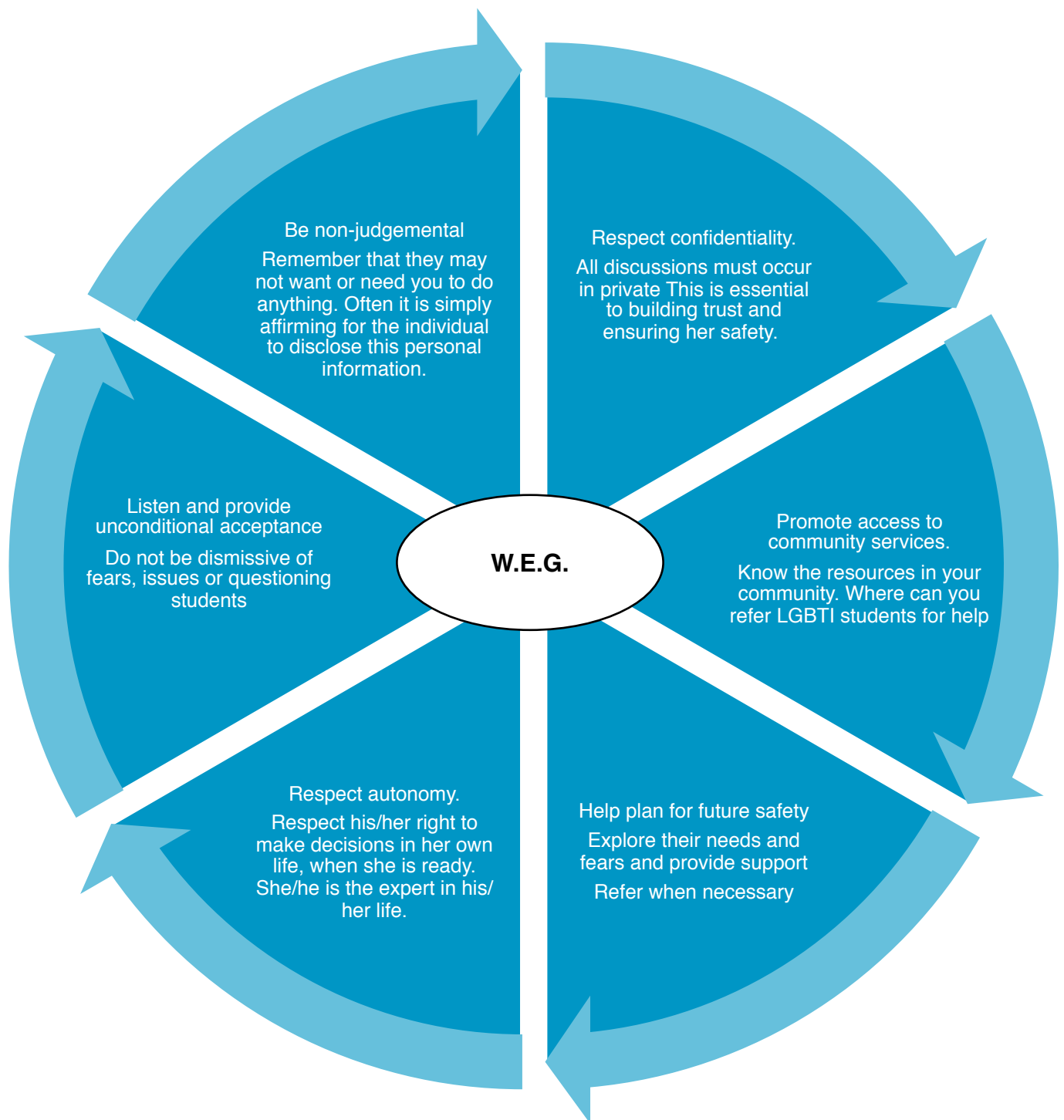
LGBTI HUMAN RIGHTS

In 1993 the African National Congress in the Bill of Rights, endorsed legal recognition of same-sex marriages, and the interim constitution opposed discrimination on the basis of sexual orientation. These provisions were kept in the new constitution, approved in 1996, due to the lobbying efforts of LGBTI South Africans and the support of the African National Congress. As a result, South Africa became the first nation in the world to explicitly prohibit discrimination based on sexual orientation in its constitution. Two years later, the Constitutional Court of South Africa ruled in a landmark case that the law prohibiting homosexual conduct between consenting adults in private violated the Constitution.

In 1998, Parliament passed the Employment Equity Act. The law protects South Africans from labour discrimination on the basis of sexual orientation, among other categories. In 2000, similar protections were extended to public accommodations and services, with the commencement of the Promotion of Equality and Prevention of Unfair Discrimination Act.

In December 2005, the Constitutional Court of South Africa ruled that it was unconstitutional to prevent people of the same gender from marrying when it was permitted to people of the opposite gender, and gave the South African Parliament one year to pass legislation which would allow same-sex unions. In November 2006, Parliament voted 230:41 for a bill allowing same-sex civil marriage, as well as civil unions for unmarried opposite-sex and same-sex couples. However, civil servants and clergy can refuse to solemnise same-sex unions.

HOW PEER EDUCATORS CAN HELP (WARMTH, EMPATHY AND GENUINENESS)



PSYCHOSOCIAL SUPPORT SERVICES

OUT offers psychosocial support services by trained and experienced lay counsellors.

- Face-to-face counselling sessions are based on an appointment system and are provided on Tuesdays and Thursdays between 09h00 and 16h00, at a minimal fee. To make an appointment please contact them on 012 430 3272. Sessions are conducted at the OUT offices, 1081 Pretorius Street, Hatfield, Pretoria.
- Online counselling is available for men on the men2men website (Ask Dr Dick) and for women on the womyn2womyn website (Ask Dr Delicious). Access these websites by visiting www.men2men.co.za and www.womyn2womyn.co.za.
- Telephonic counselling is provided via the OUT Helpline. You can access the helpline Monday - Friday during office hours (08h30 - 16h30) by dialling 0860 OUT OUT (0860 688 688). Reverse charge calls are accepted.

A Youth Group meets on a regular basis and is open to youth aged 18-35 years. The aim is to create a safe and affirming space for youth to share experiences and to provide support. Please contact Jay Matlou on 012 430 3272 for more information (matlouj@out.org.za).

OUT has developed a One2One Peer Education Programme where peer educators are recruited directly from the community and trained to become effective role models and opinion leaders who work directly and informally within their own social and sexual networks. If you feel passionate about becoming involved and playing your part, please contact Jay Matlou on 012 430 3272 (matlouj@out.org.za).

TEN81 Centre Clinic

The clinic is managed by registered nurses. It is conveniently located within a five-minute walk from the Hatfield Gautrain Station in Pretoria.

They offer services that are professional, free, safe, confidential, non-judgmental and inclusive. These include the following:

- HIV Counselling and Testing (HCT)
- Sexually Transmitted Infection (STI) screenings and treatment
- Tuberculosis (TB) screenings
- Provision of Post Exposure Prophylaxis (PEP)
- Anti-Retroviral Treatment (ART)
- HIV management (CD4 and viral load)
- Pap smear
- Basic wound care and clean needles and syringes
- General medical, sexual health and safer sex consultations
- Individual and couples counselling
- Condoms and lubrication

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<http://case.edu/lgbt/safe-zone/safecampus/>

<http://www.advocatesforyouth.org/publications/195-lessons>

MODULE FIVE: ALCOHOL AND DRUG ABUSE PREVENTION (ADAP)



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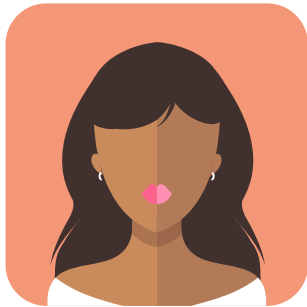
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ABBREVIATIONS

ADAP:	Alcohol and Drug Abuse Prevention	HSV:	Herpes Simplex Virus type 2
AIDS:	Acquired Immune Deficiency Syndrome	OCD:	Obsessive Compulsive Disorder
ARVs:	Antiretrovirals	STIs:	Sexual Transmitted Infections
HBV:	Hepatitis B Virus	STDs:	Sexual Transmitted Diseases
HCT:	HIV Counselling and Testing	TB:	Tuberculosis
HCV:	Hepatitis C Virus	PTSD:	Post-Traumatic Stress Disorder
HEAIDS:	Higher Education and Training HIV/AIDS Programme	PWID:	People Who Inject Drugs
HIV:	Human Immuno-Deficiency Virus	PWUD:	People Who Use Drugs
HPV:	Human Papillomavirus Virus	WHO:	World Health Organisation

CRUCIAL CONVERSATIONS

Riaan and Musa are busy swotting for a test when Katy comes up:



Hey guys. I need some smokes like desperately.

Sorry Kate, wrong stop. I gave that up some whiles ago.



I don't have any but I can tell you where to get some.



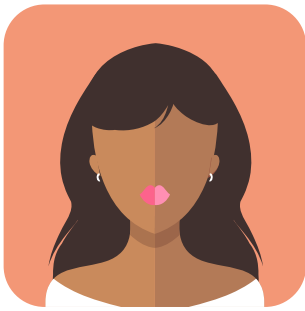
Please man, I'm like so stressed. I just need to calm the nerves. I have to present in front of the whole class tomorrow. I don't know what's wrong with Prof. He's lost it already. There's already 80 people in the class and now he's invited two lecturers we don't even know to come in to help him assess us.

Well...I could give you some ideas on how to calm down and you'll kill your presentation.



C'mon Mu that's not so lekker. Now you're giving away all our dope (ha, ha) you know what I mean.





OMG! So when you guys ace all your presentations – you have help??? What the hell!!!! -tell all now- out with it already...

Shhhhh...not so loud- are you out of your mind?



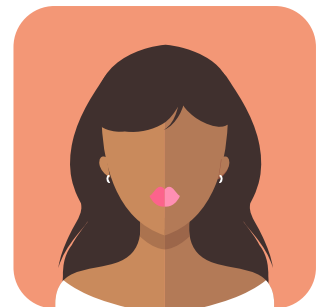
No jokes now. This is serious. There's a pill... it just takes the edge off and you feel like you can conquer the world. Honestly I'm at the top of my game, man.

But you're playing with fire, man. You're taking way too many now.



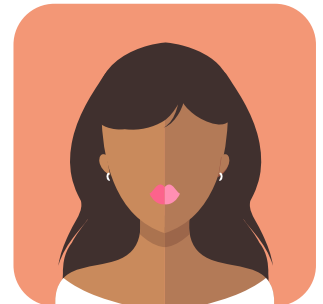
Well the one pill isn't working for me anymore. But if I take more I find that sweet spot, man, so what's the problem? I'm fine aren't I?

Ok well, I don't wanna make this a regular thing and stuff but maybe just this once. So what's the deal? How do I get it?



Well you need to "work late" at the library tonight and then, at around 7, go out to the back of the building. But you have to go alone or he won't pitch.

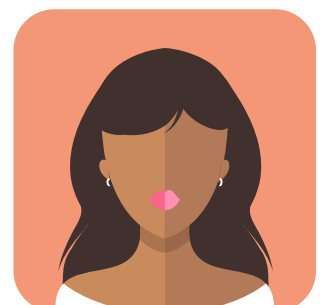
Alone? So is this guy for real? Sounds kinda dodge. I dunno...nah...don't think so...



Hey he comes through every time. He just needs to be careful. It is illegal you know so you can't blab to anyone, even if you ain't showing.

LATER THAT DAY Musa 's phone rings

Hey Mu. Where did you say I have to go to meet this oke again? I'm desperate...



AIM

The aim of the module is to expose peers to information pertaining to alcohol and drug use/abuse that can lead to better health outcomes and ultimately influence students to reduce alcohol and drug use/abuse.

OVERALL MODULE OBJECTIVES

By the end of the lesson, participants will:

- Have knowledge about drug terms, classification and social, psychological, and physiological impact of substance use/abuse
- Have information about key health issues relating to alcohol and drug use/abuse
- Be able to identify risks and dangers surrounding alcohol and drug use/abuse
- Have information on key concepts pertaining to ADAP such as addiction, overdose, craving, withdrawal and relapse
- Understand the link between ADAP and communicable diseases such as HIV, TB and STIs
- Know the association between mental illness and ADAP
- Be aware of key behavioural interventions pertaining to harm minimisation and risk reduction
- Have reinforced existing knowledge, positive attitudes and values and pro-social, healthy skills and behaviour
- Be educated on how to prevent or reduce myths and misinformation, negative attitudes and risky behaviours
- Be able to provide comprehensive, relevant and up to date information, knowledge and skills to others to improve informed decision making on a range of issues facing young men during their formative years

MODULE CONTENT

The module has 5 lessons which focus on the following targeted areas:

- Overview of alcohol and drug abuse prevention (rationale, goals and key definitions)
- Health issues related to ADAP
- Reviewing knowledge on the SA drug scene
- Impact: mental, social and physical
- Principles of good practice
- Knowledge, skills, attitudes and values required by peer educators working with ADAP programmes



LESSON ONE: OVERVIEW OF ALCOHOL AND DRUG ABUSE PREVENTION (ADAP)

AIM: To provide an overview of issues related to alcohol and drug abuse prevention

TIME: 30 minutes

LESSON OBJECTIVES:

- To provide information on the rationale, goals and key definitions pertaining to alcohol and drugs
- To provide information on key approaches and strategies to reduce alcohol and drug use/abuse

PURPOSE OF ACTIVITIES:

- To provide updated and relevant information on substance use and abuse
- To empower participants with knowledge and skills to educate their peers on ADAP

ACTIVITY: SLIDES/PLENARY DISCUSSION

OBJECTIVE: To ensure that participants know essential facts about ADAP

REQUIREMENTS: A flipchart and pens

Projector

TIME: 45 minutes

PLANNING NOTES

- Familiarise yourself with all the necessary issues pertaining to ADAP.
- Review all prepared slides.
- Consider ways to make the session interactive.

PROCEDURE

- Facilitate a buzz session on what is ADAP education, harm minimisation, rationale and key issues pertaining to alcohol and drug use/abuse.
- Present slides on above taking care not to repeat what has already been said.
- Invite comments, questions etc.
- Facilitate a plenary discussion using the **discussion questions** provided.

DISCUSSION QUESTIONS:

- Discuss whether any new information was covered.
- Discuss general attitudes and responses to individuals who use/abuse alcohol/drugs.
- What would be the impact of discriminating against these individuals?
- What do you understand by the phrase: "Alcohol is a disease"?



WHAT IS ALCOHOL AND DRUG EDUCATION?

Alcohol and Drug education is defined as: “educational programmes, policies, guidelines and procedures that contribute to the achievement of broader public health goals of preventing and reducing alcohol and drug use and substance related harm to individuals and society” [Ballard R and Robertson T (1999)]

Amongst young adults there is a definite predisposition to risk as per the characteristics of the target group:

- Young, invincible
- Prone to experimentation
- Target group are no longer considered to be minors
- As young adults the desire to have fun, be free, take risks, shed inhibitions is high
- Post school institutions are high stress environments
- There's a direct impact on health and academic performance

GOALS OF ALCOHOL AND DRUG EDUCATION

- Reduce the number of persons using alcohol and drugs
- Reduce the level of use
- Delay the uptake of alcohol and particular drugs
- Reduce the harmful use of alcohol and drugs
- Minimize the harm associated with the use of alcohol and drugs

WHAT DOES HARM MINIMISATION MEAN?

Harm minimisation accepts that, despite our best efforts, some young people will choose to use drugs, even some illicit drugs. It does not mean that alcohol or drug use is encouraged or

promoted. Abstinence messages in the form of support for the decision not to use drugs should always be an important part of all alcohol and drug use/abuse education programmes. However, harm minimisation has the important role of keeping communication about substance use open to those who may be currently using. The goal of minimising harm respects human rights, equity and discrimination issues that influence acceptance of people living with AIDS and the treatment of drug users.

HARM MINIMISATION OFFERS THREE MAJOR STRATEGIES:

1. Demand reduction - education to not use or use less
2. Supply reduction - laws, including school policies, to limit availability
3. Harm reduction - use safely, use safer substances

WHY DO PEOPLE USE ALCOHOL AND DRUGS?

A person may begin to use substances such as alcohol or drugs because of the intense and very appealing feelings they produce. These feelings, such as well-being, elation, happiness, ecstasy, excitement, and joy, can help some individuals overcome the negative circumstances they experience. They can also give them courage and confidence to do things that normally create anxiety, or enhance their performance e.g. in sport. For others, alcohol and drug use may form a significant part of their social lives and therefore they may use in order to fit in and be accepted. In some cases it may not be a choice. For example a young and disempowered partner may be forced to use. Others may get involved because they deal or sell on behalf of dealers to earn money, and still others may have been lured in to “try it for fun” and then developed a dependency.

WHAT IS ALCOHOL?

Alcohol is a drug. It is classified as a depressant, meaning that it slows down vital functions resulting in slurred speech, unsteady movement, disturbed perceptions and an inability to react quickly. It is a drug that reduces a person's ability to think rationally and distorts his or her judgment.

Although classified as a depressant, the amount of alcohol one drinks determines the type of effect. Most people drink for the stimulant effect, such as a beer or glass of wine taken to “loosen up.” But if a person consumes more than the body can handle, they then experience alcohol's depressant effect and start to lose coordination and control.

HOW DOES ALCOHOL AFFECT THE BODY?

Alcohol is absorbed into the bloodstream via small blood vessels in the walls of the stomach and small intestine. Within minutes of drinking alcohol it travels from the stomach to the brain where it quickly produces its effects, slowing the action of nerve cells. Approximately 20% of alcohol is absorbed through

the stomach. Most of the remaining 80% is absorbed through the small intestine. Alcohol is also carried by the bloodstream to the liver, which eliminates the alcohol from the blood through a process called “metabolising,” where it is converted to a nontoxic substance. The liver can only metabolise a certain amount of alcohol at a time, leaving the excess circulating throughout the body. Thus the intensity of the effect on the body is directly related to the amount consumed. When the amount of alcohol in the blood exceeds a certain level, the respiratory (breathing) system slows down markedly and can cause a coma or death because oxygen no longer reaches the brain.

WHAT IS BINGE DRINKING?

Binge drinking is the practice of consuming large quantities of alcohol in a single session, usually defined as five or more drinks at one time for a man, or four or more drinks at one time for a woman. About 90% of the alcohol consumed by youth under the age of 21 is in the form of binge drinks.

WHAT IS ALCOHOLISM OR ALCOHOL DEPENDENCE?

Alcohol dependence (alcoholism) consists of four symptoms:

Craving: a strong need, or compulsion, to drink.

Loss of control: the inability to limit one’s drinking on any given occasion.

Physical dependence: withdrawal symptoms, such as nausea, sweating, shakiness and anxiety occur when alcohol use is stopped after a period of heavy drinking. Serious dependence can lead to life-threatening withdrawal symptoms including convulsions, starting eight to twelve hours after the last drink. The delirium tremens (D.T.’s) begins three to four days later where the person becomes extremely agitated, shakes, hallucinates and loses touch with reality.

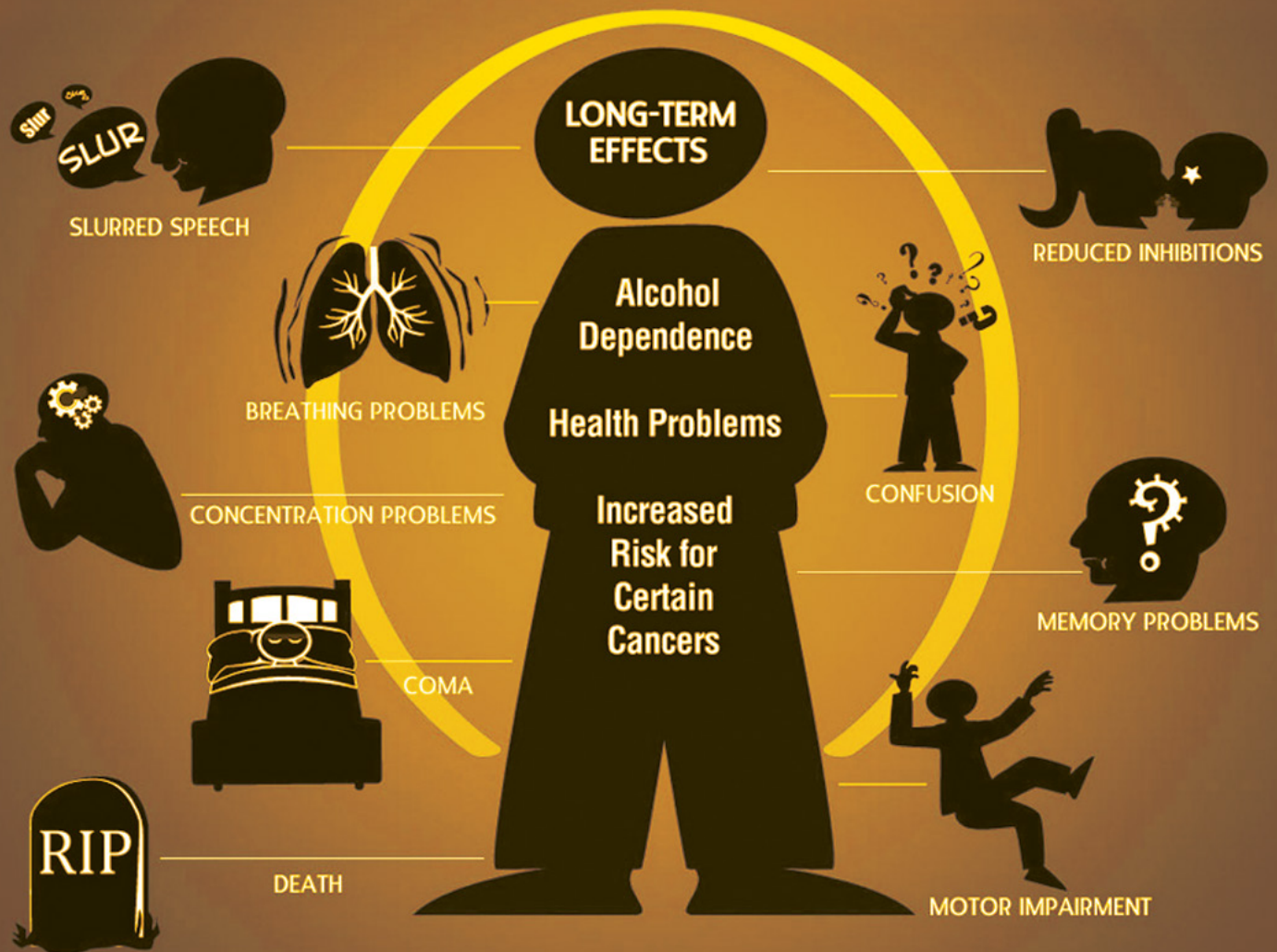
Tolerance: the need to drink greater amounts of alcohol in order to get high. An increasingly heavy drinker often says he/she could stop whenever they choose, they just never “choose” to do so. Alcoholism is a progression, a long road of deterioration in which life continuously worsens.

LONG-TERM EFFECTS OF ALCOHOL

Binge drinking and continued alcohol use in large amounts are associated with many health problems, including:

- Unintentional injuries such as car crash, falls, burns, drowning, firearm injuries
- Sexual assault, domestic violence
- Increased on-the-job injuries and loss of productivity
- Increased family problems, broken relationships
- Alcohol poisoning
- Increased risk for HIV as well as HIV transmission to others
- High blood pressure, stroke, and other heart-related diseases
- Liver disease
- Nerve damage
- Sexual problems
- Permanent damage to the brain
- Vitamin B1 deficiency, which can lead to a disorder characterised by amnesia, apathy and disorientation
- Ulcers
- Gastritis (inflammation of stomach walls)
- Cancer of the mouth and throat

CONSEQUENCES OF ALCOHOL



Alcohol's effects are well known: Slurred speech, reduced inhibitions and motor impairment, just to name a few. As serious as those consequences are, the long-term effects of alcohol abuse are worse, including dependence, increased risk for cancers and other health problems. Remember that when you take your next drink.



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LESSON TWO: HEALTH ISSUES RELATED TO ADAP

AIM: To provide an overview of alcohol and drug abuse prevention issues and show their link to health
TIME: 30 minutes

LESSON OBJECTIVES:

- To provide information on various health conditions and demonstrate the impact on substance use/abuse
 - To highlight risks and responsibilities relating to alcohol and drug use/abuse
 - To provide comprehensive information on substance use and abuse and their impact on health
 - To empower participants with the required knowledge to enable them to educate their peers on ADAP
-

PURPOSE OF ACTIVITIES:

- To provide opportunities for participants to do self study
- To give participants a chance to demonstrate their understanding of the link between alcohol and drug use and health

ACTIVITY: SELF-STUDY

OBJECTIVE: To ensure that participants know essential facts about ADAP
To assess the ability of participants to transfer knowledge clearly and accurately

REQUIREMENTS: A flipchart and pens
Projector

TIME: 60 minutes

PLANNING NOTES

- Familiarise yourself with all the necessary issues pertaining to ADAP
 - Review all prepared slides
 - Allocate topics to groups to prepare ahead of time:
 - o Group 1: Relationship between Mental illness and drug use
 - o Group 2: Risks and responsibilities regarding injecting drug use
 - o Group 3: Other common infections related to alcohol and drug use including HIV and Hepatitis C
-

PROCEDURE

- Invite groups to present on the various topics allocated on the previous day (5 mins to present)
 - Present slides on above taking care not to repeat what has already been said
 - Invite comments, questions etc.
 - Facilitate a plenary discussion using the **discussion questions** provided
-

DISCUSSION QUESTIONS:

- Discuss whether any new information was covered
- Ask how participants felt about what they learnt in the session
- Discuss the issue of ADAP and behaviour change
- Ask what they understand about the term “tough love”

KEY LEARNINGS:

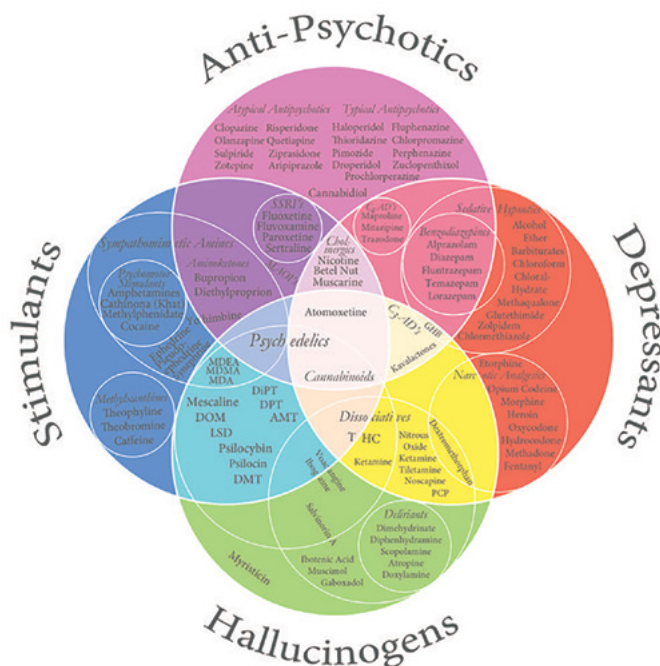
1. Alcohol and drug use and abuse can:
 - impact on existing health conditions
 - create new health conditions
 - be aggravated by health conditions
2. Young people are at high risk:
 - Due to peer pressure
 - In high stress environments where there is pressure to achieve or perform
3. There is a high correlation between risk for transmission of HIV and other STIs and alcohol and drug use/abuse
4. Behaviour change is difficult and cannot happen overnight. There are various stages in the model for behaviour change (see handout at the end of the module)

HEALTH ISSUES RELATED TO ADAP

TUBERCULOSIS (TB)

Injecting drug use/abuse is associated with increased rates of TB infection. Increased TB disease rates among drug users/abusers are likely due to other risk factors for TB disease, such as being in prison, homelessness and poverty. Tuberculosis is a leading cause of death among people who inject drugs (PWID) living with HIV.

MENTAL ILLNESS AND DRUG USE



Not all people who use/abuse drugs and alcohol are mentally ill although mental illness does have a strong association with alcohol and drug use/abuse. In some cases, mental illness may already be present before an individual becomes a user/abuser. In this situation, mental illness may be one of the factors that lead individuals to drug use as a method of coping. In other cases, mental illness may result from drug use/abuse since some drugs manipulate the brain. Also, for some, alcohol or drug use stressors, such as homelessness or unemployment can lead to mental illness.

8 Most Common Dual Diagnosis Conditions

Many addicts have a dual diagnosis—their addiction plus a mental health condition. Some addicts use drugs and alcohol to try to self-medicate their mental health symptoms, while others developed symptoms after substance abuse. These are the most common dual diagnosis conditions and how addicts try to alleviate the symptoms.

1 Bipolar Disorder

Symptoms: Intense shifts in mood, energy and functioning between depression and mania.

Most Commonly Abused Drugs: Alcohol or cocaine, depending on whether they are in a depressive or manic cycle.

2 Post-Traumatic Stress Disorder (PTSD)

Symptoms: Nightmares, hypervigilance, detachment and recurring distress after an extreme traumatic event (war, assault) created a post-traumatic stress response in their brains.

Most Commonly Abused Drugs: Alcohol, benzodiazepines and sleeping pills to find relief from symptoms.

3 Attention Deficit Hyperactivity Disorder (ADHD)

Symptoms: Persistent pattern of inattention and inability to focus and/or hyperactivity-impulsivity. Adult symptoms include "zoning out," extreme distractibility, struggling to complete tasks, chronic lateness and addictive tendencies.

Most Commonly Abused Drugs: Alcohol and/or stimulant abuse creates a heightened sense of focus or relaxation to decrease the ADHD symptoms.

4 Anxiety

Symptoms: Excessive worry and/or apprehensive expectations that have a negative impact on daily functioning.

Most Commonly Abused Drugs: Alcohol, benzodiazepines and, occasionally, cocaine to compensate for anxiety. An anxious person continues using alcohol or drugs to avoid discomfort.

5 Obsessive-Compulsive Disorder (OCD)

Symptoms: Recurrent obsessions or compulsions that are time consuming or cause noticeable distress with a negative impact on daily function.

Most Commonly Abused Drugs: Marijuana or alcohol to slow down obsessive thoughts.

6 Depression

Symptoms: Persistent sad, anxious or empty mood; feelings of hopelessness and worthlessness; decreased energy, fatigue and the loss of interest in activities that once were pleasurable.

Most Commonly Abused Drugs: Alcohol slows down and depresses sensation to mask mood.

7 Panic Disorder

Symptoms: Recurrence of panic attacks marked by sweating, trembling, feeling of choking, chest pain, nausea, feeling dizzy and fear of dying.

Most Commonly Abused Drugs: Psychiatric medications like benzodiazepines and alcohol in social situations.

8 Schizophrenia

Symptoms: Delusions, hallucinations, disorganized speech and disorganized behavior.

Most Commonly Abused Drugs: Nicotine, caffeine and other stimulants temporarily relieve disruptions and some hallucinations.

Sources:
<http://www.nimha.org/go/information/get/info/co-occurring-disorders/dual-diagnosis>
 Diagnostic and Statistical Manual of Mental Disorders, Fourth Edition (DSM-IV)

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ANXIETY

Anxiety is a normal emotion in everyday life and is closely related to fear. Anxiety can become a mental illness when it is prolonged or never-ending or when the anxiety becomes out of control and affects an individual's daily functioning. Signs and symptoms of anxiety may include excessive worry, fear, feelings of uneasiness, tightness in the chest, difficulty breathing, heart palpitations, dizziness, light headedness, nausea, diarrhoea and excessive sweating. Anxiety can also be caused by drug use/abuse, particularly the use/abuse of stimulants such as cocaine. At the extreme, anxiety can often lead to panic (or a panic attack) where a person experiences intense stress and a sense of not being in control. A special type of anxiety disorder that may affect people who use alcohol and drugs is social anxiety disorder. When someone suffers from social anxiety, they experience intense fear, panic or stress while engaging in everyday social situations. It can create feelings of humiliation, embarrassment and judgement. Social anxiety may include symptoms such as sweating, blushing, trembling, nausea and stammering. An individual who is experiencing social anxiety may be more likely to use drugs or alcohol to help them cope, and less likely to seek health care services because of fear of being judged by health care staff.

WHAT ABOUT THE USE OF NON-STERILE INJECTING EQUIPMENT?

Non-sterile injecting equipment refers to the re-use of previously used injecting equipment to inject a drug. The association between using nonsterile injecting equipment or sharing needles and HIV infection is a strong one. There are a few methods for sharing injection drugs and equipment.

Frontloading refers to a method of drug sharing where drug from one syringe is injected into the front end of another syringe. This allows for an equal division of the drug between two or more users.

Backloading refers to injecting drug from one needle into the back, open end of another syringe. This may be more difficult because it requires that the plunger be reinserted carefully without wasting any drug. Both methods can easily lead to contamination of the injection equipment by HIV and other infections.

WHAT ARE THE SIDE EFFECTS OF INJECTING DRUG USE?

Because the drugs go directly into the bloodstream when injected, the effects are more intense and addictive – and so are the risks for overdosing. There are also many social, physical and mental consequences of using injection drugs. For example, injecting drug use is much more addictive than other forms of drug use, creating physical dependence and often leading to more severe withdrawal symptoms than when non-injection users stop taking the drug.

In addition to the high risk for overdosing, injecting drug use can also result in damage to the arteries and scarring of the veins. Bacteria can also cause infections at injection sites (places on your body where you inject the drug). A common effect is skin irritation or itching that is relieved by scratching. Some injectable drugs such as heroin also make you throw up. Users sometimes judge the strength of the dose by the force of their nausea or urge to throw up.

DEPRESSANT DRUGS like opiates (e.g., heroin) and sedatives (e.g. Valium and alcohol) slow down the body's functions. A person who overdoses on a depressant will experience breathing that can become life-threateningly slow or stop altogether, leading to heart failure.

STIMULANT DRUGS, such as cocaine and speed, can cause a person who has overdosed to have a heart attack or experience cardiac arrest, collapse from exhaustion, have a seizure, or become so disoriented that they accidentally hurt themselves.

OTHER COMMON INFECTIONS

People who inject drugs (PWID) may also experience the following common infections:

ABSCESSES AND ULCERS

People who inject drugs may be affected by abscesses or ulcers around the areas of their bodies where they inject, usually the arms or legs. Abscesses may be difficult to heal. Ulcers are generally not deep and can heal with good wound care and antibiotics.

SEPTICAEMIA

Septicaemia is a life-threatening infection of the blood resulting from the presence of certain bacteria. Symptoms of septicaemia include high fever, shivers, headache and possibly convulsions.

ENDOCARDITIS

Endocarditis is an inflammation of the heart due to infection from foreign matter that may enter the bloodstream of injecting drug users through injection. Endocarditis symptoms include irregular heartbeats and chest pain.

CELLULITIS

Cellulitis is a common bacterial skin infection that can affect people who inject drugs. Cellulitis shows up as an inflammation of the skin.

PHLEBITIS

Phlebitis results in the development of track marks along the veins where an injecting drug user has injected. Track marks are actually damaged or infected veins that can lead to the formation of blood clots.

WHAT EFFECT DOES MENTAL ILLNESS HAVE ON THE HEALTH-SEEKING BEHAVIOURS AND TREATMENT OF PEOPLE WHO USE DRUGS (PWUD)?

WHAT ARE COMMON SIGNS AND SYMPTOMS OF DRUG USE?

Drug use can result in common signs and physical, behavioural and psychological symptoms. A few of these are listed below; however, be aware that these symptoms could also be associated with a variety of other conditions and are contingent on the types of drugs someone may be using.

Physical symptoms and signs	Behavioural symptoms and signs	Psychological symptoms and signs
- Red eyes or dilated or pinpoint pupils - Changes in appetite - Changes in sleep patterns - Sudden weight loss or gain - Deterioration of physical appearance - Skin manifestations such as abscesses or ulcers - Needle marks or puncture wounds on skin from injecting	- Drop in attendance and lack of interest in work or school - Unexplained need for money that leads to borrowing and stealing - Engaging in secretive or suspicious behaviour - Sudden changes in friends, favorite hang outs, and hobbies - Frequently getting into trouble (fights, accidents, illegal activities) - Possession of drug gear or paraphernalia	- Unexplained change in personality, attitude, and behaviour - Mood swings, irritability, angry outbursts, or unexplained excitement - Periods of unusual hyperactivity, agitation, or giddiness - Lack of motivation and ambition - Appears lethargic or spaced out - Appears fearful, anxious or paranoid with no reason

Depression can prevent substance users/abusers from taking their medication as the doctor instructed them to (e.g. missing doses, taking too few or too many) or even prevent them from going to a health care facility. Anxiety can lead to unwillingness to interact with others, which may include health care service providers. Also, a user/abuser may show signs of a behavioural disorder and may act out in a clinic setting and become challenging to treat. Individuals who are experiencing a mental illness may therefore be more difficult to treat for HIV and other issues.

BEHAVIOURAL RISKS

A PWUD who is experiencing depression or bipolar disorder may be less likely to protect his or her health and may choose to engage in riskier behaviours. Abuse of drugs and alcohol can make users/abusers more vulnerable to HIV and STIs because it may cause them to engage in riskier sexual behaviour, such as not using condoms, not worrying about consequences or outcomes of their actions, or by feeling an inflated sense of courage and fearlessness.

Individual risks	Societal risks	Environmental risks	Structural risks
- Unprotected sex - Substance use - Substance use while having sex - Sharing injecting needles - Multiple partners - Low self esteem - Disinhibition - Depression - Internal stigma - Lack of information	- Stigma - Discrimination - Prejudice - Social norms - Family expectations - Homophobia - Heteronormativity - Penile-vaginal normativity - Cultural / religious beliefs	- Dangerous places - Invisibility - Badly lit - High levels of crime - Having to hide - Having to buy drugs from dangerous place/person - Having to sell sex in dangerous place - High levels of violence - Unhygienic conditions	- Lack of access to justice / police support - Lack of access to health care - Difficult to get employment - Illegality of behaviours - Punitive legislation - Lack of access to commodities: lubricant, clean injecting equipment

HIV, HEPATITIS C, AND OTHER INFECTIONS

People who inject drugs (PWID) can easily get bacterial infections that travel through blood because they may use unsterilised injection equipment or share needles when they inject drugs. In these situations, they may become infected if they use a needle, syringe or other equipment that is contaminated with bacteria, viruses and/or other foreign material. These infections can lead to a variety of illnesses and can cause death of drug users, their sexual partners, and their children through mother-to-child transmission. Drug use can increase sexual risk because it can affect judgment and decision making as well as lower inhibitions during sexual activity. For example, cocaine use can sustain an enhanced period of sexual activity but also it dries out the mucous membranes and the long duration of sex might cause bleeding due to bruising and small cuts. Discrimination and stigma surrounding alcohol and drug use/abuse have been reported as barriers to accessing HIV testing services and other medical services in South Africa which adds to the overall vulnerability of people who use alcohol and drugs. Fear often prevents these individuals from seeking help, not only because of the legal aspects, but also because of a lack of easy access to treatment and medication.

HIV TRANSMISSION DURING INJECTION

HIV can be transmitted via sharing either the needle or the syringe. Sharing a filter or spoon can transmit HIV. Injecting in a public place where a lot of injecting occurs, even with all the right equipment, tends to be done quickly for fear of being caught, which increases the likelihood of mistakes and HIV transmission.

WOMEN AND INJECTING: RISKS AND VULNERABILITIES

Studies from around the world show that:

- The average HIV prevalence among women who inject is 50% higher than among men who inject
- Many women begin injecting drugs in the context of sexual relationships, and they often borrow or share injecting equipment from their male partners
- Women are more likely to be injected by their male partners – being injected by another person or being helped to inject is a predictor of HIV infection
- Women are more likely to be the last person to use shared injecting equipment
- Women who inject drugs are often dependent on their sexual partners to obtain drugs, which compromises their ability to negotiate safer sex or safe injecting practices
- Women who inject drugs have lower access to services than men who inject drugs.

SEXUALLY TRANSMITTED INFECTIONS

There is a lot of education and information about the importance of protecting against HIV and human papillomavirus (HPV) infection (the world's most common STI). But it is also important to do a better job of preventing and treating diseases like gonorrhoea, chlamydia and syphilis. These are all STIs. There are more than 200,000 stillbirths and babies dying in the womb every year, as well as deaths of over 90 000 new-borns because of not treating syphilis

STIs are common among people who use drugs and alcohol especially infections such as herpes simplex virus type 2 (HSV), human papillomavirus (HPV), gonorrhoea, chlamydia, and syphilis. Large numbers of sexual partners and frequent unprotected sex is common in this group and increases their risk for STI infection. Untreated STIs also increase the HIV risk and contributes to their transmission within communities.



LESSON THREE: ALCOHOL & DRUGS IN SOUTH AFRICA

AIM: To provide participants with information regarding terminology and classification of drugs

TIME: 30 minutes

LESSON OBJECTIVES:

- To evaluate baseline knowledge
 - To provide relevant information to increase knowledge on local drugs
 - To educate participants about how alcohol and drugs are classified and what the classification means
 - To empower participants with the required knowledge to enable them to educate their peers on ADAP
-

PURPOSE OF ACTIVITIES:

- The module is designed to educate through participative, peer-led action learning, as well as to develop skills in amongst peer educators to assist peers with challenges pertaining to alcohol and drug use

ACTIVITY: IT'S CLASSIFIED

OBJECTIVE: To provide participants with information regarding terminology and classification of drugs

REQUIREMENTS: Chart paper or whiteboard, pens, Leader's Resource Sheet with Drug Terms and classification

TIME: 30 minutes

PLANNING NOTES

- Prepare introductory slide
 - Cut up and prepare drug terms for activity
 - Familiarise yourself with the correct classification in each section
-

PROCEDURE

- Introduce the lesson using the introductory slide
 - Ask the group to consider how society usually groups/classifies various drugs and establish that the major categories may be described as:
 - o legal/illegal;
 - o medicinal/recreational;
 - o depressant/stimulant
 - Hand out the cut out drug names that were prepared for the activity so every participant has a drug
 - Ask each 'drug' to stand at one end of an imaginary line according to whether they think their drug is legal or illegal. A discussion can be conducted as to why people think it is categorised as it is.
 - This activity can then be repeated using medical/recreational or stimulant/depressant as the two opposing positions to take.
 - Facilitate a plenary discussion using the **discussion questions** provided
-

DISCUSSION QUESTIONS:

- What does it mean to be a stimulant or a depressant? Discuss the risks and dangers associated with each
- What does it mean to be a drug that has medicinal use or one that has recreational use? Discuss the risks and dangers of each
- What is the significance of whether a drug is legal or illegal? Consider whether a drug is 'safer' if it is legal

KEY LEARNINGS

STIMULANTS / UPPERS

Stimulants such as cocaine speed up the central nervous system. People using stimulants may feel happy and excited, and have more energy, concentration or motivation. However once the drug is out of the system, it creates a restlessness and a sense of agitation and irritation. Stimulants make it difficult to sleep. Users feel the urge to take it again to settle the restlessness and to “feel good “again. Repeated use leads to addiction and overdose which can cause heart attacks, exhaustion, seizures, and severe disorientation.

DEPRESSANTS/DOWNERS

Depressants are drugs like opiates (e.g., heroin) and sedatives (e.g. Valium and alcohol) which slow down the central nervous system. People using depressants may feel happy and content, as well as sleepy and relaxed. Depressants often slow down bodily functions such as breathing and heart rate, and may make it hard to speak (slurred speech) or move properly in large enough doses, in which case they may be harmful. A person who overdoses on a depressant will experience breathing that can become life-threateningly slow or stop altogether, leading to heart failure.

HALLUCINOGENS

Hallucinogens change the way people see, hear, feel or think. There are three main groups of hallucinogens: psychedelics, dissociatives and delirants. Each group has different effects. They may cause hallucinations, when a person imagines something that is not really there.

NOTES

LESSON FOUR: ALCOHOL AND DRUG ABUSE - THE EFFECTS ON MIND, BODY, SOUL

AIM: To provide participants with information regarding the social, mental and psychological impact of alcohol and drugs
TIME: 30 minutes

LESSON OBJECTIVES:

- To provide an opportunity for participants to gain knowledge and to understand the holistic impact of alcohol and drugs
 - To use the lesson to develop an end product (gallery walk) that can be used for further ongoing education
 - To enable participants to absorb the information using an high impact activity
 - To create opportunities for peer led learning
-

PURPOSE OF ACTIVITIES:

- The module is designed to educate through participative, peer-led action learning, as well as to develop skills in amongst peer educators
 - To provide an opportunity for participants to express personal opinions about alcohol and drug use/abuse
 - To empower participants to consider ways of avoiding risk
-

ACTIVITY: GETTING THE 'DOPE' ON ALCOHOL AND DRUGS

OBJECTIVE: To expose participants to information on how the use of alcohol and illicit drugs can cause physical, psychological, legal and financial harm

REQUIREMENTS: Pens, small and large paper
Drug Information Sheets
Flip chart and pens to record plenary discussion
Activity: Hand-outs
Fact Sheets: Hand-outs

TIME: 45 minutes

PLANNING NOTES

- Study the drug information sheets
 - Prepare / review slides
-

PROCEDURE

1. Divide participants into 7 groups. Allocate one **drug information sheet** to each group together with a large sheet of blank paper
2. Instruct groups to draw the outline of a person as large as the paper will allow
3. Groups must read the drug information sheet allocated to them and write and draw inside the body shape the effect the drug has on the mind and body as well as signs of use
4. On the outside of the body they must write and draw some of the social, financial, emotional, financial and mental effects of the drug
5. Groups must:
 - i. Identify the main reasons why individuals would use the drug and why some individuals may become regular or heavy users
 - ii. List how to an individual can avoid the risks and nominate a member to report for the whole group.
6. The groups must display the large sheets of paper and conduct a gallery walk (10 minutes)
7. Facilitate a plenary discussion using the **discussion questions** provided

DISCUSSION QUESTIONS:

- What did you learn about risks and dangers/about health?
 - How did the activity impact on you? (Does any of the information resonate? Do you know of someone in the grip of addiction? Have you ever thought you might be at risk?)
 - Did the information have the power to influence choices that people make (If more people knew or if people knew more of what you now know, do you think it would make a difference?)
-

KEY LEARNINGS

- Drug dependence, occurs when a person physically and/or mentally requires the drug to function normally
- Drug dependence is a chronic relapsing mental illness, not simply a behaviour that can be changed through counselling alone
- When an individual is dependent on a drug and the drug is stopped abruptly they may experience withdrawal symptoms (e.g. vomiting, diarrhoea, sweating) which indicate that physical dependence has developed
- Drug dependence can also be identified when an individual experiences disturbance in their psychological functioning, such as difficulty in concentrating, anxiety, depression, irritability, insomnia, headaches and muscle cramps

NOTES

HANDOUT: SUBSTANCES IN SOUTH AFRICA

SUBSTANCES IN SOUTH AFRICA

	Drug/Substance	Type	Street names	Method of use
ILLEGAL	Cannabis	Downer, Relaxant, Depressant	Marijuana, Dagga, Weed, Zol, Ganja, Spliff	Smoke, eat
	Heroin	Opiate, Downer, Relaxant, Depressant	Gear, smack, H, horse, brown	Inject, smoke
	Cannabis / heroin / other substances mixture	Downer, Relaxant	Nyaope, Whoonga	Usually smoked
	Methaqualone (Mandrax)	Downer, Barbiturate	Buttons, white pipe	Smoke, ingest, inject
	Cocaine / Crack cocaine	Upper / Stimulant / Amphetamine	Coke, Charlie	Snort, inject
	Ecstasy / MDMA		e, Pills	Tablet (ingest), snort, inject
	Methamphetamine		Crystal meth, crystal, Tik, ice	Ingest, inject, smoke, snort
	Methcathinone		Cat, khat	Smoke, ingest, snort
	LSD	Psychotropic / Hallucinogen	Acid	Ingest
	Mushrooms		Magic mushrooms, shrooms	Ingest
	Ketamine		K, special K	Snort, ingest
LEGAL	Caffeine	Upper / Stimulant		Coffee, tea, energy drinks
	Nicotene	Relaxant		Cigarettes, nicotine patches/pens
	Alcohol	Downer, relaxant	Booze, drink, dop, liquor	

ALCOHOL & DRUG USE Q&A GUIDE

Q1: What is alcohol and drug dependence?

Answer: Alcohol and drug dependence is a complex and often chronic brain disease. It occurs when the use of a substance dominates a person's life and becomes a compulsive behaviour that is hard to control. When a person who uses substances is experiencing dependence, he or she has extreme difficulty in resisting the urge to use despite the negative consequences and harmful effects.

Q2: What is a craving?

Answer: Cravings are strong memories that are linked to the effect of alcohol or drugs on the brain and can cause strong physiological effects.

Q3: Why do some PWID share needles?

Answer: According to recent South African research, injecting alone or in pairs is more common than group injecting. For those PWID who do share needles and syringes or use non-sterile equipment, their reasons for sharing vary. In most cases poverty is cited by PWID as a key reason for sharing needles and syringes.

Q4: What is an overdose?

Answer: People who use drugs are also vulnerable to overdose, which occurs when they take too much of a drug in a single episode of injecting or using. This can happen if a PWUD increases the quantity of the drug they are using but also if they use the same quantity of a higher quality drug.

Q5: What are the consequences of overdosing?

Answer: Overdosing can have significant consequences, including death. For PWUD, accessing care to manage an overdose can be challenging because of fear of being arrested or otherwise becoming involved in the criminal justice system after disclosing their drug use to a health care worker. There is also very little information available for PWUD on what to do if an overdose occurs, and often the health care system is not capacitated to manage such a situation.

Q6: What are the signs of an overdose?

Answer: One of the clearest signs that someone is overdosing is that their face or lips will turn blue. They may also look very pale; be very limp; be able to breathe and look at you, but not be able to talk; be breathing, but very slowly and shallowly; stop breathing altogether; have a slow pulse (heartbeat) or no pulse at all; foam at the mouth; vomit; shake or have a seizure; complain of chest pain, pressure, tightness, or shortness of breath; or suddenly collapse and become unconscious. You have about four minutes from the time your lips turn blue to coma. If a person stops breathing, it can take only a few minutes for them to die. Just waiting for them to 'get over it' is the worst thing you can do if someone is overdosing. Immediate action must be taken to help them survive.

Q7: What is withdrawal?

Answer: Withdrawal occurs when an alcoholic or PWUD suddenly stops using the substance upon which he or she has become dependent. This usually results in a number of physical and mental withdrawal symptoms, including abdominal cramps, muscle spasms, vomiting, chills, high fever, restlessness, irritability or depression (5). PWUD commonly call withdrawal 'getting sick'. The symptoms of withdrawal are very intense, leading users to do whatever is needed to remove these symptoms. Shaking and tremors are common during severe withdrawal.

Q8: What is a relapse?

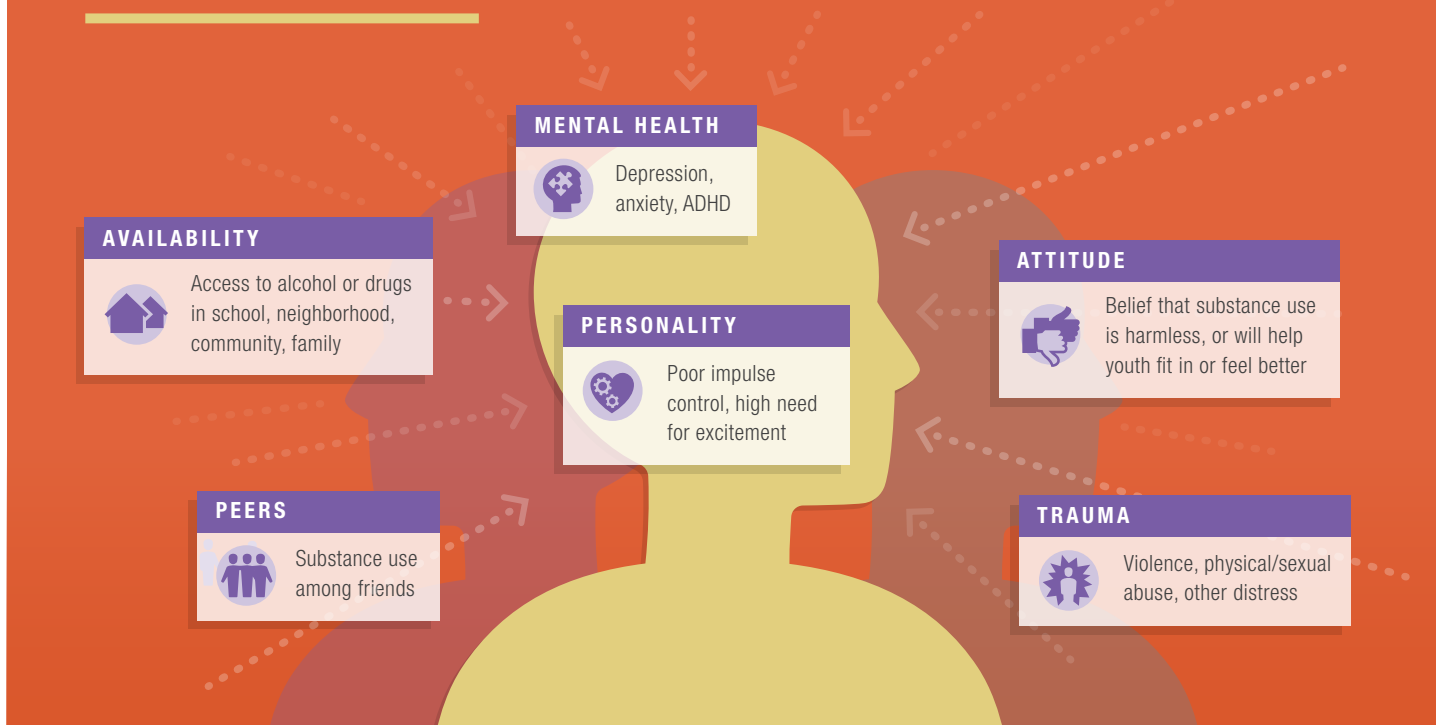
Answer: A relapse occurs when a PWUD or alcoholic begins using again after having successfully stopped using for a period of time. Relapse is common and is a normal part of recovery. Alcohol and Drug use is highly addictive and it can often take a number of attempts before a user can stop.

TIPS FOR RISK REDUCTION COUNSELLING

(1)

- ✓ Do what is possible first: Emphasis on short-term pragmatic goals (for example, preventing HIV transmission in a specific circumstance) over long-term idealistic goals (for example, overall reduction in harm/risk from drug use).
- ✓ Small changes are easier than big changes: Establishment of a scale of means to achieving specific goals: for example, a hierarchy of risks
- ✓ Say the same thing many times in many ways: Use of multiple strategies to achieve goals.
- ✓ Give clients the tools they need; Provision of the means to accomplish risk reduction, for example condoms and sterile needles and syringes.

Many factors influence whether a young person tries alcohol or other drugs.



HANDOUT: MOTIVATION FOR CHANGE: THE STAGES OF CHANGE MODEL

In *Changing for Good* (1994), Prochaska and DiClemente describe the six stages of change:

Stage #1: Pre-contemplation (before the impact is understood)

People at this stage may not see the consequences of addiction as important enough compared to the benefits. As a result, at this stage they demonstrate a lack of interest in wanting or needing to change, and having no plan or intention to change. We might describe this person as *unaware*.

Stage#2: Contemplation (thinking about the impact)

People in the contemplation stage are starting to see/realise the impact of substance use on their actions or behaviour. However, they are still conflicted about whether it is necessary or worthwhile to change. The individual in this stage is thinking about and wanting to change but not sure if he or she can change or will be able to commit to change. They lack the confidence and commitment to change behaviour; but have the intention to change at some point. We might describe this person as aware and open to change.

Between stage 2 and 3: A decision is made. People realize that the negative consequences of their behaviour outweigh the positives. They choose to change their behaviour. They make a commitment to change.

Stage #3: Preparation

At this stage people accept responsibility to change their behaviour. They weigh options and select techniques for behavioural change. Characteristics of this stage include: developing a plan to make the needed changes; building confidence and commitment to change; and having the intention to change within one month. We might describe this person as willing to change and anticipating of the benefits of change.

Stage #4: Action

At this stage people begin behavioural change efforts. Although these change efforts are self-directed, outside help may be sought. This might include rehab or therapy. Characteristics of this stage include: consciously choosing new behaviour; learning to overcome the tendencies toward unwanted behaviour; and engaging in change actions for less than six months. We might describe this person as embracing change and gaining momentum.

Stage #5: Maintenance

People in the maintenance stage have mastered the ability to sustain new behaviour with minimal effort. They have established new behavioural patterns and self-control. Characteristics of this stage include: remaining alert to high-risk situations; maintaining a focus on relapse prevention; and behavioural change that has been sustained six months. We might describe this person as persevering and consolidating their change efforts. They are integrating change into the way they live their life.

Stage #6: Termination

At the termination stage people have adopted a new self-image consistent with desired behaviour and lifestyle. They do not react to temptation in any situation. Characteristics of this stage include: confidence; enjoying self-control; and appreciation of a healthier and happier life. The relapse prevention plan has evolved into the pursuit of a meaningful and healthy lifestyle. As such, relapse into the former way of life becomes almost unthinkable.

Note: Relapse to a prior stage can occur anywhere during this process. For example, someone in the action stage may move back to the contemplation or pre-contemplation stage. (A. Tom Horvath, Ph.D., ABPP, Kaushik Misra, Ph.D., Amy K. Epner, Ph.D., and Galen Morgan Cooper, Ph.D. , edited by C. E. Zupanick, Psy.D. <http://www.amhc.org/1408-addictions/article/48541-types-of-evidenced-based-effective-treatments-for-addiction>)

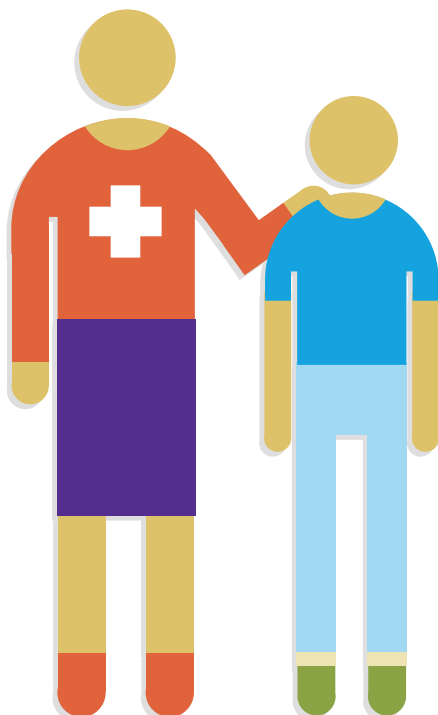
PROMOTING HIV, STI, TB SCREENING AND TREATMENT

- Each engagement with people who use drugs (PWUD) and alcohol offers an opportunity to provide educate and increase their awareness of HIV risks and the prevention tools available to them. Peer educators should take this opportunity to provide facts and support to these individuals.
- Peer Educators should educate by dispelling common misconceptions or myths associated with HIV. For example, it is a common misconception that HIV-positive individuals do not face any risk if they are re-exposed to HIV after they have become infected. This is known as HIV reinfection and, unfortunately, it can significantly reduce future treatment options as well as increase a client's viral load.
- People who use drugs and alcohol should be aware and educated about the dangers of infecting other people in their community.
- If an individual is HIV-positive, he or she risks infecting his or her sexual partners and anyone with whom they may be sharing injection equipment.

WHAT IS HARM REDUCTION?

Harm Reduction strategies:

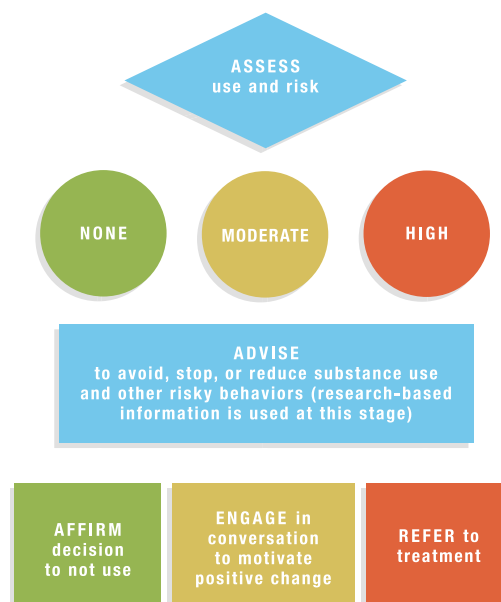
- Help reduce risk that individuals may face when using drugs or alcohol and not necessarily promote abstinence from using
- Aim to make it safer for those who continue to use alcohol and drugs and reduce risk of becoming infected with or transmitting HIV
- Can be effective methods for reducing the spread of HIV among people who inject drugs
- Not only support people who inject drugs in improving their health but also provide services that can support the broader community as well
- Also support the development of trusting relationships between users and peer educators as these interventions are non-stigmatising
- Take a neutral stance on drug use. In this way, health care workers are able to actively address the health risks that people who use drugs and alcohol face, particularly HIV acquisition, and support them in accessing care without stigmatising or isolating them.



HIERARCHY FOR INJECTING DRUG USE

The following levels of risk can be a useful tool when supporting PWID in reducing their risk:

1. You will not get infections from sharing needles if you stop or never start injecting drugs.
2. If you cannot stop using drugs, use them in any way except injecting. If you do not inject drugs, you cannot catch infections through needle sharing.
3. If you cannot stop injecting drugs, do not share needles, cookers, spoons or filters with other drug users, or use new injecting equipment every time. If you use new injection equipment every time you cannot catch viral infections such as HIV through needle sharing.
4. If you need to re-use any equipment, use your own injecting equipment every time. If you re-use your own injection equipment every time you cannot catch viral infections such as HIV unless someone else has used your equipment without your knowledge.
5. If you need to re-use any equipment and you believe you need to use someone else's equipment, then clean needles by an approved method. There is some risk for HIV transmission after needle cleaning, but cleaning in an approved manner will reduce the likelihood of transmission.



WHAT ARE COMMON SIGNS AND SYMPTOMS OF DRUG USE?

Drug use can result in common signs and physical, behavioural and psychological symptoms. A few of these are listed below; however, be aware that that these symptoms could also be associated with a variety of other conditions and are contingent on the types of drugs someone may be using.

Physical symptoms and signs	Behavioural symptoms and signs	Psychological symptoms and signs
• Red eyes or dilated or pinpoint pupils • Changes in appetite • Changes in sleep patterns • Sudden weight loss or gain • Deterioration of physical appearance • Skin manifestations such as abscesses or ulcers • Needle marks or puncture wounds on skin from injecting	• Drop in attendance and lack of interest in work or school • Unexplained need for money that leads to borrowing and stealing • Engaging in secretive or suspicious behaviour • Sudden changes in friends, favorite hang outs, and hobbies • Frequently getting into trouble (fights, accidents, illegal activities) • Possession of drug gear or paraphernalia	• Unexplained change in personality, attitude, and behaviour • Mood swings, irritability, angry outbursts, or unexplained excitement • Periods of unusual hyperactivity, agitation, or giddiness • Lack of motivation and ambition • Appears lethargic or spaced out • Appears fearful, anxious or paranoid with no reason

1. Alcohol (legal/non-medicinal - depressant/downer)
2. Heroin (Illegal/non-medicinal - depressant/downer)
3. Cannabis/marijuana (Illegal/non-medicinal - depressant/downer)
4. Amphetamine – speed (Illegal/non-medicinal - stimulant/upper)
5. Aspirin (legal/medicinal - depressant/downer)
6. Asthma pump (legal/medicinal - stimulant/upper)
7. Nicotine (legal/non-medicinal - stimulant/upper)
8. Caffeine (legal/non-medicinal - stimulant/upper)
9. Stilnox (legal/medicinal - depressant/downer)
10. LSD (Illegal/non-medicinal - - stimulant/upper)
11. Ecstasy Pill (Illegal/non-medicinal - - stimulant/upper)
12. Liquid ecstasy (Illegal/non-medicinal - - stimulant/upper)
13. Tik (Illegal/non-medicinal - - stimulant/upper)
14. Crystal meth (Illegal/non-medicinal - - stimulant/upper)
15. Crack (Illegal/non-medicinal - - stimulant/upper)
16. Cocaine (Illegal/non-medicinal - - stimulant/upper)
17. Mandrax (Illegal/non-medicinal - depressant/downer)
18. Morphine (legal/medicinal - depressant/downer)
19. Nyaope (Whoonga) (Illegal/non-medicinal - stimulant/upper)
20. Pethadine (legal/medicinal - depressant/downer)

FACT SHEET – DRUG INFORMATION

DRUG INFORMATION SHEET 1: THE TRUTH ABOUT ECSTASY

Ecstasy is illegal. It is classified as a dangerous substance with no recognised medical use. Tragically, Ecstasy is one of the most popular drugs among youth today. The majority of users are teenagers and young adults. Mixed with alcohol, Ecstasy is extremely dangerous and can be deadly. So widespread has been the harm of this “designer drug,” that emergency room incidents have skyrocketed more than 1,200% since Ecstasy became the “club drug” of choice at all-night “rave” parties and dance clubs.

WHAT IS ECSTASY

Ecstasy was originally developed by Merck Pharmaceutical in 1912. In its original form it was used as a psychotherapy medication to “lower inhibitions.” It wasn’t until the 1970s that it started being used as a party drug. By the early 1980s, it was being promoted as “the hottest thing in the continuing search for happiness through chemistry,” and the “in drug” for many weekend parties. Still legal in 1984, it was being sold under the brand name “Ecstasy,” but by 1985, the drug had been banned due to safety concerns.

What is called Ecstasy today can contain a wide mixture of substances—from LSD, cocaine, heroin, amphetamine and methamphetamine, to rat poison, caffeine, dog deworming substances, etc. Despite the cute logos dealers put on the pills, this is what makes Ecstasy particularly dangerous; a user never really knows what he is taking. The dangers are increased when users increase the dose seeking a previous high, not knowing they may be taking an entirely different combination of drugs.

Ecstasy most commonly comes in pill form but can also be injected and taken in other ways. Liquid Ecstasy is actually GHB, a nervous system depressant—a substance that can also be found in drain cleaner, floor stripper and degreasing solvents.

IMAGINARY LOVE PILL: OFF WITH THE MASK

Ecstasy is often called “the love pill” because it heightens perceptions of colour and sound and supposedly amplifies sensations when one touches or caresses another, particularly during sex. But Ecstasy often contains hallucinogens, which are drugs that act on the mind and cause people to see or feel things that are not really there. Hallucinogens can throw a person into a scary or sad experience from the past, where he or she gets stuck without even realising it. The image of Ecstasy as a “love pill” is one of many lies that are spread about the drug. Ecstasy is emotionally damaging and users often suffer depression, confusion, severe anxiety, paranoia, psychotic behaviour and other psychological problems.

CONSEQUENCES OF USING ECSTASY

Ecstasy smotheres the natural alarm signals given out by the body. As a result, after taking the drug, an individual risks going beyond his physical limitations and endurance. For example, a person on Ecstasy may not realise that he has become overheated and can faint or even die of heatstroke.

SHORT-TERM EFFECTS OF ECSTASY

- Impaired judgment
- False sense of affection
- Confusion
- Depression
- Sleep problems
- Severe anxiety
- Paranoia
- Drug cravings
- Muscle tension
- Faintness and chills or swelling
- Involuntary teeth clenching
- Blurred vision
- Nausea

LONG-TERM EFFECTS OF ECSTASY

- Long-lasting brain damage affecting thought and memory
- Damage to portions of the brain that regulate critical functions such as learning, sleep and emotion
- It is as if the brain switchboard was torn apart, then rewired backwards

- Damage to nerve branches and nerve endings
- Depression, anxiety, memory loss
- Kidney failure
- Haemorrhaging (bleeding)
- Psychosis
- Cardiovascular collapse (heart failure)
- Convulsions
- Death

IS ECSTASY ADDICTIVE? FOUR VERY REAL DANGERS EXIST:

- DANGER NO. 1: By 1995, less than 10% of Ecstasy pills on the market were pure as in its original form. Today's Ecstasy user is usually taking a mix of a wide variety of drugs, and often toxic (poisonous) substances.
- DANGER NO. 2: One has to continually increase the amount of the drug one takes in order to feel the same effects. Users say the effect of Ecstasy is greatly reduced after the first dose. And as a person takes more of the drug, the negative effects also increase.
- DANGER NO. 3: Users feel there is sometimes a need to use other drugs such as heroin or cocaine to help cope with the mental and physical pain that results after one "comes down" from Ecstasy; 92% of those who take Ecstasy also abuse other, even harder drugs.
- DANGER NO. 4: The false idea that a person only feels good with Ecstasy leads to a desire to take it more often than just at raves and techno parties.

Drug info sheets: <http://www.drugfreeworld.org/drugfacts/ecstasy.html>

DRUG INFORMATION SHEET 2: WHAT IS COCAINE?

The word cocaine refers to the drug in a powder form or crystal form. In its crystal form, it is called crack cocaine. The powder is usually mixed with substances such as corn starch, talcum powder, sugar or other drugs such as procaine (a local anaesthetic) or amphetamines. Extracted from coca leaves, cocaine was originally developed as a painkiller. It is most often snorted, with the powder absorbed into the bloodstream through the nasal tissues. It can also be swallowed or rubbed into the gums. To more rapidly absorb the drug into the body, abusers inject it, but this substantially increases the risk of overdose. Inhaling it as smoke or vapor speeds absorption with less health risk than injection.

A DEADLY WHITE POWDER

Cocaine is one of the most dangerous drugs known to man. Once a person begins taking the drug, it has proven almost impossible to become free of its grip physically and mentally. Physically it stimulates key receptors (nerve endings that sense changes in the body) within the brain that, in turn, create a euphoria (a feeling of joy and excitement) to which users quickly develop a tolerance. In other words you need to take more each time to get the same effect you get when you first take it. Cocaine use can lead to death from respiratory (breathing) failure, stroke, cerebral haemorrhage (bleeding in the brain) or heart attack. Children of cocaine-addicted mothers come into the world as addicts themselves. Many suffer birth defects and many other problems. Despite its dangers, cocaine use continues to increase — probably because users find it so difficult to escape from the first steps taken down the long dark road that leads to addiction.

WHY IS COCAINE SO HIGHLY ADDICTIVE?

Cocaine creates the greatest psychological dependence of any drug. It stimulates key pleasure centres within the brain and causes extremely heightened euphoria. A tolerance to cocaine develops quickly—the addict soon fails to achieve the same high experienced earlier from the same amount of cocaine.

EFFECTS OF COCAINE

SHORT-TERM EFFECTS

- Loss of appetite
- Increased heart rate, blood pressure, body temperature
- Contracted blood vessels (decreases blood flow to brain and heart)
- Increased rate of breathing
- Dilated pupils (widened)
- Disturbed sleep patterns
- Nausea
- Hyper-stimulation (over energetic and excited)
- Bizarre, erratic, sometimes violent behaviour

- Hallucinations, hyper-excitability, irritability
- Tactile hallucination that creates the feeling of bugs burrowing under the skin
- Intense euphoria
- Anxiety and paranoia
- Depression
- Intense drug craving
- Panic and psychosis
- Convulsions, seizures and sudden death from high doses
- Cocaine causes heart, kidney, brain and lung damage.

LONG-TERM EFFECTS

- Permanent damage to blood vessels of heart and brain
- High blood pressure, leading to heart attacks, strokes, and death
- Liver, kidney and lung damage
- Destruction of tissues in nose if sniffed
- Respiratory failure if smoked
- Infectious diseases and abscesses if injected
- Malnutrition, weight loss
- Severe tooth decay
- Auditory and tactile hallucinations (hearing things and feeling things that are not real)
- Sexual problems, reproductive damage and infertility (for both men and women)
- Disorientation, apathy, confused exhaustion
- Irritability and mood disturbances
- Increased frequency of risky behavior
- Delirium or psychosis
- Severe depression
- Tolerance and addiction (even after just one use)

DRUG INFORMATION SHEET 3: WHAT IS HEROIN?

Heroin is produced from the resin of the opium poppy and is the most dangerous and addictive drug. Pure heroin is a white, odourless crystalline-like powder with a bitter taste. The browner the colour, the more impurities it contains. It is often diluted with starch, sugars such as glucose, powdered milk, baby powder, washing powder, strychnine or other poisons before being sold.

Opioids act on many places in the brain and nervous system

- Opioids can depress breathing by changing neurochemical activity in the brain stem, where automatic body functions such as breathing and heart rate are controlled.
- Opioids can increase feelings of pleasure by altering activity in the limbic system, which controls emotions.
- Opioids can block pain messages transmitted through the spinal cord from the body.

HOW IS HEROIN LINKED TO PRESCRIPTION DRUG ABUSE?

Harmful health consequences can result from the abuse of medications that are prescribed for the treatment of pain. People often assume prescription pain relievers are safer than illegal drugs because they are medically prescribed. However, when these drugs are taken for reasons or in ways or amounts not intended by a doctor, or taken by someone other than the person for whom they are prescribed, this can result in severe adverse health effects including addiction, overdose, and death, especially when combined with other drugs or alcohol. Research now suggests that abuse of these medications may actually open the door to heroin use. Nearly half of young people who inject heroin surveyed in three recent studies reported abusing prescription drugs before starting to use heroin. Some individuals reported switching to heroin because it is cheaper and easier to obtain than prescription opioids.

SHORT-TERM EFFECTS OF HEROIN USE

“Rush”

- Depressed respiration (slow breathing)
- Clouded mental functioning (can't think clearly)
- Nausea and vomiting

- Suppression of pain
- Spontaneous abortion

LONG-TERM EFFECTS

- Addiction
- Infectious disease (e.g., HIV, hepatitis B and C)
- Collapsed veins
- Bacterial infections
- Abscesses
- Infection of heart lining and valves
- Arthritis and other rheumatologic problems
- Liver and kidney disease
- Insomnia (inability to sleep)
- Constipation
- Lung complications (including various types of pneumonia and tuberculosis) may result from the poor health of the user as well as from heroin's effect of depressing respiration
- Mental disorders such as depression and antisocial personality disorder
- Men often experience sexual dysfunction
- Women's menstrual cycles often become irregular.

There are also specific consequences associated with different routes of administration. For example, people who repeatedly snort heroin can damage the mucosal tissues in their noses as well as perforate the nasal septum (the tissue that separates the nasal passages).

Sharing of injection equipment or fluids can lead to some of the most severe consequences of heroin abuse:

- infections with hepatitis B and C, HIV, and a host of other blood-borne viruses, which drug abusers can then pass on to their sexual partners and children.

HOW DOES HEROIN USE AFFECT PREGNANT WOMEN?

Heroin use during pregnancy can result in neonatal abstinence syndrome (NAS). NAS occurs when heroin passes through the placenta to the foetus during pregnancy, causing the baby to become dependent along with the mother. Symptoms include excessive crying, fever, irritability, seizures, slow weight gain, tremors, diarrhoea, vomiting, and possibly death. NAS requires hospitalisation and treatment with medication (often morphine) to relieve symptoms; the medication is gradually tapered off until the baby adjusts to being opioid-free.

DRUG INFORMATION SHEET 4: WHAT IS CRYSTAL METH?

Crystal meth is a white crystalline drug that people take by snorting it (inhaling through the nose), smoking it or injecting it with a needle. Some even take it orally, but all develop a strong desire to continue using it because the drug creates a false sense of happiness and well-being—a rush (strong feeling) of confidence, hyperactivity and energy. One also experiences decreased appetite. These drug effects generally last from six to eight hours, but can last up to twenty-four hours. The first experience might involve some pleasure, but from the start, methamphetamine begins to destroy the user's life.

Methamphetamine is a man-made chemical, unlike cocaine, for instance, which comes from a plant. Meth is commonly manufactured in illegal, hidden laboratories, mixing various forms of chemicals to boost its potency (effects). These dangerous chemicals are potentially explosive and because the meth cooks are drug users themselves and disoriented, they are often severely burned and disfigured or killed when their preparations explode. Such accidents endanger others in nearby homes or buildings.

Crystal meth is used by individuals of all ages, but is most commonly used as a “club drug,” taken while partying in night clubs or at rave parties. It is a dangerous and potent chemical and, as with all drugs, a poison that first acts as a stimulant but then begins to systematically destroy the body. Thus it is associated with serious health conditions, including memory loss, aggression, psychotic behaviour and potential heart and brain damage. Highly addictive, meth burns up the body's resources, creating a devastating dependence that can only be relieved by taking more of the drug. Crystal meth's effect is highly concentrated, and many users report getting hooked (addicted) from the first time they use it.

THE SHORT-TERM AND LONG-TERM IMPACT OF THE INDIVIDUAL

When taken crystal meth creates a false sense of well-being and energy, and so a person will tend to push his body faster and further than it is meant to go. Thus, drug users can experience a severe “crash” or physical and mental breakdown after the effects of the drugs wear off. Because continued use of the drug decreases natural feelings of hunger, users can experience extreme weight loss. Negative effects can also include disturbed sleep patterns, hyperactivity, nausea, delusions of power, increased aggressiveness and irritability. Other serious effects can include insomnia, confusion, hallucinations, anxiety and paranoia. In some cases, use can cause convulsions that lead to death.

LONG-TERM DAMAGE

In the long term, meth use can cause irreversible harm: increased heart rate and blood pressure; damaged blood vessels in the brain that can cause strokes or an irregular heartbeat that can, in turn, cause cardiovascular collapse or death; and liver, kidney and lung damage.

Users may suffer brain damage, including memory loss and an increasing inability to grasp abstract thoughts. Those who recover are usually subject to memory gaps and extreme mood swings.

IMMEDIATE EFFECTS

Depending on how it is taken, the effects may be felt immediately (through injecting or smoking) or within 30 minutes (if snorted or swallowed).

LOW TO MODERATE DOSES

- Feelings of euphoria, excitement and a sense of wellbeing
- Increased confidence and motivation
- A sense of power and superiority over others
- Increased talkativeness
- Repetition of simple acts
- Nervousness, anxiety, agitation and panic
- Increased libido
- Paranoia
- Hallucinations
- Irritability, hostility and aggression
- Headaches
- Dizziness
- Feeling more awake and alert, reduced need for sleep and difficulty sleeping
- Abrupt shifts in thought and speech that can make people difficult to understand
- Enlarged (dilated) pupils
- Dry mouth
- Increased breathing rate
- Shortness of breath (from smoking it)
- Increased heart rate and blood pressure

- Irregular heart beat, palpitations
 - Chest pain
 - Reduced appetite
 - Stomach cramps
 - Stomach irritation (if swallowed)
 - Feeling more energetic
 - Increased sweating
 - Increased body temperature
 - Faster reaction times
 - Feelings of increased strength
 - Itching, picking and scratching.
-

HIGHER DOSES

A high dose can cause a person to overdose. This means that a person has taken more than their body can cope with. Injecting runs a greater risk of overdose due to large amounts of the drug entering the blood stream and quickly travelling to the brain.

People may also experience:

- Blurred vision
 - Tremors
 - Irregular breathing
 - Loss of coordination
 - Collapse
 - Rapid pounding heart
 - Violent or aggressive behaviour
 - Hallucinations
 - Seizures
 - Stroke
 - Coma.
-

COMING DOWN

As the effects of begin to wear off, a person may experience:

- Feeling restless, irritable and anxious
 - Aggression, that may lead to violence
 - Tension
 - Radical mood swings
 - Depression
 - Paranoia
 - Lethargy
 - Total exhaustion.
-

LONG-TERM EFFECTS

Some of the long-term effects include:

- Malnutrition and rapid weight loss due to reduced appetite
- Chronic sleeping problems
- Reduced immunity and increased susceptibility to infections due to not sleeping or eating properly
- Cracked teeth and other dental problems from clenching the jaw, grinding the teeth, dry mouth and poor hygiene
- High blood pressure and rapid and irregular heartbeat which place stress on the heart and can increase the risk of heart-related complications such as heart attack and heart failure
- Increased strain on the kidneys which can result in kidney failure

- Increased risk of stroke
- Depression, anxiety and tension
- Paranoia
- Violence
- Panic and confusion
- Muscle rigidity
- Breathlessness
- Damage to the heart muscle
- Psychological problems such as poor memory and concentration.

THE STAGES OF THE METH “EXPERIENCE”

1) The Rush—A rush is the initial response the user feels when smoking or injecting methamphetamine. During the rush, the heartbeat races and metabolism, blood pressure and pulse soar. Unlike the rush associated with crack cocaine, which lasts for approximately two to five minutes, the rush can continue for up to thirty minutes.

2) The High—The rush is followed by a high, sometimes called “the shoulder.” During the high, the user often feels aggressively smarter and becomes argumentative, often interrupting other people and finishing their sentences. The delusional effects can result in a user becoming intensely focused on an insignificant item, such as repeatedly cleaning the same window for several hours. The high can last four to sixteen hours.

3) The Binge—A binge is the uncontrolled use of a drug or alcohol. It refers to the user’s urge to maintain the high by smoking or injecting more. The binge can last three to fifteen days.

4) Tweaking—A meth abuser is most dangerous when experiencing a phase of addiction called “tweaking”—a condition reached at the end of a drug binge when there is no longer a rush or a high. Unable to relieve the horrible feelings of emptiness and craving, a user loses his sense of identity. Intense itching is common and a user can become convinced that bugs are crawling under his skin. Unable to sleep for days at a time, the user is often in a completely psychotic state and exists in his/her own world, seeing and hearing things that no one else can perceive. Hallucinations are so vivid that they seem real and the user is completely disconnected from reality. The potential for self-mutilation is high.

5) The Crash—To a binge user, the crash happens when the body shuts down, unable to cope with the drug’s overwhelming effects. This results in a long period of sleep. The crash can last one to three days.

6) Meth Hangover—After the crash, the abuser returns in a deteriorated state, starved, dehydrated and utterly exhausted physically, mentally and emotionally. This stage ordinarily lasts from two to fourteen days.

7) Withdrawal—Often thirty to ninety days can pass after the last drug use before the user realises that he is in withdrawal. First, they become depressed, lose energy and the ability to

experience pleasure. Then the craving for more hits, and the user often becomes suicidal. Since meth withdrawal is extremely painful and difficult, most users revert. Thus 93% of those in traditional treatment return to abusing crystal meth.

DRUG INFORMATION SHEET 5: WHAT IS CANNABIS?

Cannabis sativa, is a species of the Cannabinaceae family of plants. Cannabis is also known as ganja, grass, hashish, hemp, Indian hemp, marijuana, pot, reefer and weed. The dried leaves and flowers of the cannabis plant are known as marijuana, which can be smoked (through a pipe or bong or hand-rolled into a joint) or taken orally with food (baked in cookies).

WHAT ARE THE EFFECTS OF CANNABIS?

The effects experienced by the cannabis user will depend on the dose, method of administration, prior experience, any concurrent drug use, personal expectations, mood state and the social environment in which the drug is used.

EFFECTS OF CANNABIS INCLUDE:

- An altered state of consciousness - “high”, very happy, euphoric, relaxed, sociable and uninhibited.
- Distorted perceptions of time and space. The user may feel more sensitive to things around them, and may also experience a more vivid sense of taste, sight, smell and hearing.
- Increased pulse and heart rate, bloodshot eyes, dilated pupils, and often increased appetite.
- Impaired coordination and concentration, making activities such as driving a car or operating machinery difficult and dangerous.
- Negative experiences, such as anxiousness, panic, self-consciousness and paranoid thoughts.
- Large quantities of cannabis cause users to become sedated or disoriented and experience toxic psychosis - not knowing who they are, where they are, or what time it is, fluctuating emotions, fragmentary thoughts, paranoia, panic attacks, hallucinations and feelings of unreality.
- The effects of cannabis are felt within minutes, reach their peak in 10 to 30 minutes, and may linger for two or three hours.

MEDICAL CANNABIS (MEDICAL MARIJUANA)

There are eight medical conditions for which patients can use cannabis:

- Cancer
- Glaucoma
- HIV/AIDS
- Muscle spasms
- Seizures
- Severe pain
- Severe nausea
- Cachexia or dramatic weight loss and muscle atrophy (wasting syndrome)

DRUG INFORMATION SHEET 6: WHAT IS LSD?

LSD is an odourless and colourless drug available in two forms: paper stamps impregnated with LSD or micro-tablets (“microdots”) containing LSD in very low concentrations per tablet.

WHAT ARE THE RISKS OF LSD?

The effects of LSD are unpredictable. They depend on the amount taken, the person’s mood and personality, and the surroundings in which the drug is used. It is a roll of the dice - a racing, distorted high or a severe, paranoid low. The worst part is that the LSD user is unable to tell which sensations are created by the drug and which are part of reality.

Taken in a large enough dose, LSD produces delusions and visual hallucinations. The user’s sense of time and self changes. Sizes and shapes of objects become distorted, as do movements, colours and sounds. Even one’s sense of touch and the normal bodily sensations turn into something strange and bizarre. Sensations may seem to “cross over,” giving the user the feeling of hearing colours and seeing sounds. These changes can be frightening and can cause panic.

The ability to make sensible judgments and see common dangers is impaired. An LSD user might try to step out a window to get a “closer look” at the ground. He might consider it fun to admire the sunset, blissfully unaware that he is standing in the middle of a busy intersection.

Many LSD users experience flashbacks, or a recurrence of the LSD trip, often without warning, long after taking LSD. Bad trips and flashbacks are only part of the risks of LSD use. LSD users may manifest relatively long-lasting psychoses or severe depression.

Because LSD accumulates in the body, users develop a tolerance for the drug. In other words, some repeat users have to take it in increasingly higher doses to achieve a “high.” This increases the physical effects and also the risk of a bad trip that could cause psychosis.

PHYSICAL EFFECTS

- Dilated pupils
- Higher or lower body temperature
- Sweating or chills (“goose bumps”)
- Loss of appetite
- Sleeplessness
- Dry mouth
- Tremors

MENTAL EFFECTS

- Extreme changes in mood, anywhere from a spaced-out “bliss” to intense terror
- Delusions
- Visual hallucinations
- An artificial sense of euphoria or certainty

- Distortion of one’s sense of time and identity
- Impaired depth perception
- Impaired time perception, distorted perception of the size and shape of objects, movements, color, sounds, touch and the user’s own body image
- Severe, terrifying thoughts and feelings
- Fear of losing control
- Panic attacks
- Flashbacks, or a recurrence of the LSD trip, often without warning long after taking LSD
- Severe depression or psychosis

DRUG INFORMATION SHEET 7: WHAT IS ALCOHOL?

Alcohol is a drug. It is classed as a depressant, meaning that it slows down vital functions resulting in slurred speech, unsteady movement, disturbed perceptions and an inability to react quickly. Although classified as a depressant, the amount of alcohol consumed determines the type of effect. Most people drink for the stimulant effect, such as a beer or glass of wine taken to “loosen up.” But if a person consumes more than the body can handle, they then experience alcohol’s depressant effect. They start to feel “stupid” or lose coordination and control.

ALCOHOL CONTENT

Fermented drinks, such as beer and wine, contain from 2% alcohol to 20% alcohol. Distilled drinks, or liquor, contain from 40% to 50% or more alcohol. The usual alcohol content for each is:

- Beer 2–6% alcohol
- Cider 4–8% alcohol
- Wine 8–20% alcohol
- Tequila 40% alcohol
- Rum 40% or more alcohol
- Brandy 40% or more alcohol
- Gin 40–47% alcohol
- Whiskey 40–50% alcohol
- Vodka 40–50% alcohol
- Liqueurs 15–60% alcohol

UNDERSTANDING HOW ALCOHOL AFFECTS THE BODY

Alcohol is absorbed into the bloodstream via small blood vessels in the walls of the stomach and small intestine. Within minutes of drinking alcohol, it travels from the stomach to the brain, where it quickly produces its effects, slowing the action of nerve cells. Alcohol is also carried by the bloodstream to the liver, which eliminates the alcohol from the blood through a process called “metabolising,” where it is converted to a non-toxic substance. The liver can only metabolise a certain amount at a time, leaving the excess circulating throughout the body. Thus the intensity of the effect on the body is directly related to the amount consumed.

YOUNG PEOPLE VERSUS ADULTS – WHAT’S THE DIFFERENCE?

A young person’s body cannot cope with alcohol the same way an adult’s can. Drinking is more harmful to teens than adults because their brains are still developing throughout adolescence and well into young adulthood. Drinking during this critical growth period can lead to lifelong damage in brain function, particularly as it relates to memory, motor skills (ability to move) and coordination. According to research, young people who begin drinking before age 15 are four times more likely to develop alcohol dependence than those who begin drinking at age 21.

WHAT IS ALCOHOLISM OR ALCOHOL DEPENDENCE?

Alcohol dependence (alcoholism) consists of four symptoms:

- Craving: a strong need, or compulsion, to drink.
- Loss of control: the inability to limit one’s drinking on any given occasion.
- Physical dependence: withdrawal symptoms, such as nausea, sweating, shakiness and anxiety, occur when alcohol use is stopped after a period of heavy drinking.
- Tolerance: the need to drink greater amounts of alcohol in order to get high.

SHORT-TERM EFFECTS OF ALCOHOL

Depending on how much is taken and the physical condition of the individual, alcohol can cause:

- Slurred speech
- Drowsiness
- Vomiting
- Diarrhea
- Upset stomach
- Headaches
- Breathing difficulties
- Distorted vision and hearing
- Impaired judgment
- Decreased perception and coordination
- Unconsciousness
- Anaemia (loss of red blood cells)
- Coma
- Blackouts (memory lapses, where the drinker cannot remember events that occurred while under the influence)

LONG-TERM EFFECTS OF ALCOHOL

Binge drinking and continued alcohol use in large amounts are associated with many health problems, including:

- Unintentional injuries such as car crash, falls, burns, drowning
- Intentional injuries such as firearm injuries, sexual assault, domestic violence
- Increased on-the-job injuries and loss of productivity
- Increased family problems, broken relationships
- Alcohol poisoning
- High blood pressure, stroke, and other heart-related diseases
- Liver disease
- Nerve damage
- Sexual problems
- Permanent damage to the brain
- Vitamin B1 deficiency, which can lead to a disorder characterised by amnesia, apathy and disorientation
- Ulcers
- Gastritis (inflammation of stomach walls)
- Malnutrition
- Cancer of the mouth and throat

HOW PEER EDUCATORS CAN HELP



PRINCIPLES OF GOOD PRACTICE IN ADAP EDUCATION

ADAP EDUCATION MUST INVOLVE A TRAINED FACILITATOR/PEER EDUCATOR

- Peer educators must have specific knowledge of the students and the learning context
- Peer educators are best placed to identify and respond to the needs of students and to coordinate drug education with other activities
- Selected external resources are recommended for use where appropriate

ADAP PROGRAMMES SHOULD HAVE SEQUENCE, PROGRESSION AND CONTINUITY

- Health messages must be regular, timely and come from a credible source

EDUCATION MESSAGES SHOULD BE CONSISTENT AND COHERENT, AND BASED ON STUDENT NEEDS

- Programmes must be linked to the overall goal of prevention and harm reduction through behaviour change
- They must be in line with broader policies and practices that reinforce programme objectives to maximise the potential for success
- They must provide information about the harmful long-term effects of drug use as well as demonstrate understanding of the perceived benefits for users in order to demonstrate empathy and understanding of the challenges for users

EFFECTIVE ADAP EDUCATION PROGRAMMES:

- Include a balance of knowledge, attitudes and skills
- Are participant centred
- Contribute to long term positive outcomes in the health curriculum
- Should reflect an understanding of characteristics of the individual, the social context, the drug, and the interrelationship of these factors
- Involve students in at both planning and implementation stage
- Respond to developmental, gender, cultural, language, socio- economic and lifestyle differences relevant to the levels of student drug use
- Address the motivations for drug use derived from influences such as culture and gender
- Involve a collaborative approach to reinforce desired behaviours
- Provide a supportive environment for programmes

The emphasis of education should be on substance use likely to occur in the target group, and use that causes the most harm to the individual and society.

KNOWLEDGE, ATTITUDES, VALUES AND SKILLS FOR PEER EDUCATORS INVOLVED IN ADAP PROGRAMMES

Knowledge	Attitudes	Values analysis & clarification skills	Skills	Coping skills
General: knowledge about alcohol and drugs (composition/legality/effects)	Unconditional acceptance	Skills for identifying important influences on values & attitudes, and aligning values, attitudes & behaviour	Empathy	Self-control skills
Social, emotional, physical and financial impact	Warmth, empathy, genuineness	Non-judgmental	Active listening	Coping with (peer) pressure
Current issues, policies, protocols and laws	Respect	Non-discriminatory	Giving & receiving feedback	Time management skills
Current trends in use and abuse (local and global)	Conviction regarding the legitimacy of the issues	No stereotyping	Decision making skills	Dealing with emotions: grief, anxiety
Available resources	Care and compassion	Belief in the potential to change	Assertion & refusal skills	Dealing with difficult situations (conflict, loss, abuse trauma)
Knowledge of addiction		Understanding of social disadvantage	Critical and creative thinking skills	Help seeking skills
			Problem solving skills	
			Skills for assessing risks	
			Skills for information gathering and generating alternatives	
			Group management skills	
			Facilitation skills	

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CHAPTER 2: **PEER MENTOR GUIDE**



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TRAINING OF PEER MENTORS: PROGRAMME OVERVIEW

BACKGROUND AND CONTEXT

Most tertiary and post school institutions are focused on producing qualified graduates. Their primary objectives are teaching, assessing performance, student administration, budget effectiveness, assessing the needs of the labour market and to some extent research and application. Historically there has been little regard for issues such as the health status or health needs of its students. Needless to say, institutional strategic plans, budgets and protocols seldom included focus or plans relating to the student or staff wellness management. One topic that has remained largely outside the scope of concern in many tertiary institutions throughout the world is HIV/AIDS even though the effect of the epidemic on the education sector is well documented. The World Bank reminds us that higher education is a critical pillar of human development worldwide. It trains teachers, technicians, artisans, environmental specialists, civil servants, humanists, entrepreneurs, scientists, social scientists, doctors, nurses, and others to develop the capacity and skills that drive local economies, support civil society, teach children, lead effective governments and make important decisions which affect entire societies (World Bank, 2002). In this light, it becomes a crucial area of focus.

WHY PEER EDUCATION AS A STRATEGY?

- Peer education as a strategy within a post school context empowers young people by working with other young people and relies on the positive strength, power and resilience of the peer group. By exposing selected peers to appropriate training and support, they become the drivers of the educational process
- Peer education relies on the interaction of two similar individuals (people of the same age or health status)
- Evidence suggests that information received from someone of the same group is more readily accepted and trusted
- It is a common trend that young people tend to talk with their peers about most subjects, including sensitive issues such as reproductive health and HIV/AIDS
- A trained peer educator can help individuals with questions and concerns about HIV/AIDS
- Peer education programmes tend to be quite flexible, rooted in the realities of individual communities, and can be used in a variety of settings and in combination with other activities and programmes
- Peer education programmes can provide strong benefits to peer educators themselves

- Peer education programmes allow for direct participation of young people in programmes designed to affect them, thereby promoting positive life skills such as leadership and communication as well as creating opportunities for mentoring and future job contacts

Peer education programmes can be cost effective

- Peer educators are in the ideal position to give voice to their own (student population) health needs, so that if they're empowered with knowledge and authority they can be trained to play a significant role supporting the health (social, mental and physical wellbeing) of their peers

WHY TRAIN PEER EDUCATORS?

Peer educators have the potential to be trained to:

- Understand the facts about HIV/AIDS
- Communicate effectively with individuals
- Be comfortable with discussing sex and sexuality
- Have an understanding and compassion for infected individuals
- Listen effectively
- Respect the confidentiality of peers

ENVISAGED TRAINING OUTCOMES

Upon implementation, this training course will deliver peer educators who are trained to support and educate their peers at both individual and group levels. The services they deliver will lead to increased access to health and social services delivery among student communities. Their actions, and its results, will ultimately play a significant role in the development of a service that is preventive and promotive in nature. It is envisaged that trained peer educators will be able to:

- Give voice to the health needs of the student population
- Disseminate information and create awareness around critical health and wellness issues
- Communicate clear-cut promotive and preventive health promotion messages, specifically to mitigate against the quadruple burden of disease
- Organise and conduct educational events to empower their peers with knowledge around prevention and treatment
- Interact on a one-to one basis to screen, educate and refer their peers in accordance with their health needs
- Assist to identify and follow up with high-risk individuals and groups
- Advocate for behaviour change around high risk behaviour

DEFINING PEER EDUCATION

WHAT IS PEER EDUCATION?

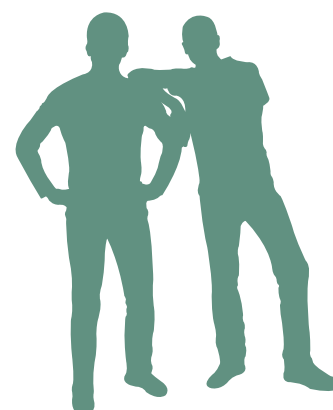
In general, peer education is defined as a process, a strategy, a communication channel and a tool. Most commonly, in terms of youth, it is viewed as a “process whereby well-trained and motivated young people undertake informal or organised educational activities with their peers (those similar to themselves in age, background or interests).”

In practice, peer education is a term widely used to describe a range of strategies where people from a similar age group, background, culture and/or social status educate and inform each other about a wide variety of issues. Although peer education is used with different populations, in this context, it focuses on programmes for young emerging adults.

Through a participatory process, peer education creates an environment where young people feel safe and able to share information, skills and values.

HOW DOES PEER EDUCATION DIFFER FROM OTHER PEER-LED APPROACHES?

Peer education activities are interactive and generally take place over an extended period in the form of a planned calendar of educational events, as opposed to a one-time event. Venues for peer education include campus halls, lecture theatres, social clubs, cafeterias or any place where young people spend time together. Peers can play various roles in helping other young people maintain and improve their health. Some of these roles require more training and involvement than others. Information sharing could involve young people distributing health-related materials at a special event and might require only brief training and short-term involvement. Peer education typically provides ongoing sessions aimed at helping young people build skills and change risky behaviours which involves more than information sharing. Peer counselling, on the other hand, goes beyond peer education to provide psychological support and should only be conducted by well-trained counsellors.



DIFFERENT TYPES OF PEER-LED APPROACHES

	Information Sharing	Peer Education	Peer Counselling
Objectives	Awareness Information Attitude change	Awareness Information Attitude change Skills development	Information Attitude change Self-esteem Psychological support
Coverage	High	Medium	Low
Intensity	Low	Medium/high	High
Confidentiality	None	Important	Essential
Focus	Community Large Groups	Small groups In some cases, individuals	Individuals In some cases, homogeneous groups
Training required	Brief	Structured workshop and refresher courses	Intense and long
Examples of activities	Informative theatre or puppet show	Repeated group events based on a curriculum	Counselling of young people living with HIV Clinic-based youth counselling

BENEFITS OF USING THE PEER EDUCATION APPROACH

The rationale behind peer education is that peers:

- Can be a trusted and credible source of information to programme managers
- Are often trusted by their peers more than staff or external “experts”
- Share similar experiences and are therefore better placed to provide relevant, meaningful, explicit and honest information
- Are better able to address issues related to sexuality than non-peers
- Often have an effect on knowledge, attitudes, norms, motivation, and behaviour
- Are able to access marginalised or vulnerable groups
- Have increased access to services and service providers

GLOBAL RESEARCH ON THE BENEFITS OF PEER EDUCATION (ADAMCHAK 2006) SHOWS THAT THIS APPROACH CAN:

- Positively influence young people’s attitudes and knowledge about reproductive health and HIV
- Influence youth to decrease their number of sexual partners and increase their use of condoms and other contraception
- Reach groups of youth with education and health materials, such as brochures and condoms
- Increase the use of sexual and reproductive health services by vulnerable young people
- Improve community norms about youth and sexual behaviour

IMPORTANT ISSUES FOR PROGRAMME MANAGERS

- Be aware that projects that promote abstinence without addressing other behaviours may have some benefits but are unlikely to change long-term behaviours. This means that building skills for behaviour change must be included in educational events.
- It is necessary to have a plan for ongoing recruitment and training because peer educators are a workforce that is constantly in transition. They eventually develop out of peer programmes (graduate, leave etc.). Hence new peer educators must be trained frequently.
- Peer educators require hands on support and mentorship for the following reasons:
- Young people require more training and supervision than adults
 - o They may require adult support and reinforcement to challenge peers to develop critical thinking or to change social or cultural norms
 - o They may require assistance and support to avert stigma associated with speaking out on sensitive topics
 - o They may lack the maturity, skills, and knowledge to respond to challenges from their peers on contentious issues

Therefore, successful peer education programmes require intensive planning, coordination, supervision and resources. Moreover, for peer education programmes to work they must motivate the peer educators and make them feel they are valued members of the organization. This instils a feeling of ownership, which shows in their work and which young educators will pass onto their peers.

Peer education programmes do not take place in a vacuum.

They are shaped by, and respond to, prevailing social norms and community contexts. Managers should take the social context of the programme into consideration at all stages of planning and implementation, identifying potential opposition and supportive networks.

PEER EDUCATION: A THEORETICAL FRAMEWORK

THEORIES OF RELEVANCE

Peer education is a strategy that has grown out of a number of behavioural change theories. These theories attempt to address the debates that surround the nature of behaviour change. At the heart of these debates are questions about how the individual functions in society, and how that society influences the individual’s behaviour. There are a number of barriers to understanding new health messages, and peer educators have the advantage of being able to frame messages in a way that resonates with young audiences, so that they can identify with both the message and the messenger.

Behaviour change theories can be divided into two broad categories that address these debates. These categories can be summarised as those that focus on individual psychological processes, and those that focus on community or societal changes.

THEORIES OF INDIVIDUAL BEHAVIOUR CHANGE

Individual behaviour change theories suggest that the components necessary for an individual to change behaviour include an ability to explore personal attitudes, beliefs and intentions; an understanding of personal risk; and the knowledge and opportunity to practice skills that will reduce the possibility of risk and the practice of risk-taking behaviours.

THE IMBR MODEL: INFORMATION, MOTIVATION, BEHAVIOURAL SKILLS AND RESOURCES

The IMBR model (information, motivation, behavioural skills and resources) addresses health-related behaviour in a way that is comprehensive and clear, and that can be applied to and across different cultures. Peer education in the context of the IMBR focuses on:

- Information (the “what”) relating to knowledge and facts relevant to the focus of the project e.g. HIV or wellness management
- Motivation (the “why”) relating to the risks and benefits pertaining to lifestyle choices and behaviour
- Behavioural skills (the “how”) that involve transferring skills that empower peers to make healthy responsible choices that reduce risk

IN THE CONTEXT OF PEER EDUCATION

A programme that does not have a comprehensive approach, including all four IMBR concepts, probably lacks essential components for reducing risk behaviour and promoting healthier lifestyles. A programme might, for example, explain to young people the need for contraception and describe contraceptive methods, but might omit to demonstrate their proper use. Participants would then be informed about what to do but not how to do it. Other programmes might inform participants of the what and the how of certain healthy behaviours, but not give them strong emotional or intellectual reasons as to why they would want to practise such behaviours. Although resources can be considered part of ‘information’, it is important to provide young people with information about where to access appropriate resources or services beyond the scope of peer education sessions. Such resources might include, for example, youth friendly clinics, counselling services, HIV/STI and pregnancy testing, care programmes, and commodities (e.g. condoms and contraceptives).

- Resources (the “where”) pointing to available and accessible resources relevant to the needs of the peer community

As an example, if a young person knows that safer sex may prevent the spread of HIV, he or she might still need to be motivated practice safer sex.

THEORY OF REASONED ACTION

The theory of reasoned action states that the intention or desire to adopt a new behaviour is influenced both by the individuals’ beliefs (i.e. family norms) or community standards to which the individual is exposed.

Therefore, the intention of a person to adopt a recommended behaviour is determined by the person’s attitudes and beliefs about the consequences of the behaviour. For example, a young woman who thinks that using contraception will have positive results for her, will have a positive attitude towards contraceptive use.

The person’s subjective or personal viewpoint about an issue is also influenced by significant others as well as the norm or the standard in their community or culture. This is based on what others think he or she should do, and whether important individuals approve or disapprove of the behaviour.

IN THE CONTEXT OF PEER EDUCATION

The relevance to peer education is that:

- Subjective beliefs and norms are shaped by the education input and more especially by the methods employed to deliver crucial messages
- Young people’s attitudes are highly influenced by their perceptions of what their peers do and think
- Young people are highly motivated by the expectations of respected individuals

Clearly, for behaviour change to occur, information, skills, motivation, and resources need to be in place, but factors such as cultural heritage and socio-economic status can undermine the outcomes of even the best intervention efforts.

HEALTH BELIEF MODEL

The health belief model was developed in the early 1950s by social psychologists Godfrey Hochbaum, Stephen Kegels, and Irwin Rosenstock. It was used to explain and predict health behaviour, mainly through perceived susceptibility, perceived barriers, and perceived benefits. That is, if a person desires a particular health outcome he or she will take actions to help bring about that outcome. Hence, the peer education strategy feeds this model by influencing perceptions through persuasive messaging. This model suggests that if a person has a desire to avoid illness or to get well (value) and the belief that a specific health action would prevent illness (expectancy), then a positive behavioural action would be taken towards that behaviour. Unfortunately, this model of behaviour change does not sufficiently take into account things like habits, attitudes, and emotions. Although the model is useful, the effects of a number of factors on behaviour (culture, social influence, socio-economic status, personal experiences, etc.) need to be considered if the model is to be integrated into peer education work.



IN THE CONTEXT OF PEER EDUCATION

The health belief model's most relevant concept is that of perceived barriers, or a person's opinion of the tangible and psychological costs of the advised action. In this regard, a peer educator could reduce perceived barriers through reassurance, correction of misinformation, incentives, and assistance. For example, if a young person does not seek health care in the local clinic because he or she feels that his or her confidentiality is not respected, the peer educator may provide information about a youth-friendly service, thus helping to overcome the barrier to accessing proper health care.

Theories of individual behaviour change have come in for criticism when applied in non-western contexts. Social conditions in these contexts often determine that individuals do not have the personal power or efficacy to carry out decisions that they might have made on an individual level.

An additional problem is that non-consensual sex is prevalent in South Africa. Where rape is common an individual has no power to protect him or herself from HIV infection. Cultural patterns and behaviours may also render these theories inappropriate. In many non-western contexts notions of health are more closely tied to the health of the community rather than that of the individual. A good example would be practices such as virginity testing and cultural circumcision, where the strong traditional heritage overrides rationality.

SOCIAL BEHAVIOUR CHANGE THEORIES

Social behaviour change theories address some of the concerns raised by the inadequacies of the individual models, and place more emphasis on "the interactive relationship of behaviour in its social, cultural, and economic dimension" (UNAIDS, 1999:8). These theories recognise the important role that societal norms play in determining an individual's behaviour and view the individual in the context of a particular community.

These theories argue that to change the behaviour of the individual the behaviour of the entire community needs to change, to ensure group support for individuals who wish to adopt new behaviours.

SOCIAL LEARNING THEORY

Social learning theory, as outlined by the American psychologist Albert Bandura, suggests that people learn through direct experience i.e. watching the actions of others. Bandura's theory also suggests that learning occurs through the observation of role models as well as through training that develops self-efficacy, for example, through practice of responses to simulated situations. The theory takes into account the social nature of individual behaviour change. Viewing the actions of others can allow insight into the consequences of their own actions.

THE SOCIAL NETWORK THEORY

Wolf, Tawfik, and Bond, proponents of the Social Network Theory, suggest that the persuasiveness of one's social network plays a vital role in behaviour modification or change.



IN THE CONTEXT OF PEER EDUCATION

The inclusion of interactive experimental learning activities are extremely important and peer educators can be important role models. The implication for the classroom is to make training as interactive and experiential as possible.

A further implication is that peer educators are visible and their actions and attitudes will be emulated by their peers. This theory implies that choice of peer educators is important and it is also significant to check whether or not selected peer educators are comfortable being viewed as role models.

In other words, individuals are strongly influenced by the behaviour of their peers and seek personal affirmation by following those with significant influence.

The theory suggests that the size, closeness, and demographic composition of a social network will affect the efficacy of peer education. This means that correctly identifying peer educators with leadership qualities will impact the efficacy of a peer education programme.

DIFFUSION OF INNOVATIONS THEORY

The diffusion of innovation theory (Ryan and Gross, [1943] in Rogers, 1983) proposes that gate-keepers and opinion leaders in a particular community may have the power to influence the behaviour of others. This implies that an innovation (the new behaviour) can be filtered down through leaders. Thus, trained peer leaders could influence the actions of their peers. Like the Social Network Theory, this theory emphasizes that influential leaders and respected individuals influence norms by disseminating information through one-to-one contacts and group discussions. Friendship groups and social networks are important routes of communication and change.

IN THE CONTEXT OF PEER EDUCATION

The selected peer educators should be trustworthy and credible opinion leaders within the target group. The opinion leader's role as educator is especially important in outreach work, where the target audience is not reached through formally planned activities but through everyday social contacts.

THEORY OF PARTICIPATORY EDUCATION

Adult educator Paulo Freire proposes that the full participation and empowerment of the people affected by a problem is essential in order to enact change. Hence, peer educators are themselves a part of the target group, and the primary strategy employed is to enable cognitive restructuring to take place, so that people may be empowered to take charge of their lives.

Participatory education concepts are based on Paulo Freire's critique of education in Brazil in the 1970's. Freire suggested that the 'banking' concept of learning, where learners are empty vessels to be filled with knowledge, was both inappropriate and ineffective, particularly with adult learners. He proposed that education be viewed as a participative facilitated process, whereby learners build on their own prior knowledge and experience to develop a conscious and critical view of the world.

The process of Freire's methodology is action-reflection praxis, where participants are encouraged to take a step back from their lives and their practices and examine them objectively in order to develop a critical consciousness of what they see. This reflection offers the perspective and strength to then re-engage in action and continue the cycle.

IN THE CONTEXT OF PEER EDUCATION

The relevance of this theory is obvious. Many advocates of peer education claim that the (horizontal) process of peers talking among themselves and determining a course of action is a key to the success of a peer education project. It also speaks to how important it is for peer education to be delivered in an interactive manner using participatory approaches and techniques.

The theory suggests that this will enable participants to come up with ideas and solutions, which they will then own. This increases the likelihood that cognitive restructuring will take place, and more significantly, behaviour change will take place.

TRANSLATING THEORY INTO PRACTICE

Whether you are providing training of trainers (ToT), training of peer educators, or peer education sessions with the target population, there are some basic methodological considerations for translating the theory into practice.

Most important are learning based on experience and observation (experiential learning) and use of interactive methodologies and drama.

EXPERIENTIAL LEARNING

There is an ancient proverb that says:

**Tell me... I forget, show me... I remember,
involve me... I understand.**

'Involving' the participants in a training workshop in an active way that incorporates their own experience is essential. Such experiential learning gives the trainees an opportunity to begin developing their skills with immediate feedback. It also gives them the opportunity to participate in many of the training exercises and techniques first-hand, before they engage other peer educator trainees in such exercises.

USE OF ROLE PLAY

Peer education uses a range of interactive techniques; brainstorming, small-group discussions, case studies, quizzes, etc. Another commonly used, and highly interactive technique, is role play. Good, believable role play is a technique that can help achieve several major objectives of a health education programme. It can:

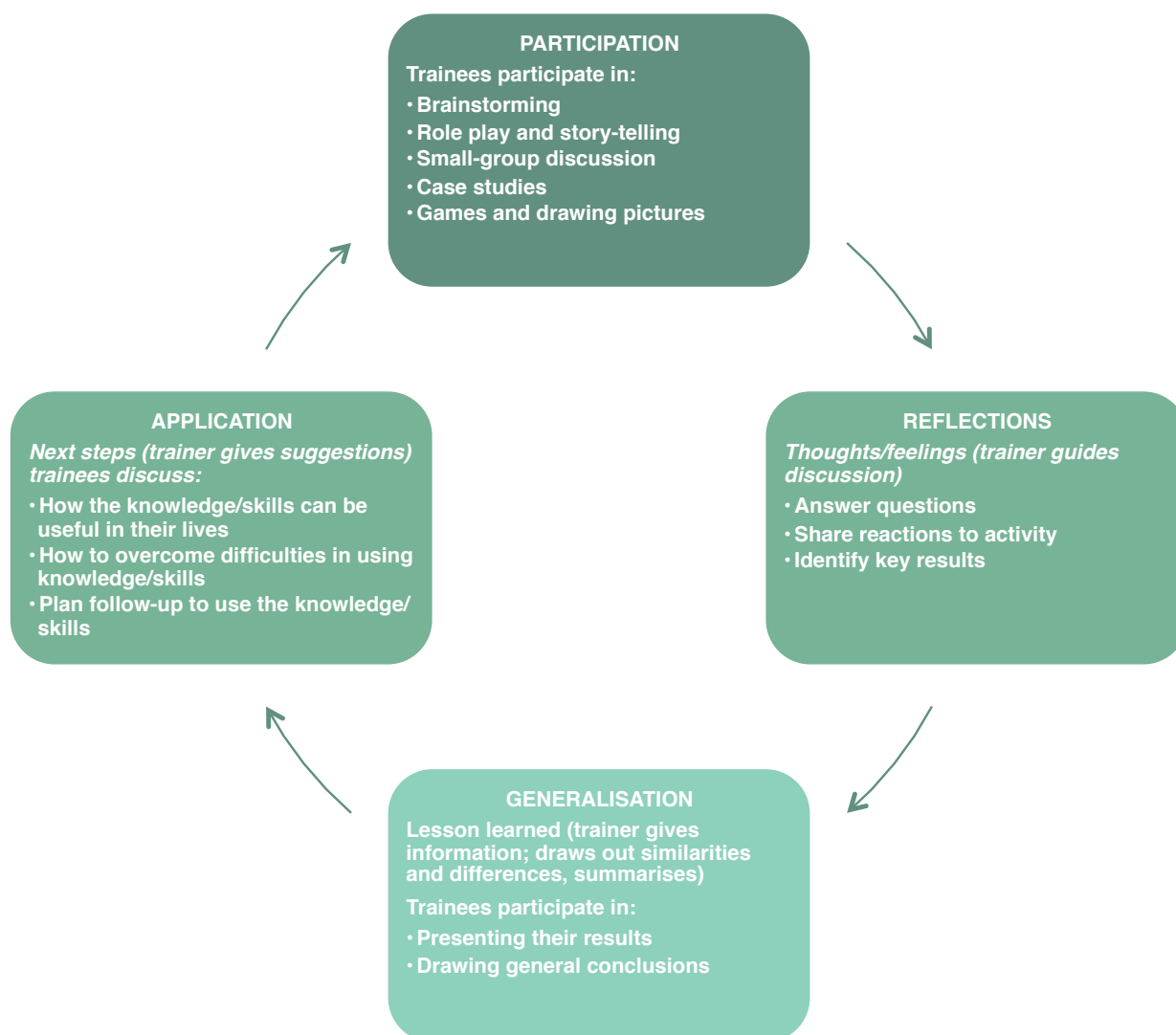
- Provide information; role play is an attractive way to deliver information through humour and real life drama. It permits educators to dramatise the myths that people spread and show how to break them down. In role play, people can explore problems that they might feel uncomfortable about discussing in real life
- Create motivation; role play can effectively dramatise life's external situational pressures (life-stressors) and difficult psychosocial situations, which are sometimes the consequences of poor decision-making and risk behaviour. They can bring to life the realities of, for example, getting an unwanted positive pregnancy test result, testing positive for STIs or HIV, etc. They can demonstrate the difficulties

of having to disclose sensitive and painful information to a loved one or partner. Strong role play engages the hearts and minds of the audience, and can motivate them to change their attitudes on certain issues

- Build skills; when done well, role play has the potential to shape behaviour. It can demonstrate various skills, such as negotiation, refusal, and decision-making, and also practical expertise, for example, how to use a condom correctly
- Make a linkage to resources; role play can provide an opportunity to inform the audience about the services that exist in the community, or to discuss their characteristics, for example, whether they are accessible to young people, whether they respect their right to confidentiality, etc.

For all these reasons, peer education should dedicate sufficient time to using role play and to training the peer educators in good acting skills. The training of trainers proposed in this manual is based upon an experiential learning model, using highly interactive techniques. The model includes four elements; direct experience (an activity in which learners create an experience), reflection on the experience, generalisation (lessons learned) and applying lessons learned. It can be summarised in a diagram as follows:

DIRECT EXPERIENCE: (TRAINER INTRODUCES THE ACTIVITY/EXERCISE AND EXPLAINS HOW TO DO IT)



ADULT LEARNING PRINCIPLES

PRINCIPLES OF ADULT LEARNING

Adult learners are unique; they are people with years of experience, they have established values, beliefs, and opinions, and they have a deep need to be self-directing. This component explains adult learning principles and styles. A comprehensive understanding of adult learning principles is critical to developing successful education programs that result in participant engagement and the facilitation of learning. Adults have special needs and requirements as learners.

1. ADULTS ARE AUTONOMOUS AND SELF-DIRECTED

Adults need to be free to direct their own learning. If the learning engagement is classroom-based the facilitator must actively involve adult participants in the learning process. Specifically, they have to be sure to act as facilitators, guiding participants to their own knowledge rather than supplying them with all of the facts. They should allow the participants to assume responsibility for their learning and engage them in discussions, presentations, and group-based tasks. If the learning engagement is an e-Learning course the course should be designed to allow participants to explore topics in greater detail and choose from multiple learning activities.

2. ADULTS BRING KNOWLEDGE AND EXPERIENCE TO EACH LEARNING ACTIVITY

Over their lives, adults have accumulated a wealth of life experience and knowledge. This may include family memories, work-related experiences, and previous education. Linking new material in a course to learners' existing knowledge and experience creates a powerful and relevant learning experience. Relating theories and concepts to the participants and recognizing the value of experience in learning are two important factors to keep in mind as well.

3. ADULTS NEED LEARNING TO BE RELEVANT AND PRACTICAL

Every day, the human brain takes in hundreds of thousands of sensory inputs. As the brain processes these inputs, it begins to sort out information it deems relevant and important. Relevancy increases the likelihood of information being retained. Adults must see a reason for learning something and the learning must be applicable to their work or other responsibilities in order for it to be valuable for them. Therefore, learning engagements must identify objectives for adult participants before the course begins. By nature, most adults are practical about their learning.

Typically, they will focus on the aspects of a program most useful to them in their work. Participants must know how the content will be useful to them.

4. ADULTS ARE GOAL-ORIENTED

Adults primarily participate in learning programs to achieve a particular goal. Therefore, they appreciate an educational program that is organised and has clearly defined learning objectives. These need to be communicated early in the course.

5. ADULTS ARE PROBLEM-ORIENTED AND WANT TO APPLY WHAT THEY'VE LEARNED

Adult learners want to be able to apply their learning to their work or personal life immediately. Using examples to help them see the connection between classroom theories and practical application, utilising problem-solving activities as part of the learning experience, and creating action plans together with learners are important concepts that enable life application.

6. ADULTS ARE MOTIVATED BY INTRINSIC AND EXTRINSIC FACTORS

Learning is driven by participant motivation. The more motivated someone is to participate in training, the more he or she is likely to learn and retain information. Adults are motivated by both internal and external factors. During the first several weeks on the job, adults are highly motivated to learn. Similarly, motivation is high when they are faced with learning a new work process or approach to a problem. However, as they become more familiar with the content, motivation to learn may wane until a specific need arises.

7. ADULTS ARE PRESSED FOR TIME

In today's fast-paced world, adults have to juggle demanding jobs, family responsibilities, and community commitments. Even if they are highly motivated to learn, the pressures of life often limit the time many adults can invest in learning. Therefore, in many cases, learning must be available when it is convenient for the learner and delivered in "manageable chunks." These may come in the form of modularised e-Learning programs, podcasts, or webcasts or may be strategically delivered through informal training initiatives.

8. ADULTS HAVE DIFFERENT LEARNING STYLES

A learning style refers to how a person learns, categorises, and processes new content. Each person may have multiple preferred learning styles. In training, each of these styles should be considered when delivering content.

SUMMARY

A principle, philosophers say, is the beginning of action. In order to facilitate actions that result in learning among adults, it is critical to have a fundamental knowledge of all of the principles as well as an understanding of how to implement them in the design of learning and training initiatives. Learners use all three modalities to receive and learn new information and experiences. However, according to the VAK or modality theory, one or two of these receiving styles is normally dominant. This dominant style defines the best way for a person to learn new information by filtering what is to be learned. This style may not always be the same for some tasks. The learner may prefer one style of learning for one task, and a combination of others for a different task.

Classically, our learning style is forced upon us through life like this:

- From pre-school to Grade 3 new information is presented to us kinaesthetically
- In Grades 4 to 8 information is visually presented
- From Grade 9 to tertiary and on into the business environment, information is presented to us mostly through auditory means, such as lectures.

According to the VAK theorists, we need to present information using all three styles. This allows all learners the opportunity to become involved, no matter what their preferred style may be.

RECOGNISING AND IMPLEMENTING THE THREE STYLES

AUDITORY LEARNERS

Auditory learners often do better talking, discussing and hearing what was said. To integrate this style into the learning environment:

- Begin new material with a brief explanation of what is coming.
- Conclude with a summary of what has been covered. This is the old adage of “tell them what they are going to learn, teach them, and tell them what they have learned.”
- Use the Socratic method of questioning learners to draw as much information from them as possible and then fill in the gaps with your own expertise.
- Include auditory activities, such as brainstorming, buzz groups, or Jeopardy. Leave plenty of time to debrief activities. This allows them to make connections of what they learned and how it applies to their situation.
- Have the learners verbalise the questions.
- Develop an internal dialogue between yourself and the learners.

VISUAL LEARNERS

Visual learners like to learn through written language, such as reading and writing tasks. They remember what has been written down, even if they do not read it more than once. They like to write down directions and pay better attention to lectures if they watch them. Learners who are visual-spatial usually do better with charts, demonstrations, videos, and other visual materials. They easily visualise faces and places by using their imagination and seldom get lost in new surroundings. To integrate this style into the learning environment:

- Use graphs, charts, illustrations, slides or other visual aids
- Include outlines, concept maps, agendas, handouts, etc. for reading and taking notes
- Include plenty of content in handouts to reread after the learning session
- Leave white space in handouts for note-taking
- Invite questions to help them stay alert in auditory environments
- Post flip charts to show what will come and what has been presented
- Emphasise key points to cue when to take notes
- Eliminate potential distractions
- Supplement textual information with illustrations whenever possible

KINAESTHETIC LEARNERS

Kinaesthetic learners do best while touching and moving. It has two sub-channels: kinaesthetic (movement) and tactile (touch). These learners tend to lose concentration if there is little or no external stimulation or movement. When listening to lectures they may want to take notes for the sake of moving their hands. When reading, they like to scan the material first, and then focus in on the details (get the big picture first). They typically use coloured high-lighters and take notes by drawing pictures, diagrams, or doodling. To integrate this style into the learning environment use activities that get the learners up and moving:

- Use role play and drama
- Give tasks
- Play music, when appropriate, during activities
- Use coloured markers to emphasize key points on flip charts or white boards.
- Give frequent stretch breaks (brain breaks).
- Guide learners through a visualisation of complex tasks.

THE ENERGY CYCLE

UNDERSTANDING YOUR PERSONAL ENERGY CYCLE

An important piece of productivity concerns what time of day you select to work on which activities. Everyone has a natural time during the day when they are “UP” (prime time) and a natural time when they are “DOWN” (down time). During prime time your brain is “on”; your batteries are charged and you’re able to focus. During down time your brain feels “slow”; it’s difficult to muddle through your work.

First, let’s chart your personal energy levels. Get out a piece of paper and draw a big “L” for a graph with a vertical and a horizontal axis. Mark the vertical axis “0” to “100” to represent your energy level as a percentage. Mark the horizontal axis with your work hours, in one hour increments. When you arrive at training in the morning, draw a dot where energy intersects with time, to indicate how you feel in terms of your energy level.

As the day progresses, draw a series of dots horizontally to show how your energy ebbs and flows. Then connect the dots and analyse your line chart.

Draw a horizontal dotted line at about the 75% energy level mark across your line drawing to represent your peak productivity zone. Write these exact time ranges out to the side of your graph. These are your “expensive” hours, compared to other times

during the day, because your brain is capable of doing higher-level activities in that range.

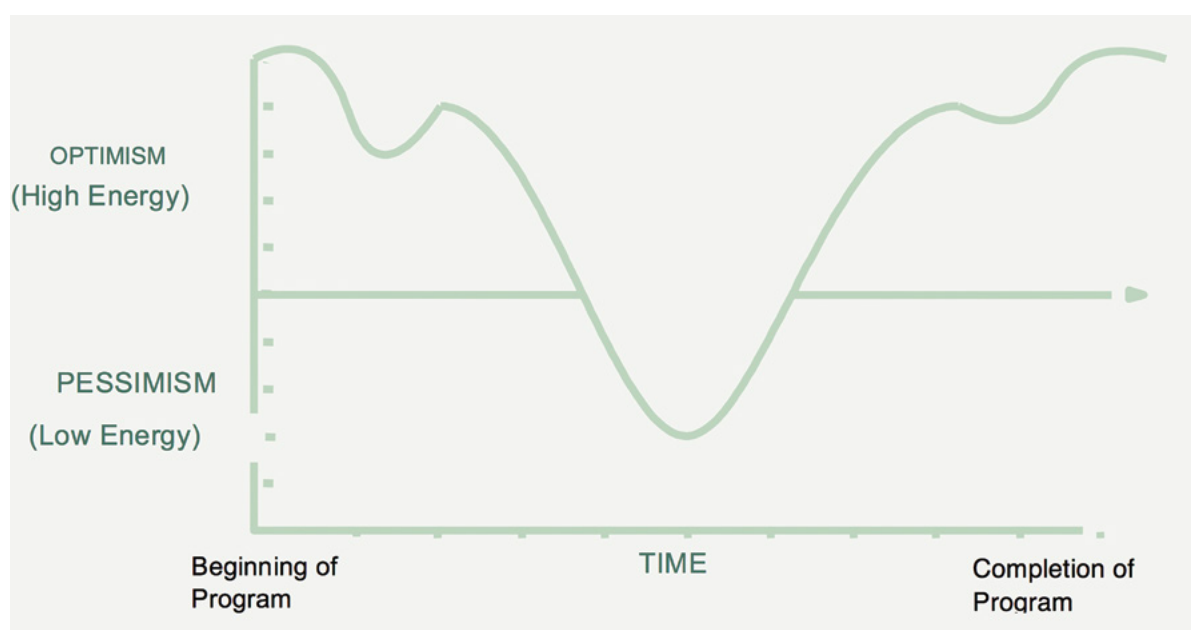
It’s important to know when you’re in prime time, because you can get so much more done.

The key is to focus on:

1. Important tasks
2. Critical decisions
3. Problem solving
4. Complex thought

Things to consider when using energisers

- Use energisers frequently during a training session, whenever people look sleepy or tired, or to create a natural break between activities.
- Try to choose games that are appropriate for the local context. Think carefully, for example, about games that involve touch, particularly of different body parts.
- Try to select games in which everybody can participate, and be sensitive to the needs and circumstances of the group. For example, some games may exclude people with disabilities.
- Try to ensure the safety of the group, particularly with games that involve running. For example, make sure that there is enough space and that the floor is clear.
- Try not to use only competitive games, but also include ones that encourage team building.
- Try to avoid energisers that are too long. Keep them short and move on to the next planned activity when everyone has had a chance to move about and wake up.



FACILITATION

Facilitation is a way of working with people. It enables and empowers people to learn skills, think critically and creatively, and ultimately realise their own potential. In the role of facilitator, one does not perform the task or provide solutions or teach. He/she uses a particular set of unique skills in a process which allows the individuals or group to reach their decision, set their goal, or learn a skill.

Facilitation is a developmental educational method which encourages people to share ideas, resources, opinions, and to think critically in order to identify needs and find effective ways of satisfying those needs. Through facilitation, group members come to value and develop their own expertise and skills. Facilitation involves many facets of interaction between individuals, such as clarification, conflict management, and planning. It can be learned and developed through practice and supervision.

An openness to constant learning and development is necessary for anyone seeking to improve his/her facilitation skills. A facilitator helps people to decide what they want to accomplish, reminds them of their responsibility in achieving it, and encourages and helps them to complete an agreed task or activity. The facilitator ensures that the needs of individuals within the group are recognised, acknowledged, and responded to; this is seen as an integral part of the task at hand and not superfluous to it.

In some settings the facilitator plays an objective role by asking questions, encouraging responses, and enabling group members to discuss, respond and reach a conclusion. In other situations, they may be stimulating group members to create solutions to problems they have identified, by offering suggestions or creating simulations which the group can practise. In facilitation there is an equal emphasis on achieving the task and on the process involved in that achievement.

Facilitators use a wide range of styles with groups, depending on the task, activity, people involved, time available and needs of the group members. The various styles enable particular matters to be addressed most effectively, and facilitators should be able to modify their style to meet the group's needs.

For the task at hand, a facilitator's approach/role may be one or a combination of the following:

Directive:

Giving people information, instructing them how to do something, for example: 'This is how to develop a work plan.'

Exploratory:

Asking questions, encouraging people to voice their experience and ideas, for example: 'What did you find useful in the last peer education session you were part of?'

Delegating:

Assigning tasks, roles, or functions to individuals. For example, in planning a facilitation training session with a group, some organisational tasks may need to be shared.

Participative:

Taking part in discussion, sharing personal experiences and encouraging others to do likewise, for example: 'The first time I ever did a skills- sharing workshop like this was . . .'

For the process, a facilitator's approach/role may be one or a combination of the following:

Interpretive:

Assisting participants to verbalise a contribution or helping someone to find the words to express what they mean.

Cathartic:

Encouraging and modelling the expressions of feelings and emotions as they emerge by asking a question such as: 'And was that a very painful time?'

Evaluative:

Assessing what someone says, providing a statement of value in relation to behaviour, for example: 'That seems to have worked well for you.'

Sharing:

Encouraging the sharing of past and present feelings and those about future events, with a question such as: 'Does anyone else feel this way?'

Directive:

Guiding members as they explore their feelings and begin to express them, for example: 'Let's take a few moments to gather our thoughts and think about how this event has affected the group.'

FACILITATION TECHNIQUES

THE FACILITATOR GUIDING A DISCUSSION

Facilitators have the unique role of needing to portray leadership, whilst allowing participants to take charge and stamp their ownership of the process. In other words, demonstrating the paradox of leading by following. So in guiding discussions, one needs to resist the impulse to provide all the answers and inputs. Instead, facilitators are required to lead the discussion, by non-directive positioning and the asking of strategic questions designed to foster critical thinking. Thus providing a catalyst for cognitive restructuring.

ASKING PROBING QUESTIONS

Questioning is a technique that facilitators must use with caution. One needs to be careful not to come across as interrogating. Questions must be subtle and thought-provoking, and hold the inherent potential to stimulate debate, discussion, and personal growth.

CREATING A LEARNER-CENTERED ETHOS

Facilitators need to be committed to making the course learner-centred. This provides an opportunity to model skills that enable participants to own the process and take charge of discussions, whilst still maintaining control without being controlling or authoritative. This would demonstrate to peer educators' appropriate techniques to patiently listen and respond to the concerns of their peers, whilst resisting the impulse to tell them what to do or not do.

Techniques to foster such a context include:

ENCOURAGING INTERACTION

- During training, interact at least once with every participant individually
- Motivate, manage, and encourage participants to interact with you by being approachable and attentive, and reaching out particularly to those that appear disengaged, diffident, or introverted. This will help them overcome shyness, gain confidence, feel valued and accepted, and increase the likelihood of them engaging and becoming invested in the goals and objectives of the course
- Make an effort to learn participants' names early in the course, and use their names whenever it is appropriate. Use names when you invite participants to speak or answer questions, or when you refer to their comments, or thank them. This will make them feel valued and respected
- Be available, approachable, and accessible to participants during training. Remain in the room, and encourage them to come to you with their concerns or if they need clarity about anything

REINFORCING PARTICIPANTS' EFFORTS

- Take care not to seem authoritative or threatening
- Be careful not to use facial expressions or comments that could make participants feel ridiculed, embarrassed, or humiliated
- Be patient and accommodating when asking or answering questions
- Be attentive and show interest in what participants say.

For example, focus by making non-threatening eye contact, use minimal encouragers, ask for clarity or more information, affirm their contribution

- Praise or thank participants who make an effort, including, for example, when they try hard, ask for an explanation or clarity regarding an unclear/confusing point, participate in a group discussion, or provide assistance and support to fellow participants
- Model appropriate communication skills (open questions, tuning in to non-verbal cues, showing empathy, and using good listening skills)

YOUR ROLE AS A TRAINER

DELIVERING THE TRAINING

A useful way to begin preparing is to review the material. Such a review might consist of a walkthrough of each lesson including; preparation, gathering and organising the material, tools, and equipment (assessment tools, handouts, slides, flipchart and pens, and projector) needed for the workshop, as well as review of all relevant policies, protocols and guidelines. This will provide you with a clear mental image of how each lesson's methods and materials support the specified objectives. Having such an image firmly in mind can save on preparation time. Trainers are advised to adhere to the prescribed methodology to ensure attainment of training goals, objectives, and envisaged outcomes. As a trainer you are required to facilitate not teach. You are required to:

- Control the process in a non-directive way
- Make learning happen
- Set the stage and provide the props to create the conditions for "ah ahhh" moments to occur
- Make participants feel "in charge" of the process
- Provide the triggers that allow participants to make the necessary connections
- Act as a catalyst for participants to grow in confidence
- Allow participants to take ownership of ideas and 'solutions'

LIMITATIONS OF FACILITATION

Facilitation has its limitations. Facilitated groups are not therapy groups. Personal development groups may be led by facilitators who are also trained in the use of other, more specific skills such as organisational development or community development. Gaining consensus on the purpose of each group is important, so that appropriate and relevant boundaries can be developed and maintained.

GROUP MANAGEMENT

KNOW-IT-ALLS, TALKERS, AND BULLIES: MANAGING HECKLERS

It is sometimes said that “there is one in every group.” A difficult person makes the trainer’s job a challenge at best. Learning how to manage those participants who don’t want to be in training is an essential skill of trainers. This section explores handling difficult people and how to turn them into active participants in the learning process.

Groups are dynamic. It is fascinating the way in which the individuals in the group interact and form a pattern of interaction.

A group can sometimes take on a singular identity. The study of these interaction patterns is called “group dynamics”

An examination of both content and process within a group can speak volumes about the work, impact, and effectiveness of a facilitator. An examination of content would, for example, be the teaching of the peer educator content and skills. An examination of process is done by exploring what is happening in the group both for the facilitator and individual members. In other words, is there conflict, or dominance, or bullying, or clique formation?

From the facilitator’s point of view, by exploring the work of groups at these levels, he or she acquires a more complete picture of the group and its dynamic. An understanding of this two-tier analysis can assist the facilitator when they are planning sessions and designing exercises and effective intervention.

TO BE EFFECTIVE, A FACILITATOR MUST:

- Recognise both individual and group needs
- Be able to identify and recognise the needs, roles and resistance of group members
- Distinguish between what is happening because of the facilitator and what is happening because of the group’s make-up
- Have a clearer view of how certain conflicts or difficulties may be resolved
- Be aware that self-awareness is critical
- Be clear about where the facilitator’s involvement and commitment to a group begins and ends. In this way, the facilitator takes care of her/himself
- Be mindful that it is important not to fulfil unreasonable expectations which the group may have
- Challenge and work with negative projections within the group

COMMUNICATING WITH THE GROUP

When interacting with the group, the facilitator must remember that a two-way process of transmitting and receiving information is in operation. Both sender and receiver interpret messages in different ways, depending on a number of factors such as class, ethnic background, gender, sexual orientation, and age. A facilitator’s interaction with group members is both verbal and non-verbal. The verbal interactions are stated openly using speech. The non-verbal are not stated in speech, but are transmitted in other ways, such as in body language. Both have a powerful effect on the group, and a facilitator needs to be aware of the impact he or she can have as a result of the position they hold within a group.

VERBAL INTERACTION EXAMPLES

- Comments to people: ‘You’re very brave to say that.’ (I wouldn’t have said that)
- Expressions of values and attitudes: ‘I really like strong women.’ (If you’re not strong, I don’t like you)
- Volume and tone of voice (loud-aggressive, intimidating; soft-passive, weak, lacking in confidence)
- Revealing political agenda by excluding or including certain beliefs: ‘The unemployed have only themselves to blame’; ‘I believe that while it is hard, we need to work towards including and accepting homosexuals’ (discriminatory)

NON-VERBAL INTERACTION EXAMPLES

- Facial expressions and posture.
- Assumptions based on gender
- Assumption of a person’s social class
- Perception of a person’s racial or ethnic background
- How and when a person chooses to sit, move, or hold themselves

How the facilitator interacts with group members strongly influences the group process. It is not only what the facilitator says, but how he or she behaves that is important. People take in messages (verbal and non-verbal) from each other all the time. The non-verbal can either enhance or contradict a point being communicated. Saying you are not angry, while clearly giving off the body language of a stiff back and hurt expression are contradictory signals.

Remember: You have got to be on the balcony and in the dance

One needs to be constantly self-aware and reflecting on one’s own feelings, emotions, and attitude. Facilitators are human beings that have been socialised within a particular context. It can be difficult to eliminate ideas, assumptions, beliefs of, and perhaps prejudices against, other people. However, observing responses, behaviours and attitudes from group members in

relation to oneself is a valuable barometer to gauge one's own attitude, behaviour, and responses.

It is the facilitator's job to recognise how these assumptions operate, negatively and positively, within the group. It is also the facilitator's role to work at challenging and diluting the negative impacts of any prejudice within a group. The facilitator's ability and readiness to challenge negative effects of social prejudice is an important indicator to group members, that this behaviour is not to be tolerated. The facilitator can encourage the group members to create a group contract which excludes prejudices, stereotyping, or discrimination by agreeing that these are all to be challenged. This creates a safe space outside of which members agree to leave their negative beliefs and values.

Overcoming the fear of speaking before a group:

- Know your material well
- Practice your material
- Use participatory techniques
- Learn participants' names and use them
- Establish your credibility early
- Use eye contact to establish rapport
- Exhibit your advance preparation (via handouts, etc.)
- Anticipate potential problems and prepare reasonable responses
- Check the facilities and equipment in advance
- Obtain information about the group in advance
- Convince yourself to relax (breathe deeply; meditate; talk to yourself)
- Prepare an outline and follow it
- Manage your appearance (dress comfortably and appropriately)
- Rest so that you are physically and psychologically alert
- Use your own style (don't imitate someone else)
- Use your own words
- Put yourself in your trainees' shoes (they're asking, "What's in it for me")
- Assume they're on your side (not antagonistic or hostile)
- Provide an overview of the workshop
- Accept some fears as being good (energising stress vs. destructive)
- Identify your fears, categorise them as controllable or uncontrollable, and confront them
- Give special emphasis to the first five minutes (super preparation)
- Image yourself as a good speaker (self-fulfilling prophecy)
- Practice responses to tough questions or situations
- Create an informal setting

CO-FACILITATION: WORKING WITH A PARTNER

The technique of working with a partner can be rewarding for a group and its facilitators, but it is a practice which requires planning and periodic evaluation with the co-worker, if it is to yield results. Co-facilitation has advantages and disadvantages. Not everyone wants to or is ready to co-facilitate. If a facilitator chooses to co-facilitate, it is crucial to plan how the two will share

the work, how they can best support each other, and that they learn to communicate effectively. It must never be forgotten that the focus of co-facilitation remains the needs of the group. Co-facilitation may be used as a training mechanism for one of the co-facilitating pair. Whilst this works well, care must be taken that the group does not lose out by time or energy being directed towards the co-facilitators.

ADVANTAGES OF CO-FACILITATION:

- Eases the pressure of full responsibility
- Allows for joint planning, evaluation, and feedback
- Brings different experiences and attributes to the group
- Means a greater sharing of skills, resources, and energy
- Enables less experienced facilitators to develop skills

DISADVANTAGES OF CO-FACILITATION:

- Joint planning, evaluation, and feedback is time-consuming
- Co-facilitators can be 'played off against each other'
- One co-facilitator may get on better with the group
- Feelings of insecurity may arise for facilitators
- One co-facilitator may dominate
- Rivalry between co-facilitators may develop
- Vague definitions and unclear delineation of responsibilities may cause problems between co-facilitators

Remember:

- Choose to work with someone whose values are similar to your own
- Plan work together
- Decide how sections of the session will be shared
- Discuss how the session will be divided between two
- Decide who will take which section
- Consider how to behave when the other person is facilitating
- Decide whether to leave the room, participate, or remain silent
- Plan to let the group know the session/programme will be jointly facilitated
- Decide what to do when one person wants to interrupt the other
- Allow some flexibility so that non-threatening, non-challenging contributions may be welcomed by the other partner (for instance, if one facilitator has forgotten something, or if one facilitator has a different experience that could be usefully shared)
- Support each other during the session— offer appropriate feedback and provide a second voice to the facilitator if you detect resistance or dissent from the group
- Agree never to side with the group against your co-facilitator
- After the session, evaluate both performances jointly
- Give positive and critical feedback on how the two facilitators might improve their work together
- Plan the next session on the basis of the previous one

FACILITATOR COMPETENCY MODEL

Desired Competency	Demonstrated Proficiency (in other area of life/work or as a facilitator if currently a trainer)	Application as a Facilitator
Technically Current/Capable Open to Learning	- Engaged in life-long learning - Picks up on the need to change personal and interpersonal behaviour quickly - Seeks feedback - Picks up on verbal and non-verbal cues regarding their impact on others, and adjusts	- Shares personal successes and failures to enhance participant learning - Applies experience as a Peer Mentor to facilitate excellent learning and experience for participants - Remains current in and shows ability to apply knowledge to the learning experience
Capable of Creating an Environment for Learning	- Creates a positive and safe atmosphere for participants to experiment to enhance personal learning - Expresses thoughts, feelings, and ideas in a clear, concise, and compelling manner - Adjusts style to engage participants - Assesses learning needs and adjusts as necessary Additional for Lead Facilitators: - Manages the front of the room – has an engaging presentation style and platform skills - Promotes a successful experience in training and facilitating experiential adult learning - Promotes a successful experience in leading other trainers	- Demonstrates a style that values different learning styles (visual, experiential, auditory) as well as backgrounds and experience (industry vs. academia) - Brings personal background and experience as a Peer Mentor, onsite experiences, etc., that contribute to participant understanding - Makes appropriate judgements that demonstrate competence in delivering experiential learning Additional for Lead Facilitators: - Demonstrates an ability to lead Support Facilitators - Treats facilitators as valued members of the training team - Intervenes as necessary to ensure facilitators are fully contributing members of the training team, enhancing the learning of participants
Interpersonally Skilled	- Builds trusts - Listens and places input into context - Maintains an open mind and avoids personal bias - Finds common ground and gets cooperation easily - Diffuses high tension situations - Displays cultural sensitivity - Communicates well	- Provides constructive feedback to assist in their learning - Deals tactfully with participants who don't complete training requirements - Makes an objective evaluation of a participant's potential Peer Educator - Effectively communicates concerns to individuals and/or appropriate parties

PRINCIPLES AND VALUES OF FACILITATION

Bearing in mind that the TOT facilitation stage is an opportunity for facilitators to model all desired facilitation behaviours, it is imperative to demonstrate verbally and non-verbally, their commitment to the following principles:

- **Listening:** Facilitation entails listening to what people are saying and tuning in to what they are not saying. This includes being aware of verbal and non-verbal means of communication.
- **Confidentiality:** In order to participate fully, participants must be confident that everything of relevance can be discussed freely without inappropriate disclosure by the facilitator/s outside the group. Groups normally set the parameters of confidentiality at the outset.
- **Respect:** A facilitator must acknowledge and respect every participant. It is crucial to be assertive when required to prevent dominant and/or aggressive participants from undermining the basic respect that should be accorded to each individual in the group.
- **Equality:** Every participant has an equal right to contribute, influence, and determine the direction of the group. Facilitators have the responsibility to encourage participation and gatekeep dominant participants with this in mind.
- **The value of personal experience:** Participation and sharing of personal experience should be affirmed. Every participant's contribution to a discussion or skill-sharing activity from this perspective is equally valid and valuable.
- **Agreed goals:** Participants must be encouraged to agree on a common vision and goal for the programme to mark their ownership of the process. Facilitators need to manage group consensus to this end.
- **Group process:** Facilitation requires giving attention to how the group operates. This includes attempting to resolve conflict or challenges that might arise in the group relating to individual members that may impact the group dynamic.
- **Trust and safety:** Participants should not fear ostracisation, discrimination, or judgement related to their personal views or lifestyle. Facilitators must model qualities such as warmth, empathy and genuineness, a non-judgemental attitude, and the skill of neutrality. They are therefore compelled to act appropriately should trust and safety of participants be compromised in any way.
- **Inclusion and encouragement:** Everyone in the group must feel included and equally valued. All members must be encouraged to participate, share ideas, suggestions and solutions, and take initiative.
- **The importance of a positive/beneficial experience:** Facilitators must recognise that everyone is entitled to positive experience in the group. To this end realistic and valid individual needs of participants should not be compromised by unrealistic or contradictory expectations of the group or the facilitator.

- **Participation:** Consultation with group members on direction, pace, content and method, with an openness to change, is vital. Facilitation succeeds when there is a consistent consultative ethos, combined with a genuine belief in the value of encouraging trainees to own the process through participation, interaction, and feedback.

APPROACHES AND METHODOLOGIES TO TEACHING AND LEARNING

Skills-building is an essential part of peer education training. There are various tried and tested approaches and methodologies that are recommended to achieve this.

Hands-on practice is the key to mastering application which is the focus of the training. The emphasis is on transference of skills to empower participants to educate, screen, refer, report, and record. The competency-based participatory training approach used in the Facilitator's Guide reflects key principles of behaviour change communication (BCC) with a focus on the promotion of small doable actions, and recognition of the widely acknowledged theory that adults learn best by reflecting on their own personal experiences. The approach uses the experiential learning cycle method and prepares participants for immediate application of skills.

The course employs a variety of training methods, including the use of visual aids, demonstrations, group discussion, case studies, role plays, and practice. Role playing and simulations are crucial, high impact learning tools. Delivering good, believable role play techniques requires some training. (See Resource Sheet 1.2 for information on suggested training approaches and methodologies.)

IMPORTANT TEACHING AND LEARNING PRINCIPLES

IMMEDIACY

Enable participants to identify how he/she can use his/her knowledge, skills, and attitudes in the real and immediate context.

EXPERIENCE

Kinaesthetic learning (learning by doing) has been known to be the most effective way to learn a skill. Hence, facilitators need to create opportunities to engage participants in appropriate activity, and then enable them to make the necessary connections in respect of application.

APPROACHES AND METHODOLOGIES FOR TRAINING ADULTS



APPROACHES AND METHODOLOGIES FOR TRAINING ADULTS



RECRUITMENT OF PEER EDUCATORS

Steps to follow:

STEP 1: IDENTIFY SOURCES AND CHANNELS FOR RECRUITING PEER EDUCATORS

Where will you get them from? How will you find them?

It is important to develop a plan to identify recruitment sources (staff recommendations, student leaders, peer nominations) and channels (word of mouth, announcements in lectures, social media, etc.).

- Observe the target group to find the natural leaders
- Conduct awareness and education programmes and identify those who demonstrate interest
- Call for volunteers to assist at FTF activations and observe candidates who show passion and promise
- Talk to the members of the target group about who they feel most comfortable talking to about HIV/STIs

STEP 2: ADVERTISE THE SCOPE OF ACTIVITIES PEER EDUCATORS WOULD BE REQUIRED TO DO AS FOLLOWS:

- Conduct informal, small-group discussions about health and wellness including sexual and reproductive health
- Organise and conduct formal group discussions with peers about lifestyle and behaviour change
- Hold regular meetings
- Distribute educational materials
- Display posters and other educational materials
- Present video/DVD screenings
- Design/develop educational materials
- Perform in dramas
- Participate in special events e.g. World AIDS Day, sporting carnivals
- Participate in media coverage of peer education work
- Be engaged in activities to motivate and support behaviour change and build skills
 - Talk to peers one-to-one
 - Teach peers how to do a personal risk assessment
 - Teach peers how to negotiate safe sex (including condom use)
 - Recommend or refer peers for HIV and STI testing
 - Provide referrals to specialist services for clients requiring more substantial information or support

RECRUITMENT STRATEGIES:

- Build in incentives
- Make the advertisements easy to understand
- Allow sufficient time for interested people to consider the advertisement and apply

- Provide contact details such as a phone number so that interested people can obtain further information if they need it

STEP 1: INVITE INTERESTED INDIVIDUALS TO APPLY

Design a simple form, for completion by interested applicants. The effort to obtain, complete, and submit the form already indicates enthusiasm and commitment. The form should ask why applicants are interested, what characteristics and qualities they possess that might make them effective peer educators; year of study, availability for training, availability to execute peer educator duties (how many hours per week), desire to commit, period available to be appointed, etc.

STEP 2: COMPILE A SHORTLIST

Based on responses, compile a shortlist of suitable candidates. Selecting and keeping peer educators are two factors that are crucial to the success of a peer education project. One study of peer education projects found that a key challenge was selecting peer educators who are acceptable to both project staff and to community members (Horizons 2000, p. 16). In this context the implication would be finding students that are popular amongst the staff and student community. The study also highlighted key characteristics to look for when selecting peer educators as follows:

- confidence
- technical competency
- communication skills
- compassion

SELECTION OF PEER EDUCATORS

STEP 1: DECIDE ON CRITERIA FOR PEER EDUCATOR SELECTION

Programme managers must first construct a consensus driven criteria list for selecting peer educators. In a post-school context, criteria must include availability, gender, desire and motivation, popularity, reputation amongst staff and students, acceptability by target audience, previous experience, personal traits (behaviour, team player, volunteer spirit, potential for leadership, etc.), and other characteristics deemed relevant for a particular programme. Selection must consider the diversity of the target group and find peer educators that are representative of the range. A project team should not expect members of the target group to share all the same characteristics, attitudes, beliefs, values, and practices. In composing the selection criteria, you could choose from the elements listed below:

Successful candidates must:

- be members of the intended target group
- be understanding of and relate to the target group
- be acceptable to and respected by the 'host community', in this instance the institution
- be motivated to work on reducing the impact of HIV and STIs
- be non-judgemental and compassionate
- be able to communicate effectively, and educate and persuade others
- be honest and able to maintain confidentiality
- be a strong role model for the behaviour they seek to promote with others, and have self-confidence and leadership qualities
- have strong interpersonal communication skills, be able to engage with their peers and establish rapport, and be able to listen and reflect
- be comfortable talking about sexual, personal and taboo topics, or be able to develop that quality during training
- be able to work in a team
- possess, or be able to develop, public speaking skills
- be able to control and facilitate a group
- be reliable and able to commit to the work programme which may include after-hours work

STEP 2: IDENTIFY THE NUMBER OF PEER EDUCATORS THAT THE PROJECT NEEDS

Select the required number from the candidates shortlisted during recruitment.

STEP 3: COMPOSE AN INTERVIEW PANEL

Set up a panel consisting of programme managers and members of the target group. Selection of peer educators needs to be an inclusive process. When arranging the selection process, it is important to remember the principles of involving both stakeholders and members of the target group in key stages of the project's activities – which include the stage of selecting peer educators. The following steps are necessary to arrange the selection process. Ensure gender balance on the panel.

STEP 4: MEET BEFOREHAND TO PREPARE FOR THE INTERVIEWS

Arrange selection criteria, compose questions, and prepare sheets for recording comments and scores on each criterion for each interviewee.

STEP 5: ORGANISE LOGISTICS

- Arrange a room for interviews
- Contact short-listed applicants and schedule interview times, allowing enough time for each interview
- Inform candidates if they need to prepare a short talk or presentation

STEP 6: CONDUCT INTERVIEWS

Ensure that the process is informal and not intimidating.

STEP 7: SELECT CANDIDATES

The basis of the selection criteria for peer educators is an analysis of the target group, and the skills and personal features needed to create a relationship to work with them. Seek a balanced team of peer educators that represents the important criteria across the group. Individual educators do not have to meet all criteria.

Select some charismatic facilitators, some organised and low-key observers, and some sensitive communicators.

STEP 8: INFORM SELECTED CANDIDATES

Contact and inform selected candidates.

STEP 9: CONVENE A MEETING OF SELECTED CANDIDATES

Convene a meeting of selected candidates at the earliest available opportunity.

STEP 10: SCHEDULE TRAINING OF SELECTED CANDIDATES

Set a date for training as soon as possible.

MONITORING AND EVALUATION

1. ESTABLISH RELEVANT, CLEAR OBJECTIVES

Clearly defined programme objectives are measurable, time bound, and achievable. For example; “To increase the numbers of students accessing HIV counselling and testing services to x by September 2016.”

Tips:

- Objectives used for M & E must be identical to, or fashioned from, the overall mission and goals developed at the onset of the programme
- Although programmes should have high expectations, it is practical to have intermediate objectives so that progress can be demonstrated at earlier stages

2. ESTABLISH FUNCTIONAL, RELEVANT INDICATORS

Indicators are established that reflect sex, age, religion, and ethnicity, and that allow tracking and measurement of target group performance and success of programme activities (such as drop-outs, number of stakeholder meetings, number of young people reached, number of activities, etc.).

Tips:

- Consider getting expert assistance with the task of establishing indicators; the easiest or most obvious ones are not always the most useful for programme assessment.
- Select only those indicators that reflect what the programme intends to change, and believes it can change.
- Be aware that some indicators are culturally sensitive. In some places, asking about religion and ethnicity will drive people away.

3. INCLUDE M & E IN THE WORKPLAN FROM THE START

An M & E plan, with an allocated budget, must be included in the workplan from the programme's start. It captures all aspects of the programme, including recruitment, training, peer education activities, supervision, peer educator performance, youth involvement, gender equity, and collaboration.

Tips:

- Look at each aspect of the programme from the start, with priorities indicated.
- The use of experts to advise on or conduct M & E is critical. Be sure to foster capacity building among the programme staff and the peer educators when bringing in outside experts.
- Committing to an M & E plan from the beginning not only ensures the collection of baseline measurements, but

also can help clarify priorities and the means with which to measure success.

4. IMPLEMENT BASELINE ASSESSMENT

A baseline assessment, against which to measure the achievement of objectives, should be implemented. This information is useful for tracking and monitoring programme activities.

Tips:

- With expert assistance, design a reasonable baseline assessment derived from established indicators, that you intend to later measure with a follow-up survey.
- While there are other ways of measuring programme impact, pre and post-intervention measurements of behaviour and health outcomes are most dramatic and meaningful to stakeholders. However, measurements can be expensive and take a long time, especially if measuring health outcomes. Assessments of knowledge are easy to obtain, but knowledge gains are not predictive of behaviour change.

5. DEVELOP MONITORING TOOLS AND A MEASURING SYSTEM

Tested and usable monitoring tools (questionnaires, diaries, tracking forms, etc.) must be developed as part of the M & E system, for monitoring and for measurement of performance and progress. Staff and peer educators are trained to use them.

Tips:

- Be sure to develop qualitative and quantitative tools.
- Documenting the education process and peer educator insights can be as valuable as quantitative changes; this qualitative information sheds light on the reasons and conditions under which interventions occur.
- If possible, identify tools that have already been developed, and can be adapted by your programme.
- Peer educators may need to monitor many of their activities themselves. They can easily be trained to observe accurately and precisely, but there are validity issues that need to be addressed.

6. ENSURE CAPACITY TO PLAN AND IMPLEMENT M & E

Programme staff need to have the capacity to plan and implement M & E, or appropriate external support and assistance are identified.

Tips:

- Expertise in evaluation methodology and design are essential to successfully implemented M&E.
- Expertise is sometimes available for free, such as from the university/institution itself, especially where there may be academic interest in the programme.

7. ESTABLISH MEANS FOR PEER PARTICIPATION IN PLANNING AND IMPLEMENTING M & E

Peer educators must be involved in planning M & E functions, including instrument development and testing, and in implementing M & E, including planning, monitoring, and evaluation tasks.

Tips:

- Peer educators can offer relevance to the M&E design and operation.
- Peer educators must be trained to be effective partners in M & E activities.

8. USE M & E FOR DECISION-MAKING

Findings from timely reporting on programme activities, as well as from any evaluation that takes place, should be used to make adjustments in programme operations, and to plan future activities.

Tips:

- Do not wait until a final report has been completed before you use your M & E data; plan to get findings quickly, and address challenges as they occur.
- Look for unexpected implications of the M & E data, including evidence that was not consciously sought.
- Be aware that monitoring data are not always rigorous or reliable. Thus, data should inform decisions to the extent that they are credible; when data are merely suggestive, a more focused assessment may be called for.

MANAGEMENT AND OVERSIGHT

1. ENSURE COMPLIANCE WITH PROGRAMME STANDARDS

Management systematically monitors and assures compliance with standards, and initiates corrective action when shortfalls are identified.

Tips:

- While working towards overall compliance to standards, managers/peer mentors also need to set priorities, use flexibility, and address realities, making sure that standards are feasible in the context of current conditions.
- When a standard proves to be difficult to meet, management, in cooperation with peer educators, should determine whether to adjust activities to meet the standard, or adjust the standard to address practical realities.
- Programme leaders are responsible for ensuring broad oversight and guarding against the tendency for daily activities and a hectic pace to distract from attention to the

standards of practice that make programmes effective.

- At the same time, all standards are not equally vital, and they are always contextual. Sometimes there are valid reasons for a programme to fail to meet a particular standard.

2. ENSURE TECHNICAL COMPETENCY OF THE MANAGEMENT TEAM

Management, particularly peer mentors, must demonstrate technical competencies in specific areas appropriate to their responsibilities, including peer education strategies and methodologies, behaviour change communication (BCC) activities, and M & E.

Tips:

- Peer mentors require skills across a spectrum of disciplines, therefore it is important for them to be trained.
- In a resource-constrained environment, programmes should view management as a team, and ensure that all needed skills are present overall, if not with each team member.
- Management demonstrates its competencies by constantly expanding its expertise; i.e. keeping up to speed with all new policies, protocols, and developments.
- Good management recognises the need to devote time and resources to new knowledge, theory, and experience from outside (as well as within) the programme, and ensures participation in conferences and webcasts, subscribes to journals, and makes time to read and disseminate key information to staff.

3. ESTABLISH AND MAINTAIN QUALITY EXPECTATIONS OF PROGRAMME ACTIVITIES

Management and peer educators establish guidelines and expectations about the quality of various peer education activities, for example, conducting small group discussions, organizing events, or making useful referrals, and develop remedies for situations when quality standards are not being met.

Tips:

- If quality assurance measures are not already in place, a team of managers and peer educators can work together to define expectations about quality, and share these with others in the programme.
- Promote and enforce quality expectations via management and supervision activities.
- Peer educators will be more motivated to maintain the high quality of their activities if they perceive that quality is an important issue for management. Management should take care to implement quality assurance measures in training, external relations, and other activities.

TRAINING

1. ARRANGE FOR APPROPRIATELY SIZED GROUPS

Keeping the size of the trainee group between 15 to 20 is appropriate for the trainer, allows for effective participation, offers opportunities for leadership and skills practice, and allows for full interaction among peers and trainers.

Tips:

- Even when the training group is a manageable size, it can often be helpful to divide into smaller sub-groups during exercises to allow trainees to practise new skills.
- Training in 'waves' (new groups start the training program every few months) can solve the problem of group size, and also capture new recruits in a timely way.

2. STRUCTURE AGENDA AND TIME TO MEET TRAINING NEEDS

The training must be carried out so that modules are given appropriate time according to relevance and trainees' levels of knowledge and skills. Avoid overloading trainees. Include time for evaluation, summary, and planning future tasks.

Tips:

- Plan for adequate time especially for participatory activities. If the schedule cannot be followed as planned, make adjustments as soon as possible, prioritising activities to ensure there is time for the most important of them.
- Maintain balance between unanticipated discussions and the need to keep on schedule to accomplish the critical objectives of the training.
- Vary the type and pace of activities, paying attention to the tempo of the day.
- Peer educators need a minimum level of knowledge and skills, so that they can transmit accurate information on the topics for which they are being trained.

3. PROVIDE RELEVANT MATERIALS AND HANDOUTS

Participants must be provided with materials in advance and during the sessions, as appropriate, including practical handouts and materials for exercises. Copies of reference and review materials should be provided at the conclusion of the training.

Tips:

- Be a model of good planning and provide useful, professional materials in a timely and orderly way.
- Use notebook binders, colour-coding, and other means to keep materials organised and easy to access.

4. USE INTERACTIVE, PARTICIPATORY, AND SKILLS DEVELOPMENT APPROACHES

The training should model approaches that maximize trainee participation, such as interactive exercises, opportunities to practise new (or important existing) skills, and role play situations peer educators may encounter.

Tips:

- Ensure that peer educators are given ample time to practise skills during the training.

5. IMPLEMENT TOOLS AND METHODS TO EVALUATE TRAINING AND TRAINING PARTICIPANTS

Mechanisms for assessing trainees' knowledge and skills development must be in place at the onset of training (as a baseline), and used at the conclusion of the training (post-training evaluation). Tools should be available for trainees to evaluate the training.

Tips:

- Seek expert assistance in developing tools to assess trainees' progress and the training itself, including facilitator performance. Use these results to redesign training as necessary.
- Place a reasonable focus on M & E of the training itself, and look towards future M & E efforts to address ongoing assessment of peer educators' skill acquisition and performance.
- In addition to using pre and post-tests to determine whether peer educators have acquired the necessary knowledge and skills, it is useful to have trainees complete an evaluation at the end of the training. In this way, they can help identify strengths and weaknesses of the training, as well as topics about which they would like refresher courses and updates.

6. DISCUSS ETHICAL ISSUES

Ethical issues, such as confidentiality, power balance, and gender equity, that are likely to arise in connection with peer educators' activities, must be discussed as part of the training.

Tips:

- Ensure that topics are considered in light of the most pressing, relevant concerns.
- Talk about ethical issues amongst the programme planners before training commences.
- Consider working with a team of staff and peer educators to develop a code of ethics for the programme.

PLANNING CYCLE



SUPERVISION

1. SUPERVISORS MUST HAVE BEEN TRAINED IN SUPERVISION SKILLS, PROGRAMME EXPECTATIONS, AND PEER EDUCATION CONTENT AND APPROACHES

Tips:

- Supervisors of peer educators must be aware of the importance of team building.
- Supervisors need to be sensitive to group dynamics, aware of their own values and attitudes, and perceptive about psychological issues faced by peer educators.
- Supervisors must periodically review programme plans and objectives, to make sure their oversight complies with programme goals.

2. ENSURE THAT PEER EDUCATORS ARE WELL PREPARED

Supervisors must ensure that peer educators have received adequate preparation, through training and skills acquisition/practice, before they begin their work. Updates of knowledge and skills must be provided as needed.

Tips:

- After the initial training, supervisors should track performance and requests for assistance, and schedule timely refresher courses or other ways to meet identified needs.
- Some supervisory activities should take place in the field, once the peer educators have begun their work.
- Weekly meetings should be held, in which peer educators lead activities in front of the programme staff on various reproductive health issues that they choose, organise, and develop. This will provide opportunities for improving their facilitation and communication skills, updating their knowledge on reproductive health issues, and formulating their attitudes and ethics as peer educators.

3. CONTINUALLY REINFORCE MOTIVATION AND ETHICAL BEHAVIOUR

Supervisors must continually reinforce peer educator motivation, monitor limitations (personal or professional), reinforce compliance with the code of ethics, and monitor sensitivity to gender and cultural concerns. Supervisors must promote opportunities for personal development.

Tips:

- Motivating peer educators and reinforcing compliance with ethical standards requires a balance between realistically accepting young people's behaviour and managing external expectations of stakeholders as well as the target population (peers).
- Support mechanisms are needed to address motivation, stress, and burnout among peer educators.

4. MANAGE THE GROUP DYNAMIC AND ENCOURAGE TEAM BUILDING

Supervisors must manage the group dynamic, encourage team building, promote a safe environment, and stay aware of personal relationships.

Tips:

- Supervisors must monitor individual behaviour, personal interactions, and the group dynamic overall, intervening early if problems surface.
- Training and group supervision meetings should retain a climate of safety, fun, and teamwork. Icebreakers and other activities, some of them physical, are important team-building tools and should not be discarded for lack of time.
- It is necessary, not unprofessional, for peer educators and supervisors to use group sessions to address upsetting issues, with appropriate concern for emotional safety.
- Programme managers are wise to develop rules that limit the amount and nature of gossip. They might specify conditions, under which supervisors must be told immediately about the conduct of a peer educator, and other conditions in which the proper procedure is to talk directly with the offending person, rather than involve supervisors, unless the behaviour is not remedied.
- Supervisors must share supervisory responsibilities with peer educators, and involve them as active participants in the supervision process, with feedback regularly invited.

Tips:

- Supervisors and supervisees should agree explicitly on what the supervisee wants, and needs, to learn.
- Supervisors and peer educators should look at programme outcomes, when considering how supervision or supervisory activities can be changed, to improve programme results.
- Ideally, peer educators should be active participants in their own supervision.
-

LEARN SKILLS TO GIVE CONSTRUCTIVE CRITICISM

WHY IS GIVING FEEDBACK AN INTEGRAL PART OF SUPERVISION SKILLS?

The role of the peer mentor as a supervisor is to guide peer educators to perform their tasks/ roles effectively

WHAT HAPPENS WITHOUT PROPER FEEDBACK?

- Mistakes
- Failure to meet goals and targets
- Disrepute of programme and programme personnel
- Internal/external fighting
- End users suffer impact
- Funding may be withdrawn

WHAT ARE THE KEYS TO EFFECTIVE FEEDBACK?

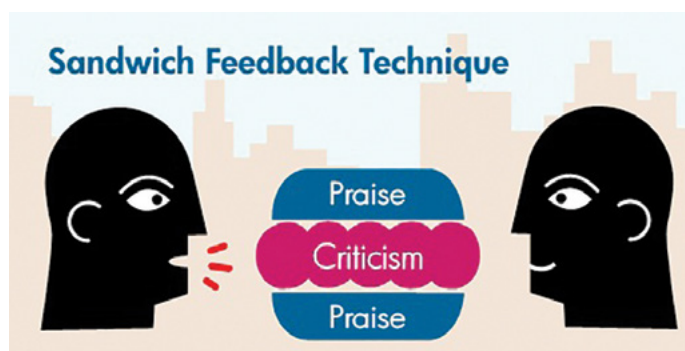
- Don't make assumptions
- Ask question to clarify issues
- Keep your focus in the conversation
- Focus on behaviour, not the person

Feedback is an important cog in communication skills, both at a personal and professional place. Effective feedback helps us in negotiation, meetings, interviewing, building consensus etc.

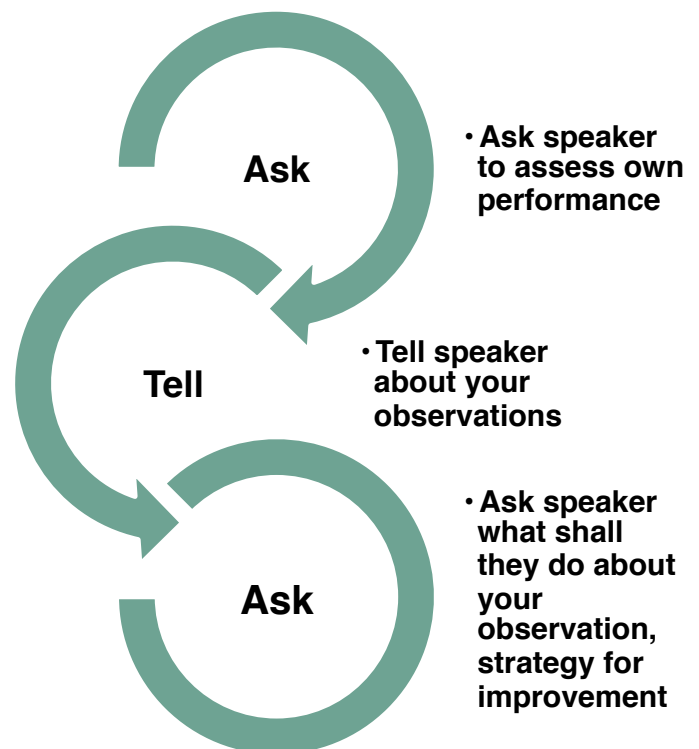
Below are the two feedback techniques that are recommended

SANDWICH FEEDBACK

The sandwich feedback technique is a three-step procedure to help managers who are ill at ease with providing corrective feedback. The sandwich feedback method consists of praise, followed by corrective feedback, followed by more praise. In other words, the sandwich feedback method involves discussing corrective feedback, that is "sandwiched" between two layers of praise.



ASK – TELL – ASK



TRAINERS' COUCH

ANSWERS TO THE TOUGHEST QUESTIONS

WHAT DO YOU DO WHEN YOU DON'T KNOW THE ANSWER?

People may come up with some excellent questions and comments or asking for an opinion, for which you have no easy answer. Sometimes you just don't know. Sometimes, it's possible to find the answer if you had the time to research. Or sometimes, the answer is complex and might depend on the situation.

- Don't try to provide answers if you don't know the answer. It's fine not to know. It's NOT fine to try to fake your way through it, because they WILL know, and you'll destroy any credibility you have.
- One option you have is to say: "I'm not sure", or "I haven't really thought about that, but it's a good question", and then throw it back to the group: "What do others think?"

HOW DO YOU KEEP TRAINING FRESH AND INTERESTING TO THE TRAINEE?

Remember that if you, the trainer is bored with your training, YOU will bore your trainees. In most instances, one cannot alter the core content, but what you can do is experiment with different approaches and methods, techniques and strategies, energisers and games, update information etc.

WHAT IF YOUR PARTICIPANTS JUST AREN'T RESPONDING?

Try different innovative techniques to get people to participate; fish bowl, post-its, small groups, individual written work etc.

DEALING WITH A RESISTANT PARTICIPANT

This is a common issue in training. Here are some possible responses, using examples that might occur in a session, on techniques for dealing with difficult participants:

- "You probably know your context better than I do, so you can pick and choose what seems to work for you. The only thing I'd suggest is that you give it a try. Or, if it's uncomfortable for you for whatever reason, don't use it. You don't need to use all the techniques to succeed."
- "That's fine. Is there any way to change the technique in such a way so that you'd be more confident about it working?" This is a technique that involves throwing back the comment, and

getting the commenter to think a little more deeply.

- A similar response is: "Do you have some ideas about what will work for your particular situation?"
- Or, invite the rest of the group to comment. "Let's see whether others in the group feel the same, or differently about this. Do you think this might work?" [to the group]

Sometimes you'll get one person who is particularly sceptical regarding the content. Typical comments include: "Well, that won't work for me", or "If someone said that to me, I'd hate it".

The general rule to follow here, is a path of least resistance:

- Give participants the 'freedom' to disagree
- Never pressure or try to convince participants to use a particular technique, but suggest that it's up to them and that they might be surprised if they try it out.
- Open up the discussion to the group, by asking: "It's OK if you're uncomfortable using this, but I'm wondering what others think. Can you see this working?"

HOW CAN A TRAINER REFOCUS A TRAINING SESSION ONCE IT'S GONE OFF TRACK?

REFOCUSING BACK TO THE AGENDA

Sometimes it's necessary to cut short discussions due to time constraints, or because the comment or questions is somewhat off-topic. If you feel the need to do that, and it's likely you will for some groups, here are some tips:

- For questions and comments that are relevant, but probably not of interest to other group members, suggest the person talk to you at break, or right after the workshop.
- For questions and comments that are not constructive, lead the discussion astray, or as sometimes happens, show a hidden agenda. It's useful to recognise that the person is passionate about the comment, but that you can't do anything about the situation they might be describing. For example, institutional management that is unsupportive, or a gripe about someone or somebody. In this situation refocus by indicating that we need to focus on things we have some control over which are our own behaviour and actions, so we need to get back to that. Then return to the agenda.

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United Nations Population Fund (UNFPA) and Family Health International (FHI). Training of Trainers Peer Education and HIV/AIDS: Concepts, uses and challenges, Report of a consultation cosponsored by UNAIDS Geneva, Switzerland 1999

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Shoemaker et al., 1998; Flanagan et al., 1996

Adamchak S. Youth peer education in reproductive health and HIV/AIDS. Youth issues paper 7.

United Nations Population Fund and Youth Peer Education Network (Y-PEER). 220 East 42nd Street, 18th Floor. New York, NY 10017, USA

MODEL ANSWERS FOR PRE AND POST TESTS

PEER EDUCATION AND FACILITATION SKILLS

Question Number	Answer letter	Answer
1	b	Involves participants in activity and uses their experiences in the classroom to facilitate their learning
2	c	Direct experience, Reflection, Generalization and Application
3	b	Energiser
4	b	"one that is of equal standing with another; one belonging to the same societal group especially based on age, grade or status"
5	c	The Social Learning Theory
6	N/A	a) Exploratory b) Participative c) Delegating d) Directive
7	N/A	- Eases the pressure of full responsibility - Allows for joint planning, evaluation and feedback - Brings different experiences and attributes to the group - Means a greater sharing of skills, resources and energy - Enables less experienced facilitators to develop skills
8	N/A	- Joint planning, evaluation and feedback are time-consuming - Co-facilitators can be 'played off against each other' - One co-facilitator may get on better with the group - Feelings of insecurity may arise for facilitators - One co-facilitator may dominate - Rivalry between co-facilitators may develop - Vague definitions and unclear delineation of responsibilities may cause problems between co-facilitators
9	b	Have a professional qualification relate to the subject matter

FIRST THINGS FIRST MODULE

Question Number	Answer letter	Answer
1	c	HIV is a virus that attacks the human immune system and AIDS. AIDS refers to the stage where the individual's immune system is depleted of CD4 cells to the point where it is no longer able to effectively fight infections/disease.
2	b	Works by interrupting the life cycle of the HI virus
3	N/A	PEP - Post Exposure Prophylaxis PICT - Provider-initiated HIV counselling and testing HTS - HIV testing services PrEP - Pre-Exposure Prophylaxis
4	b	If a woman is HIV-positive and pregnant, there's a 80 percent chance that her baby will contract the virus
5	N/A	Correct results and Connection
6	c	Sexually transmitted infections
7	b	FALSE
8	b	FALSE
9	b	FALSE
10	b	FALSE
11	a	TRUE

12	a	TRUE
13	d	All of the above
14	a	TRUE
15	d	Mood swings
16	N/A	Latent TB

WOMEN'S HEALTH MODULE

Question Number	Answer letter	Answer
1	N/A	Being Young HIV
2	N/A	a) Tertiary prevention b) Primary prevention c) Secondary Prevention
3	N/A	a) Mammogram b) Pap Smear c) High blood pressure/Hypertension Test d) Type 2 Diabetes Screening Test/Glucose Test e) Rapid HIV test/HIV antibody test
4	b	If a woman is HIV-positive and pregnant, there's an 80 percent chance that her baby will contract the virus
5	b	to help prospective parents plan how many children, if any they want and how close together they want them
6	N/A	1. They are more likely to complete their education and add value to the communities where they live and work. 2. Reduce STI's including HIV transmission. 3. Help ensure fewer HIV-infected babies. 4. Less impact on the resources that are available to communities for development.
7	b	FALSE

MEN'S HEALTH MODULE

Question Number	Answer letter	Answer
1	N/A	c. Diabetes f. Testicular Cancer
2	b	FALSE
3	b	FALSE
4	N/A	a) Prevent pregnancy b) Prevent transmission of HIV and some STIs
5	N/A	MMC - Male Medical Circumcision
6	c	6 weeks
7	b	FALSE
8	c	15 years or older

LGBTI MODULE

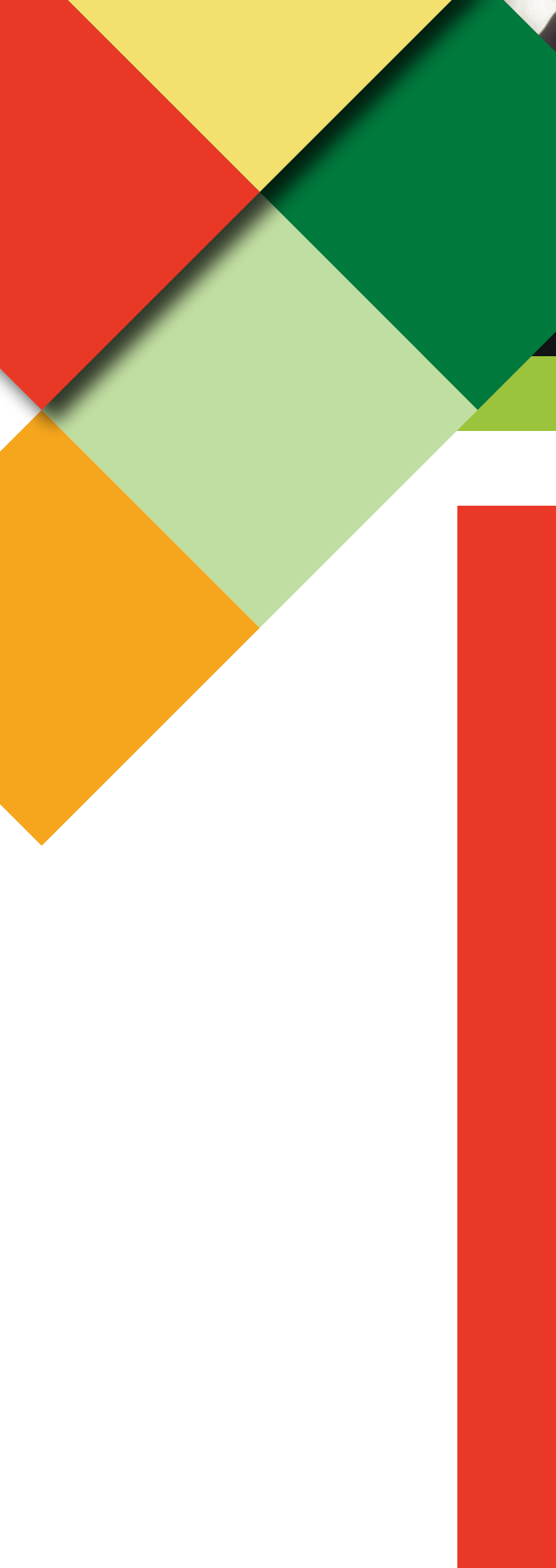
Question Number	Answer letter	Answer
1	N/A	c. Sexual Orientation
2	N/A	b. Bisexual
3	b	FALSE

4

Term	Answer	Definition/Description
1. Transphobia	E	a. The irrational fear, disgust, or hatred of gays, lesbians, and/or bisexual people, or of homosexual feelings in oneself
2. Transgender	F	b. emotional, romantic, and sexual attraction to people whose biological sex is different from and the same as one's own
3. Bisexuality	B	c. people born with ambiguous genitalia or genitalia having characteristics of both sexes.
4. Heterosexism	D	d. the assumption that everyone is heterosexual
5. Homophobia	A	e. Intense dislike of or prejudice against transsexual or transgender people
6. Intersex	C	f. an umbrella term referring to people whose gender identity I. differs from the social expectations for the biological sex II. identified as theirs at birth

ALCOHOL AND DRUG ABUSE PREVENTION MODULE

Question Number	Answer letter	Answer
1	N/A	a) Craving b) Loss of control c) Physical dependence d) Tolerance
2	N/A	a) Demand reduction b) Supply reduction c) Harm reduction
3	N/A	a) Frontloading b) Backloading
4	N/A	Depressant



DIRECTORY OF SERVICES

POWA (People Opposing Woman Abuse)

T: 011 642 43 45/6

PSH (Partners in Sexual Health)

T: 023 414 4169

Eastern Cape

Mother2mothers

0800 668 4377

Rape Crisis

T: 021 447 9762 (24-hour line)

Wits Reproductive Health Institute: Esselen Street Clinic, Johannesburg

T: 011 725 671

HIV counselling and testing, TB screening
and treatment, psychosocial support groups,
mobile clinic outreach services, ARV initiation

Marie Stopes South Africa

0800 117 785

National Toll Free PMTCT Related Informational Helpline

0800 mothers

Diabetes Action

031 205 9886

Lifeline

National Counselling Line: 0861 322 322

National AIDS Helpline: 0800 012 322

The South African Depression and Anxiety Group - Website: www.sadac.org

Suicide crisis line: 0800 567 567

(8am - 8pm, seven days a week)

Help line: 011 262 6396

(8am - 8pm, seven days a week)



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